

How Childless Older Adults in Rural China Choose between Ageing-in-place (AIP) and Institutionalization

CHEN, Hongzhou

陈虹舟

Ph.D.

The University of Hong Kong

May 2021



Abstract of thesis entitled

How Childless Older Adults in Rural China Choose between Ageing-in-place (AIP) and Institutionalization

Submitted by

CHEN, Hongzhou (陈虹舟)

For the degree of Doctor of Philosophy
at The University of Hong Kong
in May 2021

Background: Ageing-in-place (AIP) referred to ‘the ability to live in one’s home and community safely, independently, and comfortably’. From social policy perspective, it was encouraged by policy makers to address the challenge of population ageing and the long-term care (LTC) needs for older adults. While the AIP agenda in rural China anchored the long-term caring responsibilities to family members – especially adult children, the voice of childless group remain unexplored. In other words, little is known about how childless older adults living in rural China make their residential decisions (regarding to AIP or institutionalization).

Research objectives: This research aimed to gain in-depth understanding on the residential decision-making process (AIP or institutionalization) among childless older adults in rural China. The concept of *residential reasoning* – defining as the whole process of assessment, adaptation and residential decision-making – was applied to guide this study. The overall research question of, ‘how and why childless older adults choosing between AIP and institutionalization’ was investigated, followed by two sub-questions from residential reasoning model: (a) how they assess the residential circumstances in rural community; (b) how they struggled to adapt before making the



residential decisions.

Method: This research was informed by the constructivism grounded theory approach, as it enabled ‘dialogue’ between new evidence with existing theoretical framework (e.g., the residential reasoning model). There were total of 27 childless participants (aged 60 to 92) were recruited from rural area in Yunnan, China from face-to-face interview, including 17 community-dwellers and 10 institution-dwellers. 6 informants (including village cadre, anti-poverty working team members) were interviewed to enhance method rigor. Data collection and analysis were conducted simultaneously, separating from four steps: (a) the initial sampling and coding; (b) initial sampling and focused coding; (c) theoretical sampling, axial coding and theoretical coding; (d) verification. Method rigor was enhanced by triangulation, member checking, peer debriefing meeting and researcher’s self-reflective note.

Results: The long-term residential decision (regarding AIP and institutionalization in rural China) is an ongoing negotiation process; sense of mastery centred on the whole process. The mastery experience is full of ambivalence, encompassing simultaneously self-reliance and the uncomfortable experiences of rural living, including stress, suffering and powerlessness. Further, achieving mastery requires ongoing struggle and adaptation. This research identifies two levels of adaptation, namely individual and interpersonal, respectively: (a) ‘enduring *ku*’, that is, conducting farming and farm-related activities; (b) ‘having someone at home’. If childless older adults could endure *ku* or have someone at home, AIP would be preferable to institutionalization. Conversely, if they were overwhelmed by the stressful and suffering aspects of farming, or they perceived ‘having no one at home’, relocating to a rural institution would be more likely. Essentially, the findings of this study suggested that maintaining a sense of mastery was the key for childless older adults in choosing to age in place.

Discussion and implications: This study investigated the long-term residential decision process among childless older adults – one of the most disadvantaged yet unexplored groups in rural China. It provided in-depth understanding of why certain residential decisions are made and how. This research pointed out that, even though an adaptive effort has unfolded during the residential reasoning process, what remains



ignored is the individual and structural disadvantage of childlessness in rural China. No matter how childless older adults struggled to endure *ku* or having ‘someone at home’, the adaptation process struggling and messy. This reflects the disadvantageous situation of childlessness in rural China, which was further intersect with layers of constrictions such as ageing, poverty and gender inequality. Overall, the long-term care needs of childless older adults require a specific policy response.

Keywords: Long-term care, childlessness, poverty, ageing-in-place (AIP), rural China, decision-making



**How Childless Older Adults in Rural China Choose between Ageing-
in-place (AIP) and Institutionalization**

By

CHEN, Hongzhou

陈虹舟

B.A. Anhui Normal University

M.S.W. The Chinese University of Hong Kong

A thesis submitted in partial fulfilment of requirements for
the degree of Doctor of Philosophy
at the University of Hong Kong



Declaration

I declare that this thesis represents my own work, except where due acknowledgement is made, and that it has not been previously included in a thesis, dissertation or report submitted to this University or to any other institution for a degree, diploma or other qualification.

Signature 陈虹舟

Chen Hongzhou, Shirly

Date 2021.5.28



Acknowledgement

It is until now, when I type this sentence, I realize it marks an end of a journey. After 4 month of writing, 4 years of preparation – literature review, data collection and analysis, this is what I declared as my ‘production’ and ‘payback’. I always dislike functionalism perspective of knowledge production. It implies that every journey has to lead to an end - what are usually named the ‘production of intellectual work’, or ‘research conclusion’. I tend to consider this dissertation as a constructed and valuable experience instead of the ‘infinite answer to some research questions’. This work which might be challenged by myself someday. It definitely will be.

It was never easy to navigate to the ‘end’ – if there exists any marking stone. It was never easy for me – since I grow up in family of ‘middle class’ – to distinct the merit of diploma, that is, the socially constructed cultural capital with the true meaning of knowledge production (here I refer term from the objectivism perspective). I still remember the time when I struggled to navigate for the ‘most appropriate’ research topic – ‘fancy’, ‘interesting’, better to have maximized publication merit. I also remember the shock when I entered the field, observing how people live and struggled; how they suffered and thrive.

If there’s anything that I’ve learned and would never forget of this journey is the word ‘understanding’. Engaging with social science is a marvelous experience, which enabled me to question many assumptions, values and my background knowledge. Most importantly, this journey enabled me to have a glance to my own sympathy – even this term has been criticized for implying ‘prestigious’. I tend to consider this dissertation as a gift from participants. I deserve no reward but trying to see the participants, record them and have connection with them.

I’d express my sincere gratitude to my supervisor, Dr. Vivian Lou, who influenced me by her research style and her life philosophy: ‘To be a better person’. I’ll never had the chance to tell the story of participants without her support. I’m also grateful to Dr. Roberta Iversen and Dr. Liu Jinyu, who encouraged me to stay in this area and go further. I appreciate all anti-poverty teamwork members (Mr. Zhuang Xiaoxin, Mr. Wang



Zhenhua, Ms. Wang Xingxing and Mr. Ding Xionglin) to guided me to each village and provided me their precious opinions.

Finally, I'm extremely thankful to my parents, Mr. Chen Xianglin and Ms. Peng Po for their unconditional love. This journey may not be able to finish without their encouragement. My gratitude would belong to my boyfriend, Liu Zhuolin. I'll always remember the moment when we exchange our observations, opinions and criticism on social phenomena – on everything. I suppose social science changed him as well. Thanks for my friends for their considerate support: Yan Kunqi, Ma Fanya, Zhang Zihuan, Margaret Liu Yinxin, Mo Yufei, Jasmine Wang Yixin and my lovely Weibo friends. I'd express my love to Mr. Clarence and Mr. Lu Chen, who sparked my enthusiasm of writing and my passion to feel the world. It was wonderful to have your emotional companionship at the end of this journey.



Contents

Declaration	vi
Acknowledgement	vii
Contents	ix
List of Tables	xiv
List of Figures	xv
List of publications.....	xvi
Chapter One.....	1
Introduction	1
<i>1.1 The choice of the topic: ‘What about those older adults who doesn’t have children?.....</i>	<i>1</i>
<i>1.1.1 Population ageing, ageing in place and childlessness.....</i>	<i>1</i>
<i>1.1.2 Childlessness in worldwide</i>	<i>3</i>
<i>1.1.3 Childlessness in rural China</i>	<i>6</i>
<i>1.2 Residential service for the childless in rural China</i>	<i>11</i>
<i>1.3 Residential choices in later life.....</i>	<i>16</i>
<i>1.4 Gaps, research aims of this study and the structure of this thesis</i>	<i>19</i>
Chapter Two	23
Literature Review	23
<i>2.1 Key questions, key words and searching strategies of literature review</i>	<i>24</i>
<i>2.2 The positivism approach to residential decisions.....</i>	<i>25</i>
<i>2.2.1 Demographical characteristic and residential decisions</i>	<i>26</i>
<i>2.2.2 Health-related character and the residential decision.....</i>	<i>27</i>
<i>2.2.3 Social relations and the residential decisions</i>	<i>28</i>
<i>2.2.4 Residential satisfaction and the residential decisions</i>	<i>29</i>



2.2.5 Summary of this section	30
2.3 The interpretivism approach, critical turn and meaning of place	30
2.3.1 Attachment to home and residential decisions	32
2.3.2 Autonomy, identity and residential choices	33
2.3.3 Social connection, power imbalance and the residential decisions	34
2.3.4 The ambivalence and cultural expectation	34
2.3.5 Summary of this section	35
2.4 Calling for new models/perspectives for residential-decisions	35
Chapter Three	38
The Residential Reasoning Model	38
3.1 Calling for new research agenda on later-life residential decisions	38
3.1.1 The residential normalcy model	39
3.1.2 The residential reasoning model	41
3.1.3 Other related models exploring resilience or residential decisions	42
3.2 Residential reasoning model rationale	43
3.3 Residential assessment	45
3.4 Adaptation	46
3.5 The significant other(s)	48
3.6 Summary of this chapter and rationale of research questions	50
Chapter Four	52
Methodology and Research Design	52
4.1 The paradigm debate and rationale of qualitative inquiry	52
4.2 The rationale of the constructivist grounded theory approach	54
4.2.1 Inductive or abductive?	55
4.2.2 Formal theory or substantive theory?	56
4.2.3 Value free or critical awareness?	56
4.2.4 Rationale of method selection	57

4.3 Overview of research process	59
4.3.1 Participant selection: inclusion/exclusion criteria, research site and accessibility	59
4.3.2 Description of research design	63
4.3.3 Interview guideline	64
4.3.4 Ethics approval	66
4.3.5 Research process in detail	66
4.4 Enhancing the method rigor	75
4.4.1 Triangulation	75
4.4.2 Member checking	76
4.4.3 Peer debrief meeting	77
4.4.4 Reflexive notes	78
4.5 Summary of this chapter	82
Chapter Five	84
Childlessness in Rural China	84
5.1 Introduction of participants	84
5.2 Childhood experience	86
5.3 Working experience	89
5.4 Family relationship	92
5.5 Becoming childless in later life	94
5.6 Summary of this chapter	97
Chapter Six	100
Residential Assessment and Ambivalent Mastery	100
6.1 Defining ambivalence	100
6.2 Mastery of ambivalence during farming: balancing between self-reliance and stress	102
6.3 Mastery of ambivalence on interpersonal relationship: experiencing the	



<i>connection and constriction</i>	104
6.4 Mastery of ambivalence when voicing one's needs: being practical and feeling disempowered	108
6.5 Gendered mastery and placeless women	111
6.6 Summary of this chapter	114
Chapter Seven	117
Enduring Ku and Residential Decisions	117
7.1. Defining ku in narrative form	117
7.2 Enduring ku as ongoing negotiation	118
7.2.1 Routinizing ku in a flexible way	118
7.2.2 Reappraising the negative aspect of ku	120
7.2.3. Transferring the narrative of ku into toughness identity	123
7.3 Summary of this chapter	125
Chapter Eight	128
Having Someone at Home and Residential Decisions	128
8.1 'Having someone at home': revealing the significant other(s) of participants	128
8.1.1 Designating the significant others from vertical kinship tie	129
8.1.2 Designating significant others from horizontal kinship tie	131
8.2 The ongoing negotiation: How 'having someone at home' influence the residential decision	132
8.2.1 Continuing the relational closeness	132
8.2.2 Formalizing the caring relationship	134
8.2.3 Implementing the filial responsibility	136
8.3 Summary of this chapter	140
Chapter Nine	143
Discussion and Conclusion	143

9.1 Childlessness in rural China	145
9.2 Mastery of ambivalence: how childless older adults assess their residential circumstances	148
9.3 The residential decision as ongoing negotiation process	153
9.3.1 Enduring ku and residential decisions	153
9.3.2 Having someone at home and residential decisions	155
9.4 Gendered reasoning, placeless women and precarity	159
9.5 Contribution, policy implications and limitations	161
9.5.1 Contribution to literature on long-term residential decisions	161
9.5.2 Contribution to residential reasoning model.....	162
9.5.3 Policy implication	165
9.5.4 Limitations of this research	167
9.6 Future studies	168
9.7 Summary and conclusion	169
Reference	172
Appendix	203



List of Tables

Table 1.1: Childless women at the age of 40-44 (from mid-1990 to 2010)	4
Table 1.2: Policy development of childless protection in China since 1956	13
Table 4.1: Demographic features of participants	62
Table 4.2: Description of research design	63
Table 4.3: Interview guideline	65
Table 4.4: Research focus during axil coding process	70
Table 5.1: Introduction of participants	85



List of Figures

Figure 1.1

Number of Beds in institutions in China, from 2015 to 2019 14

Figure 1.2

Number of childless individuals living in rural communities and rural institutions, from 2011 to 2019 16

Figure 9.1

The Residential reasoning process of childless older adults in rural China 170



List of publications

Peer-review Journal

Chen, S. H. Z., & Lou, V. W. Q. (2021). Residential reasoning: how childless older adults choose between ageing in place (AIP) and institutionalisation in rural China. *Ageing and Society*, 1–19. <https://doi.org/10.1017/S0144686X2100074X>

Book Chapter

Peer-review Conference

Hongzhou, Chen., & Lou, V. W. Q. (2020). Certainty matters: Applying Prospect Theory to Analysis Relocation Decision-Making Among Childless Older Adults in Rural China. Paper will be presented at the Society of Social Work and Research (SSWR) conference, California, USA.

Hongzhou, Chen., & Lou, V. W. Q. (2020). Someone at Home: How Kinship Support Influence Ageing-in-Place Decision among Childless Older Adults in Rural China. Paper will be presented at the Gerontological Society of America (GSA) conference, Philadelphia, Pennsylvania, USA.

Hongzhou, Chen., & Lou, V. W. Q. (2019). Ageing in Place (AIP): Preference and the Voice of Caregiver among Hong Kong Residents. Paper presented at the 9th International Conference of Health, Wellness and Society, Berkeley, California, USA.



Chapter One

Introduction

1.1 The choice of the topic: ‘What about those older adults who doesn’t have children?’

1.1.1 Population ageing, ageing in place and childlessness

Increasing life expectancy has leads to the ageing population bulging rapidly worldwide. Among G20 members, Japan is experiencing the most drastic ageing population expansion (47 people older than 65 per 100 working-age adults in 2015, up from 19 in 1990), followed by Italy (36), Germany (34) and France (32) (The Organization for Economic Co-operation and Development, 2019). In the Asia-Pacific area, the share of the population aged 65 years and over is expected to reach 14.1% for females and 11% for males in low- and lower-middle-income countries by 2050. The number will rise to 32.6% and 23.1% for females and 27.5% and 19.4% for males for high- and upper-middle-income countries respectively in 2050 (Organisation for Economic Co-operation and Development, 2020).

The challenge of population ageing is more prominent in developing counties than developed countries, as demographic change surpasses the speed of economic development and the expansion of the welfare system (Feng, 2019). In China, for example, individuals aged over 65 years constituted 10.5% (144 million) of the total population in 2015, and the proportion is expected to reach 26% (358 million) by the year 2050 (National Bureau of Statistics of China, 2015). The number of people aged over 80 will grow even faster than any other age groups, rising from 22 million (1.5%) in 2015 to 115 million (8.2%) in 2050 (National Bureau of Statistics of China, 2015). It is estimated that the fragility rate of older adults will increase to 16.92% (of the population aged over 60) by 2050, which will cover up to 1.40% of GDP to address the aged-care services (Lu et al., 2017). This brings into focus how a social security



programme could provide available and sustainable long-term care to older adults (Robison et al., 2014).

The policy agenda of ageing in place (AIP) was initiated to address this challenge. As the neo-liberalism turns in western context champion deinstitutionalization, family and community are increasingly held responsible for care, health and well-being of elder members of society (Dalmer, 2019). AIP was defined as the opposite side of institution (also referred as elder-care facilities or residential facilities in some literature) relocation. The most predominant definition that launched by the *Centres of Disease Control and Prevention (CDC)* is, ‘the ability to live in one’s home and community safely, independently and comfortably, regardless of age, income or ability level’ (Centers for Disease Control and Prevention, 2013). AIP has proved to be beneficial to one’s wellness by providing a sense of attachment and control (Wiles et al., 2012), enabling a sense of independence and autonomy – a positive identity that has been valued prominently in Western culture (Stones & Gullifer, 2016; Vasara, 2015). Most importantly, staying in one’s own home fulfil the willingness of most older adults. It aligns with the wishes of approximately 80% of older adults who prefer to grow old in their own home rather than relocating to aged-care institutions (some referred to residential facilities or assisted living, depending on the welfare characteristics) in Western and Asian society (Ahn et al., 2020; Ewen et al., 2014; Lehning et al., 2015; Lum et al., 2016). Unsurprisingly, aged-care institutions are usually pictured as ‘one step from death’ (Peace et al., 2011).

While the AIP agenda echoes with the advocacy of neoliberalism in the Western welfare realm, the initiation of AIP in China responds to a different welfare philosophy. It has been a long tradition for policymakers to anchor their hope on families to shoulder elderly care responsibilities (Wu et al., 2009). On the other hand, the decreasing fertility rate, shrinking family size and dispersed residence among family members force the welfare system to seek a balance between a family-oriented caring system and the emerging trend of population ageing (Feng, 2019). The initiation of the AIP agenda is aimed at combining home-based care with institutional care, assuming that public-funded institutional care ought to be provided to those ‘in need’ – the fragile and dependent group who need to be specifically targeted on the policy agenda (Fang et al.,



2015). The expansion of institutional care in China is considered the ‘last step’ when familial care is not available (Yang et al., 2020).

Reviewing the long-term care policy agenda in China raises the question of who qualifies for public-funded institutional care, if it is not universal. It shifts our focus to those who don’t have family members – especially adult children – to provide long-term care and support. Unfortunately, the current literature on childless older adults in the Chinese context doesn’t give any attention to how childless older adults manage their long-term care needs and expectations, even though they constitute one of the groups most ‘at risk’ of institutionalization (Gu et al., 2007; Wang et al., 2020). Do childless older adults still prefer to age in place? or do they prefer to relocate to institutions? Why? How do they arrange their long-term care needs when faced with age-related challenges? These questions are important in addressing population ageing and preparing the long-term care services in China. This research will fill this gap, focusing on the question of where childless older adults should grow in rural China.

1.1.2 Childlessness in worldwide

The demographic trend towards fewer children is intertwined with rising life expectancy and the increasing number of old-age dependent individuals worldwide, imposing layers of challenges on the long-term care system and age-based services (Zaidi & Morgan, 2017). The Organisation for Economic Co-operation and Development (OECD) defined childlessness as the number of children a woman has at the end of her reproductive period (usually aged between 45 to 49, depending on data availability). According to the OECD, the childlessness rate has increased from 17.6% to 18.8% among women in the United States born in the 1940s and 1960s, respectively. In Australia, 7.6% of women born in the 1940s are childless, compared with 21.54% for the 1960 cohort. In Korea, 3.6% of women born in the 1940s are childless, and the number increases to 6.78% of women born in the 1960s (Organisation for Economic Co-operation and Development, 2020). The changes in the childlessness rate of women between the mid-1990s and 2010 are shown in Table 1.1.



Table 1.1***Childless women at the age of 40-44 (from mid-1990 to 2010)***

	mid-1990s		2010 (or latest year)	
	%	Reference year	%	Reference year
Australia	12.80	1996	16.00	2011
Austria	7.60	1996	21.54	2010
Canada	15.90	1991	18.94	2007
Chile	7.90	1992	7.72	2002
Czech Republic	4.90	1997	7.10	2011
Estonia	9.40	1989	10.20	2011
Finland	14.60	1990	19.89	2010
Hungary	8.50	1990	12.00	2011
Korea	3.60	1990	6.78	2005
Luxembourg	19.00	1991	15.42	2001
Mexico	7.00	1990	8.55	2010
New Zealand	11.90	1996	15.00	2006
Turkey	5.40	1990	4.50	2008
UK (England and Wales)	14.00	1995	20.00	2010
United States	17.50	1995	18.80	2010
Bulgaria	8.20	1998	11.70	2011
Croatia	9.40	1991	9.40	2001
Latvia	6.90	1995	8.70	2000
Lithuania	12.20	1995	8.40	2011
Malta	14.10	1995	12.90	2010
Romania	9.70	1992	10.50	2002

Note: The data were derived from the OECD family database (Organisation for Economic Co-operation and Development, 2020).

The increasing rate of childlessness varies across nations, with North America and Europe demonstrating the most drastic increasing trend, while it is less common in the Asia-Pacific region for older adults not to have living children. Verdery's (2019) cross-nation study suggested that one in 10 adults over the age of 50 do not have a spouse or living children in four European countries (Ireland, the Netherlands, Switzerland and Italy) and Canada. In China and Korea, less than one in 50 older adults do not have a spouse or living children. It has been estimated that there are 43.6 million older adults without a spouse and biological children around the world, which constituted 3.8% of the population over the age of 50 in 2015 (Verdery et al., 2019).

Demographers have tended to attribute this demographic trend to increasing life expectancy: a person will outlive their own children and become childless at certain point of life circle (Dykstra, 2009). Some connected this phenomenon with shrinking family size driven by the declining marriage rate and rising divorce rate (Verdery, 2015; Verdery et al., 2019). Sociologists have connected the trend with the expansion of modernization, industrialization and urbanization, which has altered people's attitude toward marriage, co-habitation and childbearing (Zaidi & Morgan, 2017). In the gerontology field, studies on demographic change and the rising rate of childlessness have tended to focus on two considerations: (a) *the pathway to becoming childlessness*; and (b) *the consequences of childlessness in later life* (Dykstra, 2009). For example, some individuals may voluntarily choose to be childless due to long-time education attendance and market participation; some may become childless involuntarily (e.g., infertility or involuntary singleness) (Dykstra, 2009; Ivanova & Dykstra, 2015). The process of becoming childless in later life can be complicated and intertwined with other life trajectories: for example, when a person has had biological children but lost their connection (Dykstra, 2009); or a person has only a distant relationship with their biological children and thus perceives themselves as childless (Allen & Wiles, 2013).

Investigating the cause of childlessness would help to clarify its consequences. In most circumstances, childlessness is not a positive situation in later life. In regard to health status, childless older adults in Europe, Mexico and the United States experience more chronic conditions than their parental counterparts (Quashie et al., 2019). Childless older adults in Switzerland are more likely to live for a long time, which is

further related to higher odds of institutionalization (Pimouguet et al., 2016). Being childless is more likely to be connected with risky health behaviour such as smoking and alcohol addiction, as these individuals lack the strength and motivation of self-control (Kendig et al., 2007).

On an interpersonal level, there is emergent research on exploring the (possible) support deficit among childless older adults. For example, it has been proved that childless older adults receive less instrumental and emotional support than their parental counterparts (Deindl & Brandt, 2017). The situation is worse when the intergenerational link is disconnected, that is, when a person has less chance of receiving support from children, children's spouses, grandchildren and other related family members during ageing (Hadley, 2019). Although the absence of children can be compensated for by friends, neighbours and extended kin (Cantor, 1979), it cannot be concluded that such outreach support can address the intense needs that arise in the face of declining health. Two pieces of evidence account for this argument. Firstly, a cross-sectional quantitative study in Europe revealed that the interpersonal resources for the childless group demonstrate mixed results: a limited amount of support could increase the quality of each source of support (Schnettler & Wöhler, 2016). Secondly, a longitudinal study suggested that the 'true challenge' for the childless is the need for advanced care that is only available among (immediate) family (Klaus & Schnettler, 2016). These two studies partially and respectively responded to the question of who will provide support to the childless group and in what circumstances. It emerged that the most significant concern regarding long-term care for the childless group is not the question of whether – whether there exists any source of support – but when, i.e. when health declines, what happens next? As Ivanova and Dykstra (2015) suggested: 'The more durable and intense bonds that (immediate) family members share are more conducive to the provision of the demanding, long-term care that is often needed eventually' (Ivanova & Dykstra, 2015).

1.1.3 Childlessness in rural China

When shifting our focus from the international perspective to China, the social, cultural, economic and institutional background and its correlation with childlessness



has to be specified. According to filial expectation and discourse, childlessness remains an ‘individual failure’ that most rural residents would like to avoid. The Chinese proverb ‘bu xiao you san, wu hou wei da’ depict the idea how childlessness deviate the patrilineal kinship system, with the lineage being the primary unit of social solidarity (Perrine Susan, 2005). Filial piety established a cultural tenant where adult children are obligated to provide food, physical and emotional care to their older parents, which was legally confirmed from the Family Support Agreement (jia ting shan yang xie yi) since 1980s (Chou, 2011). Support from children such as cash transfer, emotional comfort and daily care became extremely critical in later-life in terms of health status, overall wellness and life satisfaction (Djundeva et al., 2018; Guo, 2014; Zhang & Liu, 2007). Such context enabled the childless older adults extremely disadvantageous in rural area. For example, a cross-sectional quantitative study reported that childless older adults living in rural China has experienced significantly higher level of depressive symptom and level of life satisfaction than their parental counterpart due to lack of monetary support from adult children (Guo, 2014). If a person become childless in later life for whatever reason, the experience and consequences are considered ‘pathetic’ and might be stigmatized, even been marginalized (Zhang, 2007).

Secondly, childlessness is gendered in China, especially in rural areas. According to the filial and patriarchal tradition, only males are entitled to continue the family line (including the family name, family legacy or spirituality of the household). Women are ‘determined’ to become ‘somebody’s wife’ and ‘doomed to leave their own household’, leaving the elderly care responsibility to the son, the girl’s brother (Jin et al., 2013). In other words, women are expected to continue the family line for her husband’s household instead of her own household. Consequently, rural parents tend to invest domestic resources in sons rather than daughters (Djundeva et al., 2018). Girls are less desired by some rural parents. In extreme circumstances, they may be neglected, abandoned or even killed by their own parents (Zhang, 2006).

One consequence of such discrimination is the skewed sex ratio and under-representativeness of newborn girls in rural areas – what scholars refer to as ‘missing girls’ (Jiang et al., 2005). In other words, the number of newborn boys unproportionally surpasses that of girls. Jiang (2005) and Jin (2013) estimated that there were



approximately 11.84 million girls ‘missing’ in the birth cohort of 1929 to 1949 due to manifested gender selection (e.g., gender-based abortion). The number briefly fell after 1949, due to the dominant idea that ‘women can hold half of the sky’; after the 1980s, the male-biased sex ratio increased again to above 105 (the normal range is 97–100.3) (Jiang et al., 2005; Jin et al., 2013).

With this cohort having grown up, there exists a paradoxical demographic consequence that it is easier for women to find a marriageable mate than for men. Due to the unproportional shortage of marriageable women, a man’s household has to face a heavy financial burden when establishing the marital relationship (Jin et al., 2013). The man’s household needs to cover wedding expenses and the expense of building a new house for a young couple. For those who are financially constricted, they might end up with lifelong (involuntary) singleness and childlessness (Zhang & Liu, 2007).

Thirdly, childlessness should be explored in the rural-urban dualistic system, which distinguishes rural residents from their urban counterparts – both parents and childless older adults – in terms of economic development, cultural context, service availability (Gu et al., 2007; Treiman, 2012; Zhu, 2015; Zhu & Österle, 2017). The urban-rural income gap of China has increased over time due to urbanization (i.e., urban residents’ share of the total population). The economic development from 1970s boosted the urban-rural residents’ income ratio, surging from 2.57 in 1970 to a peak of 3.33 in 2009, and declined to 2.7 in 2018 (Yuan et al., 2020). Human capital investment has been considered one of the most significant factors that influenced the urban-rural income gap. Working in urban area at earlier age has proved to be significantly effective to improve household income (Liu et al., 2017). For childless rural residents who had no adult children to work as migration workers to provide instrumental support, and who may face the danger of age-related functional decline, maintaining daily life in rural area could remain such a difficult experience than their urban counterpart (Chen & Lou, 2021).

Similarly, while urban residents tended to configure the idea of filial piety into practical perspectives – to combine the filial love to aged parents with modernism idea of independence, autonomy and individualism, filial discourse and moral expectation



are still dominant in rural area (Guo, 2014; Sun, 2017). Rural parents still demonstrated high level of filial expectation from their adult children on monetary transfer and emotional comfort (Luo & Zhan, 2012), which again, marginalized the childless group from the continuous care they need.

The economic and cultural differentiation of urban and rural area was intensified due to the urban-rural dual social policy – specifically, the urban-biased policy in Chinese society. For example, the Household Registration System (hu kou) has separated the urban and rural residents on the name of “managing the labour force and population”, rendering the rural residents disadvantageous from resources distribution and welfare provision (Feng et al., 2020; Zeng et al., 2020). Even though the pension scheme (New Rural Pension Scheme) and social health insurance (the New Rural Cooperative Medical Scheme) had increased the household income, food expenditure, household utilization and health outcome among rural older residents (Huang & Zhang, 2021; Huang & Wu, 2020), such welfare support are far from sufficient to fulfil needs of rural residents (Yuan et al., 2020; Zhu & Österle, 2017). Despite the long-term care provision in urban and rural area are similarly constrained by insufficient human resources, limited public funding and inadequate policy attention, the expansion of rural long-term care services are particularly constricted by the widespread geographical dispersion of residents (Feng et al., 2020). Applying the Chinese Longitudinal Health Longevity Survey (CLHLS) data in 2013, Zhu (2017) reported that there were 3,682 individuals out of 18,605 sampling pool has functional disability (defined by the extend a person have any difficulties in ADL items). Among them, there were 85.8% of individuals has rural hukou and 35% of them has unmet needs (defined by any needs was unsatisfied from ADL items). Another longitudinal quantitative study, also applying the CLHLS data revealed that childless older adults experienced overall less support from informal network members and were more reliable to institutional care (Hsieh & Zhang, 2021). Overall, all these cultural, economic and institutional constriction of rural community intersect with the disadvantage of childlessness, rendering the experience of childlessness in rural area more difficult than their urban counterpart.

This research has specifically focused on the childless group in rural China for two



reasons. Firstly, the demographic trend of childlessness, intertwined with the trend of population ageing in rural areas in China, is prominent. China has a disproportionate number of people aged 60 and over living in rural areas compared to in large megacities. The rural-urban migration process, which usually refers to young workers relocating to urban areas for job opportunities, has accelerated the ageing process in rural areas (Zhou & Walker, 2016). It is estimated that by 2030, the proportion of people aged 60 or over in rural and urban areas will reach 21.8% and 14.8%, respectively (World Health Organization, 2015). Correspondingly, the number of individuals aged over 60 without biological children and living in rural areas will increase. In 2014, China had 76,000 childless older adults in urban areas and approximately 5.29 million childless older adults in rural areas; together they constituted 2.5% of the older population (aged over 60) across the country (Feng et al., 2020). By promoting the one-child policy and with the number of families who have lost their children increasing, the number of childless older adults is expected to reach 79 million by around 2050 (Hsieh & Zhang, 2021). Hence, targeting the childless group in rural area would benefit the majority of upcoming service-recipient of China's long-term care system.

Secondly, as argued before, childless older adults living in rural areas are susceptible to multiple layers of disadvantages compared to their urban counterparts. In context where farming is the dominant form of 'working', those who face advancing age and age-related challenges may find it more difficult to guarantee life materials such as food (Cai et al., 2012). Interpersonally, rural childless older adults receive overall less monetary support from their children and are more likely to live alone, which further increase mortality and institutionalization (Guo, 2014; Hsieh & Zhang, 2021). When it comes to long-term support system, rural residents have fewer resources to count on and so turn to public-funded institutions – places that usually have unqualified care (Lou & Ci, 2014). Given the individual vulnerability of childlessness and the structural – cultural, economic and institutional disadvantage of the rural context, it is necessary to pay specific attention to the childless older adults who live in China's rural area in terms of their long-term care needs and arrangement.

It has to be notified that some studies separated the group who lost their children (*Shidu* group) from the childless group who had never have biological children (Feng,



2018; Li, 2018). Not surprisingly, the death of children remain an traumatic event which could have long-term effect on one's physical, psychological and behavioural health (Feng, 2018; Zhang, 2007). This research did not distinct these two groups because absent from children's care – no matter for what reason would similarly create support deficit and the needs for long-term care in later life (Hsieh & Zhang, 2021). Besides, older adults without children are all qualified with public-funded institutional care in rural area, regardless of their marital status, physical capacity or financial conditions. The next section reviewed the residential services for childless group in rural China.

1.2 Residential service for the childless in rural China

Before introducing the current literature on long-term residential decisions and the research gap, the long-term care system in China was reviewed in order to clarify the available residential options for childless older adults. In China, especially in rural areas, the long-term care for older adults largely relies on family members, usually one's spouse or adult children (Feng et al., 2020). Following the filial and patriarchal tradition, adult children are expected to respect, obey, please and offer material and non-material support to their aged parents (Chou, 2011). For thousands of years, the cultural tenet has been internalized and practised by Chinese people to fulfil their caring responsibility as well as their positive identity (Lou & Ng, 2012). China's ageing policy agenda attempts to parallel with the cultural expectation, 'to combine the filial and patriarchal tradition – the familial care with communism idea of welfare-universalism' (Lou & Ci, 2014; Zhou et al., 2019). Consequently, the guideline of welfare provision aims to 'enable family members to covers the needs of the majority' while 'targeting those special group who are extremely vulnerable', namely, the childless older adults, orphans and individual with disability with formal support (Pan, 2017).

Lining with the welfare philosophy, the long-term care system in China integrates, at least intentionally, the familial care and community services, leaving limited space from public-funded elder-care institutions (Feng, 2019). The long-term care service guideline is called the '90-7-3 structure of service' – 90% of older adults are expected to receive informal care from family members, 7% are supported by a community service and 3% are care from elderly care institutions (Lou & Ci, 2014). Such design



caters the preference of majority of older adults in both urban and rural context, as relocating to elder-care institutions is depicted disgraceful – violating the filial principle for adult children – and stigmatized – related with idea such as ‘reliance’ and ‘uselessness’, hence are unpreferred in most of circumstances (Chen, 2015).

For childless older adults, who fall into the *Three-no category (san wu)* in Chinese ageing agenda, the public-funded residential services are always available. Three-no category includes older residents (usually aged over 60) and infants who have (a) no source of income, (b) no working ability and (c) no legitimated caregiver (Wu et al., 2009). For rural residents, because adult children are the primary sources of instrumental support, and because a person aged over 60 are unlikely to get employed, childless older adults in each village are basically all covered into the Three-no category. Childlessness is the precondition for the village committee to identify Three-no category. For those who have biological children, no matter how distant their children may reside with them or conflictive their relationship are, the welfare services for Three-no older adults are not available for them.

All Three-no older adults are covered by the *Wubao (wu bao, also referred to Five-guarantee) Scheme*, and being offered by welfare services such as food, cloth, medical care, housing service and funeral preparation by township and central government of China (Wu et al., 2008). This policy was initiated in the 1950s in order to fulfill the basic material needs of the ‘poorest poor social members’, namely childless older adults, orphans and disabled person (Li & Walker, 2018). It was until 2001, the Ministry of Civil Affairs encouraging the central and local government increasing public investment for residential care facilities for *Wubao* rural adults, rural institutions became available in some provinces (Wu et al., 2008). Since 2006, as rural institutions expended nationally, the State Council of China revised a document named *Regulations for the Rural Five Guarantees Work (nong cun wu bao gong yang tiao li)* and emphasized that, all Three-no older adults – the childless older adults – were entitled to choose between staying in their homes (in Chinese, *fen san gong yang*) and moving to rural institutions (*ji zhong gong yang*) (The State Council of China, 2006). In 2014, the Ministry of Civil Affairs renamed the *Five-guarantee Scheme* the *Assistance for Extremely Poor Households Scheme*, re-emphasizing childless older adults as being



among the ‘poorest poor’ whose basic living standard needed to be protected (The Ministry of Civil Affairs, 2016). The table summarizes the policy development of childless protection since 1956.

Table 1.2

Policy Development of childless protection in China since 1956.

Year of initiation	Name of documents (in English and in Chinese)	Policy aims
1956	Outline of National Agricultural Development 1956-1967 (农业发展规划纲要: 1956-1967)	To initiate the Agricultural Cooperative (<i>nong ye sheng chan he zuo she</i>) to provide basic material support to childless older adults, disabled villagers and orphans.
1994	Regulations for the Rural Five Guarantees Work (农村五保供养工作条例: 1994)	To legitimate the governmental responsibility to take care of Three-no older adults – older adults who has no source of income; no working ability and no legitimated caregiver.
2006	Regulations for the Rural Five Guarantees Work: 2006 revised (农村五保供养工作条例: 2006)	To emphasize the two residential services for the Three-no older adults: ageing in place or relocating to rural institutions. To underline that the residential options have to be conducted based on voluntary principle.



Year of initiation	Name of documents (in English and in Chinese)	Policy aims
2014	Provisional Regulation on Social Assistance (社会救助暂行办法 : 2014)	To integrate the <i>Minimal Living Standard Guarantee Scheme (di bao)</i> and <i>Five-guarantee Scheme (wu bao)</i> . To re-emphasize the central and township governmental responsibility on caring for the 'poorest poor' and 'vulnerable' social members.

As of 2019, 4.39 million individuals receiving support from the Assistance for Extremely Poor Households Scheme were covered by over 1.64 million beds in institutions (The Ministry of Civil Affairs, 2019). The graph shows the expansion of beds in institutions (including elderly care, child protection and other types of institutions) from 2015 to 2019.

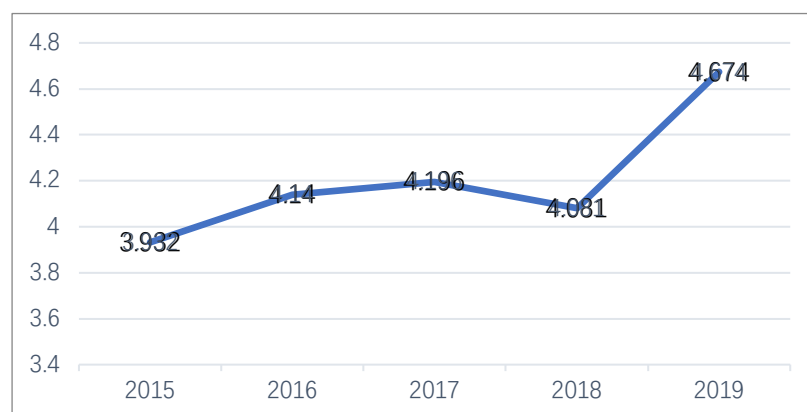


Figure 1.1 Number of beds in institutions in China, from 2015 to 2019 (Millions).

Notes: Data are from the *National Development Report of the Ministry of Civil Affairs* (The Ministry of Civil Affairs, 2019).

Entering rural institutions requires an examination of childlessness status by a village committee; the qualifying age for institution admission varies across local regulations (usually over 60). If institutions are not preferable, childless older adults can also choose to stay in their own homes until they change their mind. Both options are based on the principle of being voluntary. Once a person chooses to stay in a rural community, the welfare subsidies he/she is entitled to are guided by the local standard of the Extremely Poor Household Scheme; the amount depends on the township's economic situation. In Yunnan province, a childless person may receive approximately RMB500 to 800 (USD 62 to 100) per month in welfare subsidies. If this person chooses to relocate to an institution, the subsidies are directly allocated to the institution as a 'management fee'. Each residential dweller may receive RMB50 to 200 (USD 6.5 to 25) per month as 'pocket money'. Food, clothing and medical assistance are all covered by the institutions; no other charges are required (Wu et al., 2009).

Not surprisingly, the majority of childless older adults still prefer to stay in their rural community rather than relocating to institutions, even though the financial concern of institutionalization is dismissed. Since 2011, the number of childless older adults choosing to stay in their rural community has surpassed the number of institutionalized individuals (3.66 million against 1.84 million). While the institutionalized group decreased to 0.75 million in 2019, the AIP group remained at 3.64 million, constituting approximately 82% of childless older adult nationally (4.49 million) (National Bureau of Statistics of China., 2019). The following graph shows the increasing number and percentage of childless older adults in rural China who choose to stay in their rural community (in Chinese, *fēn sǎn gōng yáng*) and the number who choose to relocate to institutions (in Chinese, *jī Zhōng gōng yáng*).



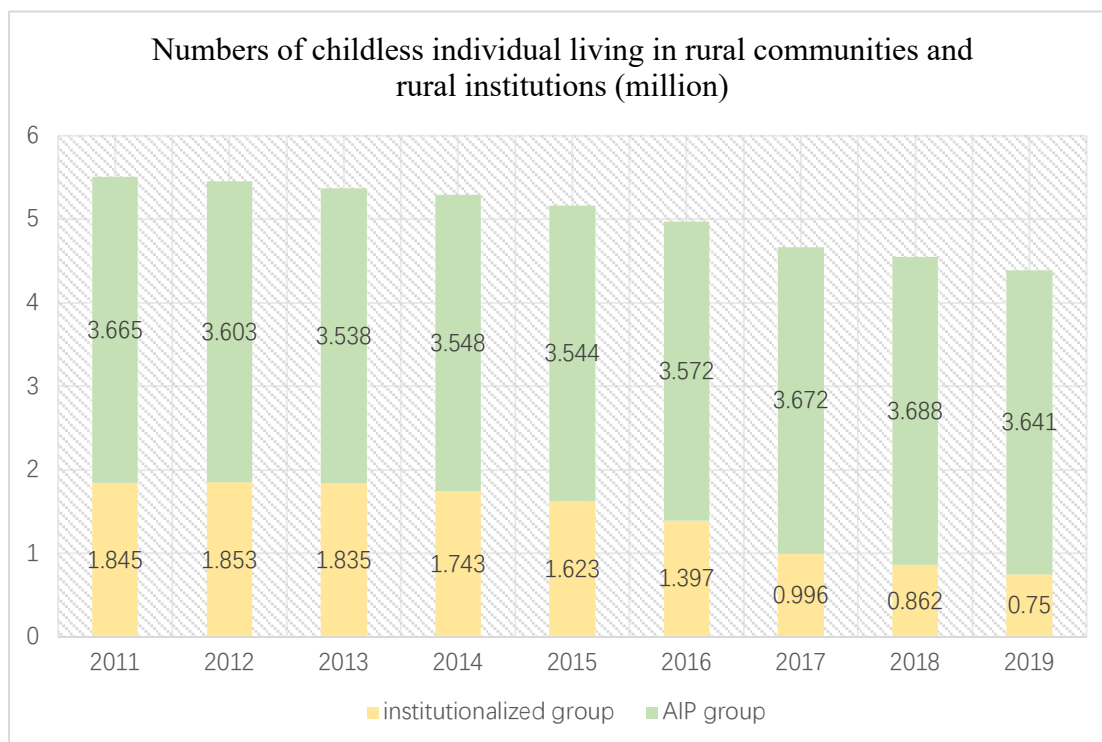


Figure 1.2 Number of childless individuals living in rural communities and rural institutions from 2011 to 2019 (millions).

Notes: Data are from the Yearly Census Data of the National Bureau of Statistics of China (National Bureau of Statistics of China, 2019).

Given the layers of disadvantages of childlessness and rural living in the Chinese context, staying in the rural community is not equivalent to ‘ageing well’, especially when age-related challenges emerge. In other words, although the choice of AIP is preferred by the majority, the prospect of staying and the associated risks deserve attention. This research sought to explore the experience, expectations and needs of residential long-term care among childless older adults in rural China, attempting to answer the question: ‘where would you prefer to grow old (in terms of AIP or relocating to institutions) and why?’

1.3 Residential choices in later life

The question of ‘where to grow old in later life’ has been intensively discussed in gerontology literature, although there is limited focus on the rural context, and even

less on the childless group in rural areas. Overall, the existing literature on why older adults opt for AIP or moving to institutions falls into two broad categories: (a) the studies on the enabling/hindering factors of residential choices; and (b) the studies on lived experience of AIP or experience of residential decisions (Pani-Harreman et al., 2020; Perry et al., 2014). More recent literature has attempted to explore the role of technology in initiating AIP (Pani-Harreman et al., 2020), but this topic is beyond the scope of this research.

In rural areas in the Western context, several factors have been identified as being significant in making residential choices: (a) the health status of residents and the need to access medical support (Dye et al., 2011); (b) geographical remoteness from one's children and family members (Volckaert et al., 2020); (c) supportive facilities, especially medical facilities in rural communities (Erickson et al., 2012); (d) availability of home-based services (Anderson et al., 2018).

There is another line of studies that goes beyond the enabling/preventing factors of relocation and pays more attention to the complex and dynamic facet of later-life relocation. Studies have highlighted how ageing in the rural context involves struggling with balancing one's autonomy and practical needs (Ness et al., 2014), e.g., how the positive identity of toughness and working hard as 'Australian farmers' counterbalances the declining prospect of the rural landscape. When the younger generation migrate from rural to urban areas for job opportunities, ageing is related to the decline of one's identity when passing that identity to the next generation (O'Callaghan & Warburton, 2017). Although rural residents demonstrate a strong preference for AIP rather than institutionalization (Anderson et al., 2018; Winterton & Warburton, 2011), it is unknown how the evidence gained from the parental group can accommodate the voice of the childless – not to mention how childlessness in rural China presents different cultural, historical and demographic characteristics.

Following the findings and argument from previous research, Granbom (2014) applied the term *residential reasoning* to describe the process of later-life relocation and struggling, considering it to be an 'ongoing negotiation process' (Granbom et al., 2014). Defined as 'the combined process of adaptation, struggling and residential



decision’, the choice of AIP or institutionalization is considered a dynamic process rather than a momentary event that is promoted or hindered by certain factors (Golant, 2011; Granbom et al., 2014; Koss & Ekerdt, 2016). This model seemed appropriate to describe the experience of residential decision-making. Specifically, there are two key terms from the residential reasoning model that need to be specified:

(a) Residential assessment: the extent to which an older adult assesses their current residential circumstances. If a person feels mastery and comfortable while staying, the residential congruence is achieved. He/she is more likely to choose AIP rather than institutionalization.

(b) Residential adaptation: since relocating is not preferable in most circumstances, older adults will make an adaptive effort to deal with age-related and health-related challenges while staying. Individuals either change their cognition or attitude toward the environmental difficulties (accommodative strategy) or adjust their behaviour (assimilative strategy) to reach for the ‘best congruence’ of the environment.

This model seems appropriate to sensitize this research for two reasons. Firstly, because the residential experience among the childless is unknown, it is methodologically possible and necessary to apply an existing model/perspective to enable an in-depth understanding and explanation of certain phenomena (Creswell et al., 2007). The argument that the ‘residential decision is an ongoing negotiation process’ provides a reasonable starting point, as the process is proved to be monumental; it may take a long time to assess, compare and hesitate (L. Chen, 2017; Söderberg et al., 2013; Sussman & Orav-Lakaski, 2020). Secondly, for childless older adults living in rural China, the role of adaptation and individual resilience is undeniable. Given the layers of disadvantage they’ve encountered for a lifetime, such as household poverty, cultural stigmatization and gender discrimination, it is reasonable to assume that the process encompasses simultaneous pondering and effortful adaptation.

Overall, the concept of residential reasoning seems suitable to guide this research to investigate the residential choices among childless older adults in rural China. It enables this research to explore the way residential environment of rural living is assessed, and



the way adaptation effort is conducted to reach for desirable (residential-related) consequences. More detailed illustration of this model is elucidated on chapter three.

The next section summarizes the gaps in previous research, the research aims of this study and its potential contribution. Then the structure of this thesis is illustrated in the last section of this chapter.

1.4 Gaps, research aims of this study and the structure of this thesis

Taking together the demographical trend of childlessness and its characteristics in rural China, and the available residential services for the childless group in rural China, there are three limitations in the current literature on later-life residential decisions.

Firstly, the existing literature doesn't pay sufficient attention to the group of childless older adults, who, ironically, remain one of the groups 'at risk' of institutionalization (Ivanova & Dykstra, 2015; Wang et al., 2020). In Western literature, the childless group are considered a 'demographic variable' or 'category of disadvantage', rendering the homogeneity of childlessness, including their experience, voices and complexity, marginalized in the quantitative-dominated research field (Medeiros et al., 2013). In Chinese literature, likewise, the discussion on the long-term care system and its policy implications tends to underlie the macro perspective – retrieving its cultural and historical origin, unfolding the current development and possible trends (Feng et al., 2020; Feng, 2019; Yang et al., 2021). The voice of the childless group – a group who are supposed to be targeted under the current long-term system – is ironically missing.

Secondly and relatedly, because individual lived experience, complexity and subjectivity matter but are marginalized in the research tradition, a limited (but growing) amount of research has recognized the dynamic process of residential decisions. In other words, 'the decision of where to grow old is a negotiation process instead of a momentary event' (Granbom et al., 2014). This is particularly true in rural China because the opposite option to AIP, institutionalization, has been historically and culturally stigmatized (L. Chen, 2017). Even if a person is in need of advancing care,



he/she might struggle for a long time before the decision is finally made.

Thirdly, this research doesn't imply that the residential reasoning model is able to explain all the experience of the childless group. Indeed, the 'struggling and messy' side of negotiation has been neglected in current studies that have attempted to apply this model to empirical data (Koss & Ekerdt, 2016; Stafford, 2017). As Golant (2015) emphasized, different individuals have different ways of coping, and this is further embedded in their social positions and the resources available to them (Golant, 2015). When unfolding the residential reasoning process of childless older adults in rural China, the 'struggling, hesitation and messy side' – the uneven experience has to be underscored, or the assumption that 'any adaptation could lead to a desirable outcome' will seem prestigious. Using the residential reasoning model is also accompanied by examining and revising this model (Charmaz, 2014).

Overall, this research aims to gain in-depth understanding of the residential reasoning process among childless older adults in rural China. The research not only pays attention to the question *why* (what makes childless older adults make certain choices) but also the question *how* (how these residential choices are made through negotiation and adaptation). This purpose includes the following general research question:

How do childless older adults in rural China make the residential decision between ageing-in-place (AIP) and institutionalization? Why?

Specifically, it is further related to two subquestions, based on the argument of the residential reasoning model:

(a) How do they assess their residential circumstances during AIP?

(b) How do they adapt and what are the residential consequences (AIP or institutionalization)?

This research will potentially contribute to the long-term care policy agenda in



rural China when revealing the voice, lived experience and long-term care needs of the childless group. It will benefit the literature on later-life residential decision-making by introducing a dynamic perspective. This research applies the existing model to unfold the experience of the childless group in rural China, seeking revise this model. Overall, by investigating one of the most disadvantaged yet unexplored groups in rural China, this research aims to gain an in-depth understanding of what childless older adults need in the context of their long-term care arrangements.

This dissertation contains nine chapters. The first chapter introduces the research background and aims, and the second chapter reviews the current literature on later-life residential decisions and its empirical findings. This review includes two lines of research: the objectivism line, which regards the residential decision as the outcome of promoting/hindering factors; and the second line, which cherishes the lived experience and subjectivity when making the decision. Following the argument of the second line, the third chapter introduces the origin, key concepts and limitations of the residential reasoning model. Three key concepts are specified: *residential assessment*, *residential adaptation* and *significant other(s)*. Significant other(s) can be considered the extension of residential adaptation on an interpersonal level. In the fourth chapter, the research design, along with its epistemological rationale and details on its practice, is presented.

Each finding chapters of research responded to key word on residential reasoning model, respectively. The fifth chapter provides details on the participants' life experience, hopefully could provide contextual understanding on childlessness in the context of rural China. The sixth chapter answers the question of how participants assess the living circumstances in rural communities while elucidating the term 'ambivalence'. It is suggested that the ageing in rural community is fall into mastery zone yet full of stress and suffering. On the seventh and eighth chapter, the adaptation strategy on individual level and interpersonal level are elucidated, respectively, reporting how childless older adults struggle to achieve residential congruence and how such strategies may (or may not) enable AIP. The final chapter integrates the findings, a discussion, implications and limitations underlying the core argument of this research that 'deciding on where to grow old is a negotiation process that encompasses simultaneously resilient characteristics and a struggling side'. Childless older adults



have to make a real effort in order to age in place, which further reflects their individual disadvantage and the structural oppression of childlessness in rural areas.



Chapter Two

Literature Review

This research aims to gain in-depth understanding on the residential reasoning process among childless older adults in rural China, exploring how certain residential decisions (regarding AIP and institutionalization) are made and why. This chapter reviews the literature on residential decision-making in later life. The topic of where to grow old or whether older adults should choose AIP has been established in demography and environmental gerontology for over three decades (Greenfield, 2012; Pani-Harreman et al., 2020; Vasunilashorn et al., 2012). Many models/perspectives and vast amounts of evidence have been initiated to identify the (individual, interpersonal and environmental) influential factors during residential decisions (Ahn et al., 2020; Perry et al., 2014; Vasunilashorn et al., 2012). Partially due to the paradigm shift in environmental gerontology, more recent studies have begun to point out more complex considerations of AIP and residential decision-making, underlining how places are constructed and created during AIP (Cutchin, 2003; Koss & Ekerdt, 2016; Peace et al., 2011; Vasara, 2015). This chapter follows the theoretical development of this topic, categorizing the previous line of research as the positivism perspective on residential decision-making and the second as the interpretivism perspective.

This chapter contains three sections. The first section reviews the individual, interpersonal and environmental influencer factors on residential decision-making, before presenting the influencer factors on residential decision-making in the rural context. Afterwards, this chapter introduces the lived experience during AIP and the perceptions of making residential decisions in later life. It should be noted that most of the studies reviewed in this chapter were conducted in urban areas, although not many of them focused on the childless group. Finally, realizing the limitations of the previous theoretical lines, this research provides a rational explanation for why residential decision-making ought to be understood as an ongoing negotiation process. The introduction of a residential reasoning model, along with detailed explanation of its key



concepts and limitations, can be found in the next chapter.

2.1 Key questions, key words and searching strategies of literature review

The literature review of this research was conducted from an iterative process, guiding by two research question of ‘how older adults make the residential decision-making’. Noticeably, even this research aimed to focus the childless group who living in rural context, the relevant studies covering the specific population are limited. Hence this chapter reviewed the research population who are aged over 60, regardless of their parental status (aged parents or childless group) or residential circumstances (urban/rural), then underlining findings that close to the population of this research (childless older adults or older adults who living in rural context). The exploratory process would increase the researcher’s familiarity on this topic, so that research gaps was gradually formulated (Arksey & O’Malley, 2005).

The words ‘residential decision-making’ was the key search term of this review, which was linked with term such as ‘ageing (aging) in place’, ‘long-term care’, ‘housing for older adults’, ‘independent living’, ‘ageing (aging) in community’, ‘relocation’, ‘admission for residential facilities’. The term ‘older adults’ ‘aged’, ‘elder’, ‘childless older adults’, ‘childless group’ was used to restrict the research population.

Four electronic databases (PubMed, PsychInfo, Social Science Citation Index, and SAGE) were used for review. Google Scholar was also used to maximize the search results. This review included searching results that covered the topic of residential decision-making among older adults that was published in English language, with no restrictions on types of study (qualitative, quantitative, mixed method study; systematic review and meta-analysis) and research site. Publications with topics that are different from the research goals are excluded (e.g. the insurance decision-making, palliative care for elder adults or clinical decision-making). There were 4210 studies were identified during the process; 568 publications were considered suitable for the current study.



There were a huge amount of research has attempted to explore where to grow old (Greenfield, 2012; Pani-Harreman et al., 2020; Perry et al., 2014). In most circumstances, growing old in one's own home instead of relocating to an institution is the preference of the majority of older adults as well as policymakers (Chui, 2008; Dalmer, 2019; Kendig et al., 2017; Lu et al., 2017). To enable AIP while preventing institutionalization, studies put substantive effort into exploring its enabling factors, key experiences and obstacles. Broadly speaking, the existing literature can be categorized into two types, depending on its theoretical orientation and the paradigm stance: the positivism approach to residential decisions and the interpretivism approach.

2.2 The positivism approach to residential decisions

The first approach follows the tradition of demography and geography, emphasizing the spatial process, trends and outcome of population ageing (e.g., distribution of older population or spatial allocation of resources for older adults) (Andrews et al., 2007; Perry et al., 2014; Scharlach, 2017). With the topic of residential choice (also referred to as 'migration pattern' in some studies), scholars have tended to focus on demographical attributes of migrants –age, gender, income level – and the place characteristics that attract older adults to 'move in' or 'leave' (Ewen et al., 2014; Kendig et al., 2017; Weeks et al., 2012). This allows researchers to explore factors that motivate a change of residence across communities, states and countries.

For example, Wiseman (1980) explored how older adults examined both indigenous and exogenous factors when deciding where to relocate, including health-, wealth-, social- and housing market-related factors (Wiseman, 1980). Litwak (1987) categorized two types of relocation after retirement: the amenities-oriented move, where older adults relocate to certain locations to enhance their lifestyle or improve access to friends; and the assistance-oriented move, where older adults adapt to their increasing needs and seek help from families or institutions (Litwak & Longino, 1987). These frameworks have established the foundations for exploring how resources (e.g., health status, financial ability, social support) influence residential decision-making based on their needs and expectations in later life. The development of a statistical approach and geographic-related techniques (e.g., GOS) further strengthens our



understanding of the later-life migration pattern and demographic features.

For environmental gerontologists, the question of where to grow old needs to go beyond the statistical description of patterns to a theoretical explanation of ‘why people choose to move’. Lawton (1973) promoted an ecological theory to fulfil this goal (Lawton, 1999; Scheidt & Windley, 2003). The ecological model sought to explain how older adults respond to the incongruence between the physical environment and their changing needs. Concepts such as environmental press, individual competence and adaptation have been used and proliferated among followers. According to Lawton’s model, when individual competence decreases to a certain point and the environmental press is too prominent to be adapted, relocating to a more supportive environment is inevitable (Perry et al., 2014).

Accommodating the language of the migration model and ecological perspective, there is substantive evidence attempting to identify the individual, interpersonal and environmental factors influencing the residential decision. This chapter summarizes four of these factors: (a) demographical features; (b) health status; (c) social relations; and (d) residential satisfaction. Although the identified factors are mostly applied in the urban context, some of them have proved significant in the rural context as well.

2.2.1 Demographical characteristic and residential decisions

Increasing age imposes greater needs for advanced and intensive care, although the role of age remains inconsistent in the literature. For example, some suggest that increasing age leads to a greater risk of health decline (e.g., ADL score, IADL score and cognitive function) and institutionalization (Atkins, 2018; Black, 2008; Castora-Binkley et al., 2014; Gu, Dupre, et al., 2007); others suggest that, regardless of the reality of age, older adults demonstrate a stronger wish to age in place due to lifelong familiarity with their own home (Stones & Gullifer, 2016).

Gender is another factor influencing residential decisions, although, again, the evidence is inconsistent. A longitudinal quantitative research conducted in Australia revealed that those aged 75, women and respondents who were house renters were less



likely to age in place and more likely to be institutionalized (Kendig et al., 2017). Women in Japan and the US were found to be more likely than men to move away from their own home – whether to move close to their family members or to enter assisted living facilities (Banaszak-Holl et al., 2004; Matsumoto et al., 2016). In Spain and China, conversely, men were more likely than women to move into institutions (Costa-Font et al., 2009; Gu, Dupre, et al., 2007; Qian et al., 2017).

Some studies have explored the socio-economic factors of older adults and their residential plan. Financial ability generally increases the odds of, and confidence regarding, AIP in the Western context (Ewen et al., 2014). Comparatively, low-income older adults and house renters are more likely to rely on their relatives to fulfil long-term care needs (Costa-Font et al., 2009; Matsumoto et al., 2016), although subjectively they would prefer to age in place (Lum et al., 2016). It seems that the effect of socio-economic indicators on residential decision-making is not the willingness or preference of participants, but the (financial) ability to choose (Feng, 2019). In rural Utah, however, neither age, gender nor income level influences the desire for AIP or institutionalization (Erickson et al., 2012).

2.2.2 Health-related character and the residential decision

Health-related indicators are a significant factor in influencing where to grow old. For example, in the US, cognitive decline is considered one of the most significant reasons for older adults to move into institutions (Banaszak-Holl et al., 2004). In Australia, those who report a higher level of depressive symptoms are less likely to grow old in their own home, even if their desire for AIP is strong (Kendig et al., 2017). More recent research conducted in the US found that health-related characteristics were able to increase behaviour control during AIP, and therefore enhance the confidence with AIP (Ahn et al., 2020). In China, functional decline (measured by ADL score) and having a chronic condition play a key role in determining the need for institutionalization (Gu, Dupre, et al., 2007).

Although the importance of health status is widely recognized when it comes to residential decisions, there is conflicting evidence. The quantitative studies of Ewen



(2014) and Okamura (2020) found that health-related characteristics are not significant in predicting the AIP intention (Ewen et al., 2014; Okamura et al., 2020). For the childless group, health-related characteristics (measured by ADL score, self-rated health and cognitive impairment level) remain the most significant determinants of nursing home utilization (Aykan, 2003). However, a more recent research in the US suggested that health-related variables (measured by physical and cognitive ability, and depression) are not significantly different between newly admitted institution residents and those on the waiting list (Medeiros et al., 2013). One of the possible reasons for this inconsistency is that relocation to institutions could be subjectively perceived as being due to a health decline rather than a clinical level of illness (Wu et al., 2015).

The importance of health is also prominent among rural residents. In Australia, the anticipated relocation decision is framed around health-related and caring needs: older adults assess the extent to which they will gain appropriate care as health declines in deciding where to grow old (Anderson et al., 2018). In Canada and the US, rural residents struggle to manage their physical and cognitive condition in order to AIP (Bacsu et al., 2014; Dye et al., 2011). These qualitative findings correspond with the general assumption of Lawton's model, that is, health decline – especially cognitive impairment – remains one of the most significant factors of assistant-living relocation in rural areas (Gaugler et al., 2007). When it comes to physical impairment caring in daily living, however, rural residents can always adapt to avoid relocating (Ness et al., 2014).

2.2.3 Social relations and the residential decisions

The extent to which older adults can receive appropriate care and support – usually from their adult children or spouse – is identified as being significant in influencing where to grow old (Kendig et al., 2017; Okamura et al., 2020; Wang et al., 2020). The situation is more prominent in China, since adult children are one of the most important sources of support for fragile, aged parents (Cai et al., 2012; Chou, 2010). Because the value of 'raising children to ensure old-age care' is deeply rooted in cultural expectation, both cross-sectional and longitudinal data have proved that the absence of children's care increases institutionalization (Gu, Dupre, et al., 2007; Wang et al., 2020).



Moving beyond the domestic domain, the importance of community support has attracted growing intention in recent decades. Older adults who report a higher level of community participation, civic engagement and volunteering opportunities are more likely to AIP (Ahn et al., 2020; Tang & Lee, 2011). In Hong Kong, low-income older adults are more likely to be influenced by availability of medical facilities in the community in the context of long-term residential decisions (Lum et al., 2016). In mainland China, similarly, community participation and a trusting atmosphere enhance the odds of staying in one's own home over institutionalization (Jiang et al., 2018).

For older adults living in rural areas, recent evidence has pointed to a more complex landscape regarding the role of family carers. For example, a qualitative research revealed that most older adults may consider moving close to their children to prepare for the future challenges they face (Anderson et al., 2018). Another qualitative research revealed that the key element in determining where to grow old is not children's willingness to provide care but the geographical proximity that enhances children's capacity to provide care. If adult children live far away from their parents, it is difficult for rural older adults to choose to AIP, even if they expect to do so (Volckaert et al., 2020). This finding is in line with Pimouguet's (2016) study that indicated that living alone rather than having offspring is the most significant factor in predicting institutionalization and mortality (Pimouguet et al., 2016).

2.2.4 Residential satisfaction and the residential decisions

Residential satisfaction reflects the broad attitudinal evaluation of home and community; it has proved to be important in shaping residential choice (Erickson et al., 2012; Erickson et al., 2006; Ewen et al., 2014). Because older adults usually stay a substantial amount of time in their own home, as they grow old, the residential environment becomes increasingly crucial and sensitive (Wiles et al., 2012). Residential satisfaction has been shown to promote life satisfaction (Byrnes et al., 2006; Kahana et al., 2003) while functional decline undermines residential satisfaction, rendering the residential environment 'unfit' (Oswald et al., 2003, 2007). Not surprisingly, house and neighbourhood satisfaction were found to be important in shaping the intention of AIP (Hillcoat-Nallétamby & Ogg, 2014).



In rural areas, having medical facilities nearby substantially enhances community satisfaction, hence increasing the confidence with AIP (Erickson et al., 2012). A quantitative research conducted in rural Utah supported this argument, revealing that the willingness to stay put depended on the quality of service local facilities provided (Dye et al., 2011). In rural Australia, where driving is the main way to commute, older adults may consider moving to an urban area when their driving capacity declines (Anderson et al., 2018).

2.2.5 Summary of this section

Following the language of the migration model and ecological perspective, previous research identified the demographical characteristics of later-life immigrants (who), the reasons to move (why) and the environmental attractiveness that triggers/prevents moving. Demographical factors (e.g., age, gender, financial ability), health-related characteristics, social relations and environmental satisfaction have proved to be significant influencers on the residential decision in later life, although some factors are inconsistent in different contexts. Such understanding runs parallel with the positivist tradition of geography and demography, which regards later-life relocation as the result of person-environment incongruence. When environments are unable to meet one's needs and goals, relocation becomes preferable to staying (Wahl & Gerstorf, 2020).

While the migration model and ecological perspective tend to relate relocation to the consequence of individual incapacity (mental or physical), there were increasing realization and underlining of individual agency. In other words, older adults are not passively influenced by their living environment; they also experience, create and negotiate with the environment (Andrews et al., 2013; Cutchin, 2003; Pani-Harreman et al., 2020; Perry et al., 2014). Such understanding sets the stage for considering AIP and the residential decision from a more interpretivist and critical perspective. The next section presents the theoretical argument and evidence.

2.3 The interpretivism approach, critical turn and meaning of place



It was not until the mid-1990s that researchers began to incorporate the interpretivism approach and critical perspective into research on ageing, place and residential decisions (Andrews et al., 2009; Pani-Harreman et al., 2020). Gerontologists and geographers have gradually realized that place is not merely a surface on which individual activities and social structures unfold; rather, it is a continuously constructed entity through day-to-day person-environment interaction (Andrews et al., 2013; Cutchin, 2003; Wanka, 2018). The main arguments of this approach are presented below:

Firstly, place is not only a physical entity. Rather, it is a combination of physical, social and symbolic attributes and is *experienced* by individuals rather than *prior* to individuals (Andrews et al., 2009). For example, when referring to one's home, the idea of 'my home' is not only a physical setting that contains my house, my furniture and decorations. The social and symbolic dimensions extend 'home' from its physical entity to the domain of autonomy, familiarity and self-identity (Coleman & Wiles, 2020; Dobner et al., 2016; Stones & Gullifer, 2016).

Secondly, the physical, social and symbolic meanings of place are constructed (Wiles, 2003). Individuals gain knowledge through daily interaction with a place. They 'sense the place' in a way that evokes a broad range of emotional reactions (Andrews et al., 2013). They 'make the geographical space meaningful' so that their own experience and identity are positioned in physical entities (van Hees et al., 2017).

Thirdly, individuals have different experiences of place, which are further shaped by their life experience, perceptions and social attributes such as gender, age and income (Barry et al., 2018; Wiles, 2003). For example, the increasing privatization of caring renders the caregiving burden disproportionately anchored on women's shoulder (Wiles, 2005). From this perspective, the constructed meaning of home for women is different from that for men (Barry et al., 2018).

The interpretivism and critical approach to residential decisions broaden the understanding of how ageing is *contextualized* rather than *located* in a certain geographical place (Andrews et al., 2013). Following this paradigm and argument,



upcoming scholars have shifted their attention from the objective factors that shape the residential decision to the subjective process of how the residential decision is co-constructed with the physical, social and symbolic place (Wiles et al., 2012). During the process, several facets/themes were intensively examined in previous studies.

2.3.1 Attachment to home and residential decisions

One of the most prominent research on ageing and place was conducted by Graham Rowles. In his innovative work *Prisoners of Space?: Exploring the Geographical Experience of Older People*, he investigates the way older adults interpret their own home from three dimensions: the geographical dimension (e.g., the building, street or store); the social dimension (e.g., the social relation established within a certain geographical location); and the biographical dimension (e.g., the lifelong memory within a certain geographical location). Older adults combined their past experience into the new circumstance in order to ‘make the physical surroundings meaningful and connected’ (Rowles, 1978). This process can initiate what he calls *place attachment* – a strong sentimental bonding between older adults and their home. It may also hinder them from leaving their home, even if advancing needs emerge (Chaudhury & Rowles., 2005; Gitlin, 2003).

Place attachment has proved to be an important influencer in deciding where to grow old. As long as home enables an autobiographical connection with one’s past and present, it is unlikely that older adults will relocate to another address (Chaudhury & Rowles., 2005; Rowles, 1978). Staying in one’s own home triggers a sense of familiarity and security (Wiles et al., 2012); objects and possessions provoke one’s reflection on one’s identity and autobiography (Coleman & Wiles, 2020). As age increases, the attachment to home rises, because of the ‘the sad and enjoyable moments and the memories of love one was framed around the home environment’ (Stones & Gullifer, 2016).

Place attachment is especially important for rural residents because they tend to reside in one place for a long period of time. It is provoked by emotional and cognitive binding to the rural landscape (Erickson et al., 2012) and is strengthened by



generational ties to farming and farmland (O’Callaghan & Warburton, 2017). Even when advanced needs emerge, farmers’ desire to stay put is still strong due to the generational place attachment to the rural landscape (Erickson et al., 2012).

2.3.2 Autonomy, identity and residential choices

Autonomy refers to the ability to decide where, when and how to conduct daily activities in one’s own place based on one’s own desire (Golant, 2011). Staying in one’s own home enables older adults to exercise their own preference and behaviour without complying with others (Stones & Gullifer, 2016). It is related to a sense of independence – an ideal prospect that may be undermined if moving to an institution (Söderberg et al., 2013; Vasara, 2015). Accordingly, it is unsurprising to note that older adults perceive institutionalization as an unwanted detriment to their autonomy and independence (Stones & Gullifer, 2016; Vrkljan et al., 2011).

Staying in one’s own home enables a positive sense of self. The home environment can accommodate a ‘segment of oneself’ – especially the vulnerable aspect that may be inappropriate to demonstrate in the ‘public space’ (Peace et al., 2011). It helps older adults to feel secured and resourceful (Wiles et al., 2012). The positive identity is strengthened by everyday and meaningful ‘tasks’, for example, walking, gardening, reading and cooking (Vrkljan et al., 2011). A qualitative research conducted in a degraded neighbourhood in the US suggested that everyday routine was the reason to get up in the morning (Finlay et al., 2020). When health decline reached a certain point and day-to-day activities were no longer possible, adapting to changes was painful and done with reluctance (Vrkljan et al., 2011).

In rural areas, older adults demonstrate a strong desire to balance their self-reliance and age-related loss (Cloutier-Fishera et al., 2000). This complex emotion reflects the expectation to live and the realization that life will soon come to an end (Ness et al., 2014). A qualitative study conducted in rural Australia suggested that rural residents struggled to maintain their identity as a ‘rural person’ – the self-esteem and self-efficacy narrative – while struggling to address place-related change (e.g., rural to urban immigrants) (Winterton & Warburton, 2012).



2.3.3 Social connection, power imbalance and the residential decisions

Growing old in one's own home signifies a continuous social bonding with familiar social members – family, friends and neighbours. Relocating to an institution, on the other hand, means a person has to renegotiated those social connections (Gardner, 2011; Matsumoto et al., 2016).

A growing body of literature has realized how a power imbalance may be infused into such a negotiation process – especially among older adults and their carers (Dalmer, 2019; Wiles, 2005). For example, overloading caring responsibility may impose children's power over older adults and force the adult children to convince their aged parents to move to an institution (Chen, 2017). Some studies have focused on gender role and residential decisions, revealing that women are more stressed and overwhelmed than men if facing caring responsibilities (Bryant, 2017; Dalmer, 2019). When deciding where to grow old and when to retire, it is possible that male farmers marginalize the voice of their wife (Downey et al., 2017).

In rural places, the negotiation of care – whether from one's children or neighbours – is largely determined by the geography. When education and employment opportunities shift from rural to urban areas, it is increasingly difficult for older adults to gain support from their children (Volckaert et al., 2020).

2.3.4 The ambivalence and cultural expectation

Some studies have tended to focus on the struggling and ambivalent experience during residential decision-making. For example, older adults tend to delay considering the residential issue until their health status has triggered the necessity and emergency of decision-making (Lofqvist et al., 2013). In extreme cases, some even refuse to discuss the topic or justify their delaying as being 'too healthy to consider the topic' or 'too uncertain to consider' (Lindquist et al., 2016).

The ambivalence may be partially attributed to the fact that growing old in one's own home has cultural appropriateness. In the Western context, growing old in one's



own home is a powerful and ideal narrative in contrast to institutionalization – a prospect that is pictured as dependence and uselessness (Barry et al., 2018). If a person has to consider the issue of moving, he/she can feel ‘deviant’ from the mainstream expectation of being autonomous and independent (Söderberg et al., 2013; Vasara, 2015).

Studies conducted in rural areas have revealed a similar pattern, that is, the proliferated idea of self-sufficiency could prevent older adults from voicing their needs. From their narrative, needs could mean ‘weak’ (Coldwell, 2010; Winterton & Warburton, 2011). Hence, the insistence on the mainstream idea of ‘hardworking’ and ‘laboriousness’ could prevent rural residents from seeking advanced support (O’Callaghan & Warburton, 2017).

2.3.5 Summary of this section

Compared with the first approach, the interpretivism approach pays more attention to the micro level of *experience* during AIP and residential decisions. This approach regards the residential choice as a process that is co-constructed between individuals and the physical, social and symbolic entity of ‘place’ – mainly one’s home. During the process individuals demonstrate their individuality and creativity (Pani-Harreman et al., 2020). This approach also includes a critical perspective, so that an individual’s struggling experience and its intertwined consequences with mainstream identity or social inequality are unfolded (Andrews et al., 2013).

2.4 Calling for new models/perspectives for residential-decisions

Taking together the perspectives/models and empirical evidence listed above, previous research regarding the residential decision is categorized into two types: (a) studies exploring factors influencing the residential decision; and (b) studies investigating the experience of AIP and residential decisions. The former line addresses the question of who would consider moving/staying under what circumstance; the latter line focuses on the experience and process of residential decision-making.



As argued before, the current literature on residential decision-making doesn't pay sufficient attention to the childless group, even though they constitute one of the groups most vulnerable to institutionalization (Ivanova & Dykstra, 2015). Moreover, most of the research elucidated above was conducted in the urban context in a Western society, which may not accommodate the voice and lived experience of the target group in this research – childless older adults in rural China. This research argues for a profound knowledge on this topic, which may contain and specify several characteristics:

(a) Considering the residential decision as a continuous process. The decision of where to grow old is a struggling process that involves information gathering, evaluating one's status quo and adjusting after relocating (Sussman & Orav-Lakaski, 2020). For a group facing declining health (e.g., the oldest old), the choice to stay put may simultaneously be accompanied by preparing for the 'next step' of moving (Lofqvist et al., 2013). Given that the opposite option to AIP is usually institutionalization in China, it is reasonable to believe that childless older adults will stay in the rural community while anticipating the upcoming prospect, albeit unwelcome, of moving. Hence, a continuous perspective on regarding the choice as a process rather than a momentary event is required.

(b) Cherishing the subjectivity and resilient characteristics of childless older adults. The existing literature on parental status and AIP/institutionalization tends to comprise longitudinal quantitative studies, which confirm the importance of parental status on the one hand, while rendering the complexity, subjectivity and resilient characteristics of the childless group invisible on the other (Medeiros et al., 2013). If considering the residential decision as a continuous process, as argued before, the evaluation, adaptation and struggling process theoretically co-exist (Granbom et al., 2014). Anthropology studies conducted in rural China and other contexts specified the effort to adapt among the childless group when age-related challenges emerged (Wenger, 2009; Zhang, 2007); empirical evidence has pointed to the need to cherish the way they 'position the ageing experience in place' and 'make the place meaningful' (van Hees et al., 2017). This research supports and extends this argument, calling for attention not only to the process of residential decision-making but also to the adaptation effort during the process.



(c) Being critically aware of childlessness. The term ‘critical awareness’ was introduced by Charmaz (2017), who deemed critical inquiry to be a transformative paradigm and perspective that seeks to expose, oppose and redress form of oppression, inequality and social injustice (Charmaz, 2017). It seeks to investigate social phenomena through the lens of feminist inquiry or class inquiry to reveal the ‘hidden oppression’ that might be concealed from grand discourse or social theory. This awareness is important in rural China due to the lifelong exposure to poverty and environmental constriction of childlessness. After all, staying in rural communities may cater to the preference of most older adults, but layers of disadvantage and oppression such as poverty, gender discrimination and cultural stigmatization are undeniable. This concern raises the need to pay attention to the ‘hidden voice’ of participants’ experience and possible social injustice during residential decision-making.

By reviewing the two lines of research on later-life residential decisions, this chapter argues for new perspectives/models to help us to understand childless older adults and their residential decisions in rural China. These new models could enable in-depth and critical understanding of this phenomenon, capturing individuals’ complexity, subjectivity and resilient characteristics during the process. They may sensitize the observation of this research and guide the analysis throughout the whole research practice. In the next chapter the *residential reasoning model*: its origins, main arguments and keywords are introduced to justify the choice of theoretical guideline of this research.



Chapter Three

The Residential Reasoning Model

The previous chapter argues that the current literature on residential decision-making in later life may not be sufficient to accommodate the voice and experience of the childless group in rural China. This chapter introduces a model of residential decision from environmental gerontology – the *residential normalcy model* and its subsequent development as the *residential reasoning* perspective (Granbom et al., 2014) in order to continue the discussion, this chapter firstly introduces the background and theoretical origin of this model, then illustrates its main arguments and key concepts. Following a rationale of fit between the model and this research, this chapter brings attention to three limitations of this residential reasoning model and the potential theoretical contribution of this research.

3.1 Calling for new research agenda on later-life residential decisions

Absorbing ideas from the positivism and interpretivism approach (the first and second line), the recent discussion on residential decision-making has facilitated two recognitions. Firstly, the decision of relocation/AIP and residential experience contains complex, sometimes even polarized, emotional reactions. Some older adults report a sense of regret over the possible and anticipatory institutionalization (Vasara, 2015), while others might justify such a negative emotion as ‘the need to be practical’ (Lofqvist et al., 2013). It has gradually become clear that the standard evaluation of ‘residential satisfaction’ or ‘life satisfaction’ – a commonly used assessment tool for predicting AIP preference – may become unreliable in a real-life scenario, since one’s experience regarding the current living situation and prospective relocation plan is complex and ambiguous (Golant, 2011). Moreover, older adults are more likely to report the ‘positive side’ of their residential and life situation than their younger counterparts, possibly due to the intention to show control and autonomy during a qualitative or quantitative



interview (Hillcoat-Nallétamby & Ogg, 2014).

Secondly, the residential experience is a lifelong process. Older adults may stay in one place for years while modifying their home environment to prepare for their anticipated age-related needs (Johansson et al., 2009); some may choose one place only temporarily because they understand that relocation is inevitable one day (Lofqvist et al., 2013). Expanding on the previous models on the temporal dimension, some studies have suggested that the relocation plan should not be constricted to middle or later life but ought to be investigated from a life course perspective (Granbom et al., 2014; Koss & Ekerdt, 2016).

These characteristics have sparked new exploration on residential decision-making in later life, marking the importance to deem the residential-decision process from a dynamic and complex perspective. The term *residential reasoning* process was initiated to address this call. Residential reasoning is defined as the subjective process of residential decision-making that covers both the decision-making and the adjustment process (Granbom et al., 2014). Building on knowledge from the *residential normalcy model* that was initiated by the gerontologist S.M. Golant in 2011, Granbom (2014) suggested that the new model could be used as a theoretical guideline to describe and explain why certain residential decisions are made, and how older adults may struggle to adapt to age-related challenges before settling on their final decision (Granbom et al., 2014). The next section starts by introducing the residential normalcy model – the theoretical origin of the residential reasoning model – then illustrates the expansion, argument and empirical findings of Granbom.

3.1.1 The residential normalcy model

Gerontologist Stephen M. Golant initiated the residential normalcy model to address the issue of ‘why using residential satisfaction as a predictor of AIP/institutionalization seemed unreliable in some circumstances’ (Golant, 2018). After years of engagement with what sociologists called the ‘vulnerable group’ – low-income older adults and older renters who are unable to find a stable living environment – he observed that older adults tended to demonstrate strong insistence on AIP over



institutionalization, regardless of how degraded and unsafe their environmental surroundings may appear (Golant, 2008). He started to revise the proliferated model of environmental gerontology (mostly Lawton's ecological model) by capturing the emotional complexity when staying put.

Golant (2011) distinguished two types of emotional reaction when staying in one's own home: residential mastery and the residential comfort experience. It is not until an individual falls into both (mastery and comfort) zones that they reach residential congruence – a status in which one's needs and goals are satisfied. The residential comfort zone can be further conceptualized as a sense of hassle-free pleasure and enjoyment from one's home, neighbourhood and community. Residential mastery refers to the extent to which older adults are able to demonstrate competence and control, that is, when they 'know their way around' or 'believe that tangible results in residential environments can be created (e.g., modifying the living room) (Golant, 2011; Heckhausen, 1997). Once older adults fall into the zone of comfort and mastery, they are more likely to choose AIP rather than relocating (Golant, 2015).

Notably, residential comfort and mastery are weighted differently. Older adults may feel comfortable with certain aspects of their residential environment (e.g., the climate) but dissatisfied with others (e.g., their annoying neighbour) (Golant, 2011). In other words, the residential normalcy model values the 'overall positive or negative' experience rather than one aspect of dissatisfaction or pleasure; it allows individuals to be simultaneously hassled and attached when staying put.

To achieve this congruence status, older adults have to *adapt* to residential and age-related difficulties (Golant, 2011). Building on knowledge from developmental psychology, Golant (2011, 2015) identified two broad strategies involved: the *assimilative strategy*, where individuals take action to 'solve the problem' (e.g., repairing a leaking roof); and the *accumulative strategy*, where individuals adjust their aspirations and emotions to maintain general wellness (e.g., lowering one's residential expectation) (Aldwin & Igarashi, 2012; Golant, 2011, 2015; Heckhausen, 1997).

Different individuals have different adaptation abilities and strategies, which may



lead to different residential-related consequences. Those who are unable to achieve residential congruence have little choice but to move to an institution for the right support (Golant, 2015; Granbom et al., 2014). In Golant's later publication (2015), he identified three factors that influence the ability to adapt: (a) individual resilience; (b) place resilience (e.g., supportive facilities or social relations); and (c) the role of influencers (Aldwin & Igarashi, 2012; Golant, 2015). Overall, the extent to which older adults are able to adapt to difficulties is shaped by their own physical, psychological and socio-economic resources, and the decision-making context (Wahl & Gerstorf, 2020).

Capturing the emotional complexity and endeavouring to adapt, the residential normalcy model provides theoretical prediction on whether older adults will choose to stay in their current dwellings as opposed to relocating to another address – usually to an institution (Golant, 2011). It concludes that, overall, older adults have to adapt to residential and age-related difficulties to balance their goals and needs. If the challenges are too prominent to adapt, relocation remains the 'last solution' for older adults to (re-)gain new congruence in a new environment (Golant, 2011, 2015).

3.1.2 *The residential reasoning model*

After the residential normalcy model had being empirically applied in several studies, mostly in a western, urban context (Koss & Ekerdt, 2016; Stafford, 2017), Granbom (2014) applied the term *residential reasoning process* to capture the simultaneous residential decision-making and adaptation process (Granbom et al., 2014). Supporting and extending the main argument of Golant's model, the residential reasoning model uses the phrase 'ongoing negotiation process' to summarize later-life residential decision-making. Below, the main arguments of the residential reasoning model are summarized (Granbom et al., 2014):

(a) It is problematic to separate the choice of staying put (AIP) and relocation into two research themes, as is usually done in residential decision-making literature (Bolin et al., 2006; Matlabi et al., 2016; Ness et al., 2014; Scocco et al., 2006). Partially due to the research tradition where 'nursing home admission and utilization' were separate



topics to ‘AIP’, and the fact that ageing in place has multiple explanations in different contexts, the two residential options in the literature are rarely combined in empirical studies. Granbom (2014) challenge this manifested separation and argue that if the residential decision was a continuous process, it would be problematic to separate the two choices into two pages while merely focusing on one group of individuals (usually the AIP group) in research practice (Granbom et al., 2014)

(b) It usually takes years of striving and adaptation to (re-)achieve residential normalcy, which is further constrained by previous life experience (Granbom et al., 2014). For example, older adults may foster a sense of attachment to the living situation to make the place ‘meaningful’, and the behaviour could become a skilful adaptation strategy for a lifetime.

3.1.3 Other related models exploring resilience or residential decisions

Apart from Golant and Granbom et al.’s contributions, there are several related models on environmental gerontology that involve adaptation behaviour of older adults during residential decision-making: for example, the Compensatory Secondary Control Model, which builds knowledge based on developmental psychology to address control loss when age-related challenges arise (Heckhausen, 1997). Empirical data confirm that older adults tend to reappraise their aspirations to balance age-related loss and pain during AIP (Stones & Gullifer, 2016).

Peace (2011) applied the term *optional recognition* to describe how later-life residential decisions are made under the constriction of one’s recognizable option, which is further susceptible to one’s identity, social resources and the positions a person occupies. Drawing qualitative data from the UK in urban areas, they suggested that long-time residence in one place fosters a place-related identity. Such identity may contribute to one’s overall wellness but it prevents older adults from seeking support elsewhere (Peace et al., 2011).

The Integrative Framework of Ageing Well, similarly, combines environmental-related behaviour with the subjective experience of belonging to explore the



environment-person interaction. According to the framework, the behaviour-driven agency and experience-driven belonging are equally important in enhancing the P-E congruence and general wellness (Wahl Iwarsson & Oswald, 2012).

These models confirm and extend the argument of ‘place is a process of creation’ and ‘later-life residential decision-making is a process to respond to such creation and construction’. It assumed that older adults are resilient enough to adapt to environmental challenges and to make changes (Iwarsson & Oswald, 2012). Regardless of the insightfulness of these models, however, this research found the residential reasoning model to be the most appropriate fit for the target group of childless older adults and the circumstances of rural China. The next section illustrates the rationale of model selection.

3.2 Residential reasoning model rationale

In combining the characteristics of childlessness in rural China and the available long-term residential options, this research selected the residential reasoning model as a theoretical guideline for this research for four reasons:

Firstly, the residential reasoning model is able to contain the experience of those who choose AIP and those who move to institutions on the same page. Unlike the Compensatory Secondary Control Model or Intergraded Model, this model provides a framework for understanding and predicting whether older adults will choose institutionalization over AIP to interrupt their status quo (Golant, 2011). Such understanding fits with the assumption that childless older adults may choose AIP while preparing to anticipatory move in the future.

Secondly, this model provides language to explore the emotional complexity of one’s own home. While the Ecology Model (referred to in the previous chapter) encompassed and simplified the individual incompetence as ‘driven factor of relocation’ (Perry et al., 2014), the residential reasoning perspective and the residential normalcy model offer a more in-depth and complex understanding of how residential surroundings are experienced and constructed by older adults. It is more appropriate



when applied to the group of childless older adults in rural China, who may feel positive and satisfied with one segment of rural living and dissatisfied with another (e.g., the lack of support) (Zhang, 2007).

Thirdly, individual resilience and an adaptive strategy are emphasized in the residential reasoning and normalcy model, which is one unneglectable facet of childlessness in rural China. Theoretically, lifelong independent living in a rural community fosters strong resistance to the environmental constrictions and age-related difficulties (Hsieh & Zhang, 2021). The residential reasoning model helps this research to identify not only the disadvantages of this group but also their attempts to adapt, which allow for policy intervention and changes of situations.

Finally, individual resilience and social inequality coexist; resilience may even be considered the extension and counterbalance of structural and intertwined inequality (Aldwin & Igarashi, 2012). The residential normalcy model specifies how individual coping strategies are shaped and constrained by place resources and one's own social attributes, such as gender, class, race, etc. (Golant, 2015). In the context of this research, as argued before, lifelong poverty and gender may intersect with one's resilient personality and characteristics, shaping residential decisions in a subtle (or possible unequal) way.

To summarize, the residential reasoning and normalcy model regards the residential decision in later life as an ongoing negotiation process that contains individual resilience and is constrained by individual, interpersonal and environmental characteristics. It provokes two phases, i.e. assessment (through the lens of mastery or comfort) and adaptation (assimilative or accommodate), to capture the complex and dynamic process of residential decisions, which makes the model applicable to childless older adults in rural China. In line with the key concepts of the residential reasoning model (residential assessment and adaptation), the next section reviews empirical evidence on these two models and their (theoretical and empirical) gaps. The reviewing order is as follows: (a) how older adults assess their residential environment through the lens of mastery or comfort; (b) how they adapt to age-related and residential-related difficulties; (c) how significant others influence their adaptation and decision-making.



The role of significant others can be considered the extension of individual adaptation on an interpersonal level (Golant, 2015).

3.3 Residential assessment

According to Golant's (2011, 2015) model, older adults assess their residential condition from two dimensions: the extent to which the residential environment is comfortable overall and the extent to which a person feels mastery (Golant, 2011). This argument is generally supported by empirical evidence, as reviewed before, which connects residential satisfaction, place attachment and a sense of autonomy to the choice of AIP (Black, 2008; Erickson et al., 2012; Sussman & Orav-Lakaski, 2020; Wiles et al., 2012). When taking into account the complexity of residential experience, however, the previous exploration is insufficient, and even 'prestigious'.

On the one hand, for example, older adults living in a degraded neighbourhood experience simultaneously insecurity, hassling, control and attachment. They 'negotiate around' to maintain a sense of competence, regardless of the substantial environmental challenges (Finlay et al., 2020). In constrictive rural place, older adults carefully estrange themselves from residential difficulties, prioritizing the necessities of life while distancing themselves from the hassling experience (Cai et al., 2012; Milbourne & Doheny, 2012). This situation may be prominent in the childless group in rural China, which are known for counting on their own physical strength to survive over a lifetime (Pang et al., 2004). One might draw the conclusion that childless older adults could achieve residential congruence when staying in a familiar and autonomous rural community.

However, what remains undeniable is that being a childless person and living in rural China comes with many difficulties. In other words, although they may appear autonomous and under control to some extent, the overall experience is hardly comfortable. For example, the geographical remoteness and insufficient transportation system have proved to be significant in shaping one's perception of the environment and health status (Wang et al., 2016). In rural China, where kinship ties are close, it is possible that childless older adults are further excluded and stigmatized under such a



social atmosphere (Zhang, 2007). Further, the ‘trickle-down effect’ growth of the economy and proliferated anti-poverty programme in rural China have altered rural poverty into a new prospect. Intensified by ageing and declining health, household poverty creates a group of adults who are ‘living above the poverty standard’ but ‘precarious in terms of health-related risks’ (Li et al., 2016). It is reasonable to assume that older adults without children ‘adapt’ but in an extremely uncertain and anxious way. It raises the need to move beyond the binary ‘positive’ or ‘negative’ status and to explore the residential assessment from a more complex perspective.

Altogether, this study firstly aims to gain an in-depth understanding of how childless older adults assess their residential environment through the lens of mastery and comfort.

3.4 Adaptation

Older adults actively engage in managing their experience and adapting to age-related challenges – whether by adjusting their emotional/cognitive system or taking action to make changes (Golant, 2011). Previous research identified several mental strategies during AIP, such as reframing goals (Stafford, 2017) and accepting difficulties (Milbourne & Doheny, 2012). Some older adults may foster a sense of security and familiarity with their residential environment to enhance place attachment (Granbom et al., 2014; Wiles et al., 2012), even though place attachment binds them to a certain place to prevent moving (Lehning et al., 2015). Behavioural strategies are also explored, including: changing daily activities (e.g., walking, going to church), using supportive devices (e.g., alarm system, wristwatch, wheelchair) and using home modifications to achieve desirable outcomes (Mackenzie et al., 2015; Stafford, 2017).

Several studies have documented how older adults in rural areas adapt to residential and age-related challenges. Self-sufficiency and self-reliance are frequently mentioned in the literature on rural ageing, as rural residents usually obtain food and other life materials through engaging daily with farmland (Pang et al., 2004; Winterton & Warburton, 2011). In materially limited rural places, older adults tend to lower their material expectation to maintain psychological wellness (Milbourne & Doheny, 2012).



Self-sufficiency has been proved to connect with positive identity, as Australian rural residents insist on the strong narrative of ‘rural masculinity’ and ‘heroic, male pioneer’ when age-related challenges raise their heads (O’Callaghan & Warburton, 2017). However, again, the exploration of adaptation effort remains insufficient when involving the childless group.

Two limitations of the existing literature account for this argument. Firstly, the adaptation process for the disadvantaged childless group is theoretically full of struggling, messiness and hesitation, which is rarely mentioned in the existing literature. In other words, it is an uneven process that encompassed the positive and negative feelings. When listing the adaptation efforts of a certain group, existing studies tend to assume that adaptation is powerful and effective, and leads to a desirable outcome (e.g., AIP) (Johansson et al., 2009; Stafford, 2017); the ‘uneven side’ of adaptation is mostly invisible. Further, there is a lack of in-depth exploration of the uneven side of adaptation, as it is theoretically related to one’s social attributes and structural constriction (Aldwin & Igarashi, 2012). The uneven side of adaptation may be individual, as older adults may differ in terms of their adaptational ability and resources (Golant, 2015). It may be structural because the limited formal and informal support in the rural context constrains one’s ability to seek formal support other than relying on oneself (Pang et al., 2004). Possibly, disadvantaged group was bounced back from the choice of staying put and relocating, hesitating by the possibility of leaving one’s home (Finlay et al., 2020; Lofqvist et al., 2013). Such a realization shifts the attention from the *what* question— what strategy older adults apply – to the *how* question, that is, how older adults adapt, struggle and negotiate in order to maintain congruence.

Relatedly, because there is a lack of attention paid to the uneven side during adaptation, the voices of ‘failed adaptation’ – according to the language of Golant (2011) – those who attempt to adapt but are too overwhelmed by the residential difficulties, and hence move to institutions, are neglected in the literature on residential decisions. This limitation is partially due to the historical separational research topic of AIP with institutionalization (also known as ‘nursing home placement’ or ‘facility administration’ in some literature). According to literature, institutionalization is related to, or triggered by, various indicators (e.g., health decline or support deficit) (Ahn et al., 2020; Ewen



et al., 2014; Gu et al., 2007; Wang et al., 2020); the adaptation experience is mainly found in the AIP group (Johansson et al., 2009; Koss & Ekerdt, 2016; Stafford, 2017; Wenger, 2009). This evidence is not sufficient to capture residential adaptation from a continuous perspective. If the residential decision is regarded as an ongoing negotiation process rather than a momentary event, as emphasized in the residential reasoning model, the voice of adaptation during AIP and adaptation before institutionalization should be combined onto the same page to gain in-depth insights (Granbom et al., 2014).

This study explores the adaptive strategies the childless group use during and before AIP. Specific attention is paid to the uneven side of adaptation; the voices from both the AIP and the institutionalization group, who have attempted to stay in the rural community but choose to relocate, are similarly explored.

3.5 The significant other(s)

Golant's initial publication didn't distinguish individual adaptation from interpersonal support and care (Golant, 2011); in his later work, the term 'significant influencer(s)' was introduced (Golant, 2015). Golant (2015) and his followers noticed that the residential decision may be relational. There might exist a significant other(s) during the residential decision process, who is not only an enabling/preventing factor but a joint party and discussant of the residential decision (Golant, 2015; Koss & Ekerdt, 2016).

Not surprisingly, for the parental group, the 'significant other(s)' of the residential decision is the person who provides long-term care and support to them: in most cases, one's spouse and children (Chen, 2017; Gu et al., 2007; Kendig et al., 2017). However, for the childless group, the concept needs further investigation. For example, in regard to the basic question of *who* provides support and care, previous findings were inconsistent. Some research referred to friends and extended kin (Schnettler & Wöhler, 2016) while others questioned the durability of informal support, especially during the health trajectory (Deindl & Brandt, 2017). For some childless older adults in rural China, spouse care is also absent because lifelong singleness constitutes a proportion of childlessness (Jin et al., 2013). As regards *the extent* to which the significant other(s)



may provide help, similarly, some have suggested that informal support is helpful in providing emotional and informational support (Cantor, 1979; Schnettler & Wöhler, 2016) while others underline the condition of needs. Childless older adults may not regard support as ‘supportive’ when it is provided at the wrong time or by the wrong person (Allen & Wiles, 2014). Overall, previous research conducted in the Western context confirmed that receiving support from an informal network is a complex and conditional issue. Although most research has proved that receiving support from an informal network may help compensate for the general lack of support (Albertini & Mencarini, 2014; Deindl & Brandt, 2017), it is not fair to conclude that such support and care carry enough weight to influence residential decisions (Ivanova & Dykstra, 2015).

Relatedly, paying attention to a significant other(s) creates the opportunity to explore how the residential decision has been negotiated between the significant other(s) and the care recipient, that is, the childless older adult. Previous research has explored how childless older adults struggle to balance the practical need to receive support and the positive identity of self-reliance during AIP (Volckaert et al., 2020; Wenger, 2009) – how support is governed by the principle of reciprocity (Furstenberg et al., 2020) and how older adults endeavour to avoid becoming burdensome to significant others when it comes to long-term care arrangements (Chen, 2015, 2017). In rural China, receiving support and care from an informal social network (e.g., friends, neighbours and extended kin) is dependent on one’s available resources. If the support provided does not have the anticipated benefits, it will result in a tricky interpersonal dynamic for the support recipient, sometimes rendering them morally subordinate to the support provider (Bian, 2018). This raises the question of how significant others influence residential decisions among the childless group, given the theoretically precarious nature of negotiating support and care.

Hence, this study attempts to understand who (if any) the significant other(s) in the context of residential decisions might be, and under what circumstances significant others play a role in the residential reasoning process. Likewise, both the AIP and the institutionalization group should be included in this exploration and discussion.



3.6 Summary of this chapter and rationale of research questions

This chapter and the previous chapter have reviewed the perspective/framework/empirical evidence on residential decision-making in later life. Following the two epistemological stances, two approaches identify different frameworks and perspectives to understand the residential decision regarding AIP and institutionalization. The more contemporary framework aligns two approaches as well as a critical lens, regarding residential choices as a manifested ongoing negotiation process. The term *residential reasoning* is applied to describe the process.

Residential reasoning combines the residential assessment, adaptation and decision-making process, arguing for a continuous, complex, resilient and critical perspective to understand the residential decision-making among older adults. According to this model, residential assessment is based on two broad experiences: comfort and mastery. Older adults adapt to a negative residential experience by using two broad strategies: assimilative and accommodative. The way residential assessment and adaptation are conducted are shaped by individual resources and structural characteristics. When older adults are able to achieve residential congruence – to fulfil their goals and needs while staying in their home – AIP is preferable to institutionalization.

To the best of our knowledge, this model has never been used in a rural context, nor has it been applied in the childless group. Based on the reviewed literature in the previous chapter and the basic argument of the residential reasoning model, two broad gaps have been identified in this chapter, with the second broad gap containing two subquestions.

(a) It is unknown how childless older adults assess their residential environment in a degraded rural context, given that rural living is hardly comfortable.

(b1) It is unclear how childless older adults adapt to residential difficulties. This research argues that the adaptation process among the childless group comes along with uneven experience and tensions – a type of ‘oppressive voice’ that is scantily explored



in the current literature.

(b2) When shifting the adaptation to the relational level, this research investigates the role of significant others and its impact on residential decisions. This research specifies who are the significant others (if applicable) and how they influence the residential decisions in regard to AIP or institutionalization.

Overall, this research aims to gain in-depth understanding of the residential reasoning process among childless older adults in rural China, exploring how certain residential decisions (in regard to AIP and institutionalization) are made and why. Following the key concepts of the residential reasoning model and identified gaps, the sixth chapter provides a description of how childless older adults assess their residential circumstances. In the seventh and eighth chapters, this research illustrates how childless older adults adapt to difficulties on an individual and relational level in order to make certain residential decisions.



Chapter Four

Methodology and Research Design

This research aims to gain in-depth understanding of how childless older adults select one residential choice over another (AIP versus institutionalization) and why. This chapter describes the research design of this study. Four areas are examined in this chapter, namely: (a) the rationale of qualitative inquiry; (b) the rationale of the constructivist grounded theory approach; (c) the research design in detail; and (d) method rigour and the trustworthiness of this research.

4.1 The paradigm debate and rationale of qualitative inquiry

Before moving to the research practice, this chapter clarifies the paradigm stance of the researcher and this research, which is related to the fundamental debate on ‘what counts as knowledge’. This debate influences all aspects of research practice: topic selection, data collection, the form of knowledge that is presented (Spencer et al., 2014). The term *paradigm*, defined as the world view through which knowledge is filtered (Kuhn, 1962), provides a starting point from which to respond to the debate.

Epistemologically, the positivism paradigm aims to foster a format of knowledge that is stored and presented in a set of testable hypotheses (Lincoln, 2010). The interpretivist paradigm, on the other hand, stores knowledge that is fat with the juice of human endeavour, human decisions, human contradictions, human emotion and fragility (Glaser & Strauss, 2017). Both forms of knowledge are useful to explain social phenomena. For example, as reviewed in previous literature, the positivism approach to residential decision-making tends to focus on who chooses to relocate and under what circumstances, while the interpretivism approach values the experience, the complexity and the power imbalance during the residential decision-making process (Pani-Harreman et al., 2020).



This research doesn't seek to commence these two paradigms. Instead, the first section seeks to clarify the ontological, epistemological and axiological stance of this research, raising awareness that research practice, no matter what inquiry method is selected or how research is conducted, is not suspended from the paradigm stance (Lincoln, 2010).

Ontology is the study of reality. In an interpretivist world view, reality is subjective and contextual, and hence ought to be understood within the context that a phenomenon/event/individual behaviour is embedded (Spencer et al., 2014). This research regards the residential decision experience as reality, which, in turn, is embedded in the rural context and shaped by the political, economic and social process. Rather than encompassing a matrix of factors that potentially influence the residential decision, this research aims to achieve for in-depth understanding of the decision-making behaviour, questioning how certain decisions are made and under what circumstances.

The epistemological stance addresses the issue of how reality is able to be known. For the interpretivist, it is noticeable that the researcher has no better claim on knowledge than the participants, as social actors themselves ascribe meaning to situations through shared, socially interpretive practice (Bryant & Charmaz, 2007). This research values the voice of the childless from their own perspective rather than from the perspective of a policymaker or other parties conducting needs assessment.

The axiological concern deals with the question of how values and assumptions of the researcher influence the research practice (Spencer et al., 2014). From an interpretivism stance, it is unrealistic to remove the subjective influence – or even biases – of the researcher, by whom the research process is constructed with participants. This research does not attempt to eliminate the researcher's subjective influence; rather, it assumes that the dynamic interaction between the researcher and participants is viewed as a lens through which to capture the contextualized and complicated experience of the participants, to question the taken-for-granted experience and assumptions of both participants and the researcher (Bryant & Charmaz, 2007; Spencer et al., 2014).



This research values the voice of participants and regards individual experience as a process of social, economic and political construction. This research claims to have no better knowledge on the residential decision process and residential perceptions than participants, since they carry on with their daily life and make their own decisions. These characteristics match the interpretivism paradigm, which has enabled a more appropriate qualitative inquiry for this research (Creswell et al., 2007). Moreover, as argued in previous chapters, the residential experience of the childless is scarcely investigated in the literature. This paves the way for qualitative inquiry rather than quantitative verification (Lincoln, 2010).

4.2 The rationale of the constructivist grounded theory approach

Standing with the intuitivism inquiry, this section justifies the choice of the grounded theory approach and its constructivist design. Overall, the selection of the grounded theory approach was determined by the purpose of ‘investigating the residential decision process among childless older adults’. Although it is debatable whether a different qualitative design (e.g., case study, grounded theory, narrative approach) is able to apply to the same topic, this research choose grounded theory approach because it was initiated to explore process that abstracted from people (different childless older adults), place and time’ (Charmaz, 2014).

Within the grounded theory approach, the choice of a constructivist design is influenced by epistemological concerns. The aim of the grounded theory method was to challenge the prevailing methodological discourse of quantitative orthodoxy, endeavouring to dequantify the social world while maintaining the systemic and scientific characteristics of quantitative research (Glaser, 2002). For Glaser and Strauss (1967) – the originators of the grounded theory method – the innovative publication of *The Discovery of Grounded Theory* attempted to avoid, but ironically fell into, the positivism paradigm: to generate new, systemic and scientific methodology from analysing dequantified data (Bryant, 2017; Glaser & Strauss, 2000). Regardless of the subsequent criticism from grounded theorists, Glaser (2002) stands for the positivist position of the GTM, suggesting that ‘the production of grounded theory is an



abstraction from time, place and people’ (Glaser, 2002).

The constructivism grounded theory (CGT) was originated from the grounded theory method (GTM) from its paradigm stance and research practice, although both inquiries aim to generate a conceptualized theory/explanation of a process, action or interaction (Creswell et al., 2007). It raised three challenges to the classic grounded theory approach, which also responded to the method concern of this research.

4.2.1 Inductive or abductive?

Grounded theorists who follow Glaser’s methodology inquiry usually apply *inductive* logic in conducting research: from life world to theories, from empirical data to abstract concepts (Bryant, 2017). In the ideal situation, grounded theorists would ‘produce new knowledge from the field that is rarely explored’, ‘preventing any pre-assumption from the existing knowledge and literature staining the research practice (Timmermans & Tavory, 2012). Therefore, most researchers have justified the selection of the GTM as, ‘limited previous research has been conducted on this topic’.

This ‘blandness’ before entering the field was questioned at a time when most publication were digitalized (Bryant, 2017). A growing number of grounded theorist argue that the claim of ‘no existing research in this area’ is becoming increasingly unconvincing due to the rapid expansion of online libraries and qualitative studies in recent decades (Charmaz, 2014). Moreover, given the systematic academic training and increasingly accessible publications and information in contemporary society, it is gradually becoming increasingly difficult to claim that ‘the researcher was not stained by pre-assumptions and his/her own knowledge’, and that ‘this research was conducted based on inductive logic’.

The constructivist approach was, therefore, initiated to respond to this challenge. Charmaz (2014) encouraged abductive logic rather than inductive logic to conduct qualitative research, so that the researcher could be ‘sensitized by existing concepts and framework’ before entering the field, ‘bouncing back from literature to newly emerged findings’ (Charmaz, 2014). In this research, because the topic of residential decision-



making has been established for over two decades (Weaver et al., 2020). The experience of residential decision-making is hardly claimed to be ‘unexplored’ or ‘unknown’. What remains unexplored is the residential decision-making experience of the childless group, rather than residential decision-making itself. Hence, the residential reasoning model was selected to *sensitize* our understanding, to provide theoretical lens and insights for data analysis (Charmaz, 2014).

4.2.2 *Formal theory or substantive theory?*

Glaser and Strauss used the GTM to generate what they’ve labelled *formal theory* – the form of theory that is conceptualized from abstraction of time, space and people (Bryant & Charmaz, 2007). However, the GTM theory produced was criticized for being ‘too descriptive’ and as ‘simplifying individual experience into scientific linear correlation’ (Charmaz, 2008). In a constructivist approach, the definition of theory is contextual: it seeks to provide contextual and in-depth insightfulness on a specific phenomenon – ‘to explore what happened to whom in what circumstance’. Charmaz (2014) named it the *substantive theory* (Charmaz, 2014).

In this research, the contextual and historical character of childlessness renders the experience of participants diversified. The complex experience of self-reliance, for example, are different from what urban residents refer as ‘self-reliance’, because urban residents are less likely to reliance on one’s physical capacity for farming for living material. Such intensive labour force involvement of rural residents requires a contextualized and conditional – the in-depth understanding of what happen to whom in what circumstances for this research.

4.2.3 *Value free or critical awareness?*

To enhance its scientific and systematic characteristics, one premise that the classic GTM inquiry insisted on was the role of a (relatively) distant, value-free researcher (Glaser & Strauss, 2017). The GTM regards data collection, analysis and representation as a mutual process, which is criticized as if the ‘data were *given* by participants’ (Charmaz, 2008). In CGT inquiry, conversely, the influence of the researcher’s own



positionality, value system and pre-assumption ought not to be eliminated but needs to be carefully examined (Charmaz, 2020). The constructivism approach regards the research practice as ‘co-constructed’ by researchers and participants, while the ‘truth’ – using the positivism term – results from ongoing negotiation (Charmaz, 2014).

Charmaz’s later publication (2017) argues that the CGT could and should enhance the ‘critical awareness’ of the researcher through examining his/her social positions and the possible privileges that come along with his/her position. It involves the question of whether research is able to notice the power imbalance between researcher and participants when recruiting them, interpreting their experience and presenting their life experience as ‘produced knowledge’ (Charmaz, 2017). An examination of the researcher’s positionality and a reflective box are illustrated in the last section of this chapter. Overall, this research understands the need to check the researcher’s privilege during research process.

4.2.4 Rationale of method selection

To summarize, the constructivist grounded theory approach (CGT) inquiry is applicable for this research due to its purpose, logic and epistemological appropriateness. Specifically, there are four characteristics of the constructivist approach that especially benefit this research.

Firstly, the grounded theory design helps this research to reach the level of abstraction from people, time and space, ‘to generate conceptualized theory/perspective/concept from social phenomena’ (Creswell et al., 2007; Glaser, 2002). It allows this research to capture the residential decision-making process while having a conceptualized explanation of it.

Secondly, unlike the classic grounded theory approach, the constructivist grounded theory approach encourages a ‘dialogue’ between an existing research topic and a new context or new group (Charmaz, 2014). The residential normalcy model, as argued on Chapter Three, are deemed suitable to guild the analysis of this research, as it underlined the dynamic process, emotional complexity and individual resilience during



residential decision-making. The constructivism approach enabled an ‘abductive logic’ from residential reasoning model to newly collected data and analysis.

Thirdly, this research does not intend to de-contextualize participant’s experience by encompassing participants’ experience into a linear correlation. Instead, China’s rural context remains a significant aspect to understand the lived experience and decision-making process among childless older adults. The constructivism grounded theory approach encouraged an in-depth understanding of participant’s experience, with the social, cultural and historical characteristics of rural China being taken into consideration. In sum, the constructivism approach could prevent the participant’s experience of being ‘too descriptive’ as the classic grounded theory approach (Charmaz, 2014).

Finally, the critical awareness provided by the constructivist grounded theory approach helps this research to examine her positionality and pay attention to the ‘hidden, unheard and oppressive voices’ of participants. When considering the research process as co-constructed by participants and researcher, I am aware that the interpretation and presentation of participant’s narrative may be influenced by my own values, knowledge background and my social position. This realization did not aim to undermine the validation of data analysis but to remind the reader that my reflection should be also taken into consideration when reading findings and implications of this research. Further analysis and criticism are required in future related studies.

Based on the principle of the classic grounded theory approach, Charmaz (2008) provoked several extensions from grounded theorists in contemporary academia: (a) treating the research process as a construction; (b) scrutinizing the research process; (c) improving methodological strategies; and (d) collecting sufficient data (Charmaz, 2008). This research supports this argument. The selection of the constructivist grounded theory approach enabled the sensitizing concept from the beginning, providing the method guideline during research and encouraging the researcher’s reflective notes for further implications. It matches the main purpose of this research, that is, to gain in-depth understanding of the residential reasoning process from the perspective of the childless.



4.3 Overview of research process

This section provides detail on how this research has been conducted. In accordance with the CGT guidelines, the research process was gradual and spiral. Data collection and analysis were conducted simultaneously; previous analysis results guided the next stage of data collection and analysis (Charmaz, 2014). The researcher bounced back from the field to the literature, from empirical data to the initiated framework, to seek theoretical congruence (Holton, 2007). Therefore, this research is divided into four stages: (a) initial data selection and finding; (b) focused coding; (c) theoretical data selection and theoretical coding; (d) verification. Before introducing detail on each stage, this section first describes the participation selection, the overall research design and the interview guidelines.

4.3.1 Participant selection: inclusion/exclusion criteria, research site and accessibility

This research seeks to explore the residential reasoning process among childless older adults in rural China. Participants who are:

(a) aged over 60;

(b) registered as agricultural Hukou;

(c) registered as rural Three-no older adults (older adults who have no source of income, except for the government assistance scheme, no legitimate caregiver and no ability to work) were recruited for this research.

Each village committee has a registration system to record those ‘in need of welfare assistance’, namely the Assistance for Extremely Poor Household Scheme. Household under poverty line, village orphans or childless older adults are registered under the system. The childless older adults are registered as Three-no Older Adults in official document. Researcher of this study relay upon the registration system on the village committee’s office to identify and to reach for childless participants.



The criteria for Three-no are not only limited to childless status, but childlessness is the pre-condition to fall into Three-no category. In rural area in most of circumstances, residents who are childless are unlikely to receive any cash transfer since instrumental support from immigrant children are the primary source of income for most of rural older adults (Cai et al., 2012). The village committee considered this situation as ‘having no source of income’, despite some childless older adults were part-time employed by villagers and local factory (see, Chapter Five). Similarly, rural residents who are aged over 60 are hardly finding full-time occupation, which was counted as ‘having no ability to work’ from standard of villager committee. Some childless older adults may engage with farming activities to sustain daily life (see, Chapter Six), yet this situation is not considered ‘having the ability to work’. In summary, only childless residents who are aged over 60 are able to be registered as Three-no older adults. Individual who has biological children – no matter how economically constrained, physically disabled or conflictive they are with their children, the Three-no scheme and its attached welfare support are not available for them.

Interview data were collected in rural Yunnan province from February 2019 to March 2021. To maximize comparison, residential facilities and villages were selected according to their economic conditions and geographical remoteness (Miles & Huberman, 1994). Two residential facilities were selected from a town representing China’s national standard of poverty, and two from an economically privileged town. Villages near the township institution were selected afterwards, so that individuals living in a village were qualified to apply for the township institution. Five villages were selected in this research.

The selection of Yunnan province was informed by two principles: (a) the sample should be likely to generate rich information; (b) the sampling process should be feasible (Curtis et al., 2000). Yunnan is in southwest mountainous area of China and have a population of 48.3 million (as of 2018). The Gross Domestic Product (GDP) of Yunnan was RMB 2,452.1 billion (USD 277 billion) in 2020, ranked 23 out of 32 provinces nationally (The GDF of China was RMB 101 trillion, USD 14.7 trillion at 2020) (Bureau of Statistics of Yunnan Province, 2020). The mountainous landscape, the frequent natural disaster and poor infrastructure of southwest provinces creates a



significant rural poor in southwest area in China. As Liu's (2017) study suggested, there were 22 counties that located in southwest and northwest provinces had more than 2 millions residents classified as rural poor (Liu et al., 2017). The Three-no older adults living in Yunnan province are massive, with 0.11 millions of them living in rural Yunnan at 2019, constituting 2% of the whole Three-no population nationally (4.29 million in total) (National Bureau of Statistics of China., 2019). This enables this research to gain rich information on participant's experience.

This research also considered the feasibility of data collection in Yunnan province. The personal connection with township government and village cadres allows me to approach to the registration system of social assistance in township governmental office. The administrative staff and village cadre in selected sites (four towns) were supportive during the whole process of data collection and interview.

Noticeably, one might question the generalization of findings from Yunnan province to other places – such as northern plate are of China. This research argues for the analytic generalization instead of statistical generalization of findings for validation. The analytic generalization refers to the extend qualitative findings can provide the opportunity to exam, extend and revise existing theories. It sought to generate insights and new understanding of certain social phenomena, not the grand social theory that are statistically generalizable to all groups (Curtis et al., 2000). Along with this stance, this research suggest that data collected from Yunnan province allows us to have some insights on the experience of childless older adults on their residential decisions, even the statistical generalization of such finding may require further analysis. More detail would be elaborated on Chapter Nine.

I (SC) relied upon local authorities to recruit participants. After making connection with the village cadre, the village cadre or anti-poverty work team members accompanied SC to identify the childless participants in the registration system, visiting each participant's house or institution. The researcher introduced the research purpose and ethical issues, then the floor was open for conversation. If childless older adults were inarticulate or having mental conditions to answer all interview questions, their caregivers (usually the significant other(s)) were invited to the interview process to



supplement information of childless older adults. All interviewees participated voluntarily.

A total of 27 participants were interviewed for this study. Seven childless older adults were recruited on follow-up interview for *theoretical sampling* (Charmaz, 2014). Six informants (e.g., village cadre or institution staff members) were recruited for *triangulation*. Four participants were interviewed in the last stage for *member checking* (Morse et al., 2002). The next table presents the demographic features of the participants:

Table 4.1
Demographic features of participants

	Community-dwellers	Institution-dwellers	Total
Gender			
Male	13	6	19
Female	4	4	8
Age			
60-75	13	5	18
Over 76	4	5	9
Marital status			
Never married	12	3	15
Married/divorced	5	7	12
Childlessness status			
Loss of children	1	3	4
Never had children	16	7	23
Number of participants	17	10	27

One challenge raised from interview process was that some childless older adults were too cognitively impaired to provide information. This research employed interview proxies – the close family members of participants to answer the research

questions. There were four participants among the 27 childless participants who were cognitively disabled, so that their significant other(s) – the primary caregivers such as siblings, nephews and niece were interviewed instead. This situation was discussed by gerontologist and other social scientist when interviewing inarticulate participants, such as uneducated person or adolescents who had cognitive impairment (Booth & Booth, 1996; Wenger, 2001). The information validity from interview proxies could be enhanced if the information involved with fact (e.g. when did X get up and how did X conducted their daily life?), not subjective experience (e.g. does X feel lonely?) (Wenger, 2001). Therefore, this research cited narratives from the significant other(s) when it referred to fact (e.g. ‘I have been taking care of X for 5 years’) (see Chapter Eight). When involved with subjective experience (e.g. how a person negotiated with *ku* experience), this research were mainly relay on voices from childless participants (see Chapter Six and Seven).

4.3.2 Description of research design

The constructivist grounded theory approach underlines the congruence between empirical data and the initiated framework, encourage the research process to ‘bounce back’ from newly emerged analysis results and further data collection. Hence, the research focus, sampling strategy and analysis focus of each stage was informed and guided by the results from the previous stage (Charmaz, 2014). The next table shows the overall research design for each stage.

Table 4.2

Description of research design

Data collection	Data Analysis	Duration	Outcome
Research aims (stage 1): To understand the reasons for making certain residential choices (AIP or institutionalization)			
Convenience sampling	Open coding and initial coding	2019.03 – 2019.06	6 participants were recruited
Discussion and peer debriefing meeting			
Research aims (stage 2) : To gain in-depth understanding of the residential decision-making experience, which includes:			



(a) factors/characteristics enabling/preventing the residential choice;			
(b) resilient characteristics during decision-making;			
(c) significant other(s) that might influence the decision.			
Convenience sampling	Focused coding	2019.06 – 2020.05	14 participants were recruited; 5 informants were recruited
Discussion and peer debriefing meeting			
Research aims (stage 3): To supplement for properties and gaps of the initial theoretical framework that initiated from previous stage			
(a) residential assessment and experience;			
(b) adaptation and struggling;			
(c) how the significant other (s) influence residential decision.			
Theoretical sampling and follow-up interview	Axil coding and theoretical coding	2020.05 – 2021.3	7 participants were recruited; 7 follow-up interviews were conducted
Discussion and peer debriefing meeting			
Draft writing			
Research aims (stage 4): To verify findings			
Triangulation and member checking	Theoretical coding	2020.03 – 04	4 follow-up interviews were conducted; 1 informant recruited

4.3.3 Interview guideline

Qualitative data were collected from semi-structured face-to-face interviews (Charmaz, 2014). Following the methodological guidelines, the research guidelines were continuously adjusted based on the analysis results from the previous stage. The interview guidelines were as follows, and the interview guideline of Chinese version were on Appendix II.

Table 4.3

Interview guideline

	Research aims	Outline
Stage 1	General description	1. Could you describe your current life? 2. Could you tell the story of relocating/AIP decision-making? 3. What's your future plan?
Stage 2	In-depth understanding	1. How do you understand your current living situation? Are there any difficulties? How did you/will you solve them? 2. Could you tell the story of relocating/AIP decision-making? If you were going to summarize the reason for staying/relocation, how would you summarize it? 3. How do you understand <i>ku</i> ? 4. Could you describe your social network? 5. What's your future plan?
Stage 3	Theoretical understanding	1. How do you understand your current living situation? Are there any difficulties? How did you/will you solve them? 2. Could you please describe your childhood? Maybe start from your education. 3. Could you describe your marriage/family life? 4. Could you describe your house relocation history? 5. Is there anything particularly unforgettable you'd like to talk about? (e.g., the Collectivism Period, the Liberation) 6. If you were going to describe you, as a person, what words/sentence would you use? 7. Could you tell the story of relocating/AIP decision-making? 8. How do you understand the term <i>ku</i> ? 9. Could you describe your significant other(s)? 10. What's your future plan?
	Research aims	Outline

Stage 4	Verification	This is what we've learned from the previous interview. Does our analysis match your experience? Is there anything you'd like to supplement or revise?
---------	--------------	--

The interview guidelines demonstrated that the researcher's understanding of the residential decision has gradually shifted from a superficial description to an in-depth exploration of individuals' experiences, perceptions and emotions. Key experience such as *ku* and having someone at home gradually emerged from previous analysis.

What needs to be clarified is the story-like interview guideline in stage 3, during which participants were encouraged to describe their significant life events before discussing their residential decisions and plans. This interview technique helped to encourage participants, mostly low-educated farmers, to articulate their past life stories and experiences (Booth & Booth, 1996; Jaber & Holstein, 2001). Participants were comfortable with questions that they were familiar with (e.g., Did you go to primary school? How many people are there in your family?) (Phoenix & Sparkes, 2009).

Furthermore, topics such as 'whether you have someone at home to take care of you' could be 'difficult to illustrate' for some participants. Some were unsure about the prospect of 'having someone', so they tended to avoid mentioning it; some may simply have forgotten to mention it. When reviewing interpersonal relations from an autobiographical perspective, participants were encouraged to remember that they may (or may not) have 'a supportive figure' to provide help (Adriansen, 2012).

4.3.4 *Ethics approval*

This research was approved by the Human Research Ethics Committee at the University of Hong Kong (EA1901040). All interviews are based on principle of voluntariness, confidentiality and no harm. The ethics approval document was in Appendix I.

4.3.5 *Research process in detail*



This section elucidates the research purpose, sampling strategy and analysis results for each stage in detail. Because the design of each stage (except for the first stage) relies upon the analysis results from previous research, the sampling strategy and observation are shown combined.

4.3.5.1 Initial sampling and initial coding (stage 1)

The first stage attempted to reveal ‘why a certain residential choice was made’. Therefore, the sampling process was based on the principle of convenience (Charmaz, 2014). The researcher relied upon his/her social connections to recruit childless older adults who met the inclusion/exclusion criteria and were available. A total of six participants, including three community dwellers and three institution dwellers, were recruited from 2019.03 to 2019.06.

Interview data were recorded, transcribed and analysed multiple times. Four themes emerged during the initial coding process: *Ku*, *Ageing and Incompetence*, *Maintenance*, *Significant others*. This research realized that:

1) Two participants made the residential choice based on the perception of *ku* – conducting farming and farm-related activities. It was a complex experience combining both residential mastery and comfort aspects. This term seemed important to summarize participant’s experience, thus triggered our interests for further investigation.

2) The chance to *ku* – to conduct farming activities was influenced by advancing age and one’s capacity. When age-related difficulties (e.g., stroke, high blood pressure) emerged, a person’s capacity to continue *ku* was undermined.

3) No matter what difficulties they encountered, participants struggled to maintain the current situation (whether it was AIP or the life situation in an institution) for as long as possible.

4) One participant chose to AIP because she received support from a significant other (her daughter-in-law).



Overall, this stage aimed to make general sense of participants' residential decision-making experience (Charmaz, 2014). Analysis results suggested that the residential decision was related to one's individual (physical) and interpersonal resources. The term '*ku*' (which emerged in the second interview) and having someone at home (which emerged in the fourth interview) seemed important in explaining participants' experiences and decisions.

4.3.5.2 Initial sampling and focused coding (stage 2)

After the first stage of sampling and analysing, the researcher went back to HKU to seek method adjustment. A peer debriefing meeting encouraged him/her to focus on the physical and interpersonal level of competence and its relationship with residential decisions.

In the second stage of the research practice, this research aimed to gain a comprehensive understanding of the residential decision-making issue. The sampling strategy in this stage was also based on the convincing principle, yet the researcher attempted to balance the demographic features (gender and age) to maximize diversity (Charmaz, 2014). A total of 14 participants were selected during this stage, including five institution dwellers and nine community dwellers. Four out of the nine community dwellers were mentally disabled, so their caregiver was interviewed instead. Three staff members from institutions and two village cadres were interviewed to verify the trustworthiness of participants' narrative (Gioia et al., 2013).

Data analysis was based on the results from the previous stage. A focused coding strategy was conducted to capture the intensive and repeated emerged experience (Charmaz, 2014). Interview data were constantly compared. Newly emerged codes were renamed, re-examined and merged into previous codes. Initial codes generated from the previous stage turned into initial themes, such as: (a) the negotiation process; (b) *ku* as a complicated experience; (c) resilience and adaptation; (d) interpersonal resilience and complication; and (e) a sense of boundary. Specifically, this research revealed that:



1) The residential decision is an ongoing process that contains struggles and tensions. Childless older adults demonstrated resilient characteristics during the process, constrained by structural factors, however. Their material poverty, declining health and ageing constantly shadowed one's assessment of the current residential circumstances.

2) *Ku* in narrative form has two layers of meanings: (a) *ku* as a verb to describe the behaviour of 'doing stuff to make a living in a rural area' (e.g., farming, raising chickens, doing housework); (b) *ku* as an adjective to describe the emotional status of 'doing stuff', that is, stressful and suffering. It represented an implied emotional reaction where the residential mastery and comfortable experience were polarized. Childless older adults cherished the ability to *ku* – to farm to survive; the overall experience of rural living, on the other hand, was hardly comfortable.

3) Childless older adults applied several strategies to adapt to the uncomfortable experience: (a) they reappraised the discomfort aspect of *ku* as being unimportant; (b) they routinized and focused on day-to-day *ku*; (c) they fostered a positive identity of toughness. On the other hand, however, the adaption was also constrained by material, interpersonal and environmental disadvantages.

4) Receiving support from a supportive network alleviated the discomfort of *ku*. However, it was not directly attributed to AIP. Some participants received support from multiple sources but chose to relocate; some just assumed they would 'have someone at home' and chose to AIP. Similarly, to the perception of *ku*, the status of 'having someone at home' was also complicated and required negotiation.

5) The chance of receiving support from an informal network was constrained by a proliferated idea that 'everyone should take care of their own core family'. When AIP, both childless older adults and their caregiver tended to underline the (moral and instrumental) gain of having such caring relationship – what has been coded 'bounded care'.

Overall, this research identified two broad situations when a residential decision has been made: when a person can endure *ku* (conducting farming and farm-related



activities) and when they have someone at home. Both enduring *ku* and interpersonal support were susceptible to individual and structural constrictions, which rendered AIP ambivalent. This research assumed that, from the perspective of residential mastery and comfort, the mastery experience for participants might be especially ambivalent.

4.3.5.3 Theoretical sampling, axial coding and theoretical coding (stage 3)

This stage sought to formulate the initial theoretical framework that linked all the initial or revised themes/categories into a coherent framework (Charmaz, 2014). Two steps were specified in this stage.

Firstly, before collecting more samples, axial coding strategy was applied to link the initial themes/categories to achieve the initial theoretical coherence (Charmaz, 2014). Participants were intensively compared in terms of the mastery experience, *ku* experience and their interpersonal relationship. The condition (whether a person endures *ku*/has someone at home), reaction/behaviour (how this person experiences their status) and consequence (whether this person chooses to AIP) of each theme/category were pinpointed (Holton, 2007). If a person specified endurable *ku*, this research explored in detail how they adapted and struggled to endure *ku*. Those who perceived endurable *ku* was compared with cases where *ku* was unendurable, identifying their similarities and differences in terms of adaptation. A similar pattern was applied to the theme of ‘having someone at home’. The next table summarizes the research focus during the axial coding process.

Table 4.4
Research focus during axial coding process

Research focus: What’s the experience of residential congruence?	
Initial theme	Subthemes
Residential mastery and ambivalence	(a) The residential mastery experience was ambiguous (b) Residential mastery was expressed on farming, reaching out for support and the negotiating process with the formal sector (c) Residential mastery experience was gendered; women were



	more likely than men to impose their mastery on ‘others’ (e.g., nephew/niece)
Research focus: How participants adapt to residential difficulties	
Enduring <i>ku</i> and residential decisions	(a) Participants endure <i>ku</i> for as long as possible when AIP (b) The adaptation contains behavioural, psychological and existential levels (c) For those who move to institutions, enduring <i>ku</i> appeared to be unlikely (due to increasing age or physical disability)
Research focus: how the significant other(s) influence the residential decision	
Having someone at home and residential decisions	(a) On an interpersonal level, ‘having someone at home’ would substantively increase the confidence of choosing to AIP (b) ‘Having someone at home’ was an ongoing negotiation process (c) A person might refer to instrumental benefit, relational closeness or family responsibility to gain support from ‘someone’

In general, axial coding enabled this research to organize initial codes and themes into an initial theoretical framework, which contained: (a) residential mastery and ambivalence; (b) enduring *ku* as individual adaptation and its struggling side; (c) having someone at home and its complexities. Each theme responded to at least one segment of the residential reasoning model; all themes constituted the resilient characteristic and struggling side during reasoning. A central theme that related to all themes/categorizes emerged (*ambivalent mastery*). Overall, this research realized that childless older adults assess and adapt to difficulties based on the experience of residential mastery. A sense of mastery enabled them to choose AIP over institutionalization; on the other hand, however, such an experience came with feelings of ambivalence, discomfort and constriction.

In realizing the mastery and struggling side of reasoning, a theoretical sampling process was conducted. Theoretical sampling helped the researcher to clarify the properties, gaps and boundaries of each theme/category, validating the initially emerged framework with newly collected data (Charmaz, 2014). Participants were recruited to maximize the comparison in their demographic features (especially with variables of



gender); a follow-up interview was conducted to clarify and supplement details (Charmaz, 2014). Seven new samples and seven follow-up interviewees were recruited to supplement the themes/categories from 2020.5. to 2021.3.

To integrate all codes, themes and categories into a coherent theoretical framework, participants' narrative was re-examined according to the following questions:

(a) How do participants assess their current residential situation by using the concept of ambivalent mastery?

(b) How do participants struggle to endure *ku*? Under what condition do they apply a different adaptation strategy (behavioural, physiological or existential strategy)? What's the (residential-related) consequence of adaptation?

(c) How do the significant other(s) influence the residential decisions? How do participants negotiate for long-term care with the significant other(s)? Which strategy (bargaining, referring to relational closeness or continuing family relationship) do participants use to gain support? What's the (relational-related) consequence of such negotiation?

To help the reader gain a comprehensive understanding of participants' experience, key life events/trajectory was coded under the following sections: childhood memory, career path, family relationship and the reason for being childless.

Overall, this research realized that childless older adults cherished the mastery experience more than the comfort experience during residential assessment, although they were ambivalent about the feelings of mastery. Due to the individual, interpersonal disadvantage of childlessness in a rural area, the residential reasoning process was a struggle and full of messiness.

4.3.5.4 Rationale of theoretical sensitizing and theoretical saturation



This research applied the residential reasoning model as starting point to sensitize our data collection and analysis. This approach needs to be further justified. One of the debates raised from grounded theory approach (as argued on section 4.2.1) is, ‘to what extent the grounded theorist could avoid forcing data into existing literature while taking advantage of literature to inform research practice’ (Holton, 2007). According to the classic grounded theory approach – the approach that initiated by Glaser and Strauss (1967), any intention to contaminate the inductive logic from existing literature may undermine trustworthiness of qualitative research, rendering the finding ‘biased’ from pre-existing ideas, concepts and theories (Glaser, 2002). From the constructivism stance, however, it is unrealistic to estrange the researcher from their background knowledge, their preconceived concepts and ideas (Charmaz, 2008).

Thornberg (2012) initiated the idea of informed grounded theory to respond to this debate, suggesting that existing literature could be used as sensitizing concept to guild the research practice. ‘Theories are used, not to mechanically derive a hypothesis to test any assumption, but as a source of inspiration, seeing and interpretation social phenomena’ (Thornberg, 2012). Hence, the existing theories are always opening for elaboration, revising or criticizing when new findings emerged (Charmaz, 2008). The residential reasoning modal was the theoretical lens to guild data collection and analysis of this research; it sought to stimulate new questions (e.g., how residential mastery become ambivalent in rural area) and new findings. It enabled the researcher to make constant comparison between concepts such as mastery, comfortable and structural disadvantage with participant’s true voices and experiences.

What need to be underscored is that, using sensitizing concept come along with risks such as forcing empirical data into existing concepts or directing researcher’s attention away from possibly relevant aspect of data (Charmaz, 2008; Holton, 2007). However, these challenges could be addressed. Thornberg (2012) initiated five strategies to avoid data forcing while taking advantage of existing literature while, including (a) theoretical criticism, (b) theoretical pluralism, (c) theoretical sampling from literature, (d) staying grounded and (e) making research memo (Thornberg, 2012). This research applied two strategies (theoretical criticism and theoretical pluralism), making constant comparison between residential reasoning framework and other



related framework (e.g. the Compensatory Secondary Control Model, the Optional Recognition Model) (see, on Chapter Three), reviewing each frameworks with critical lens. Verification strategies such as member checking was also used to ensure that the narrative and experience of participants was initiated from participant's own standpoint, not being forced into existing concepts (see, the next section). In general, this research argues that, elaborating the risks of sensitizing concept did not mean to devalue its benefit. The sensitizing framework enables the cumulative generation of knowledge – the emergence of new knowledge, the revising and criticizing of existing knowledge which pure inductive inquiry may not being able to accomplish (Charmaz, 2008).

Using sensitizing concept from the beginning are also raise the questions of how the knowledge construction process come to an end. According to Charmaz (2014) and other grounded theorists, the sampling and analysis process could be stopped when theoretical themes/categories were saturated (Glaser & Strauss, 2017). However, Bryant (2017) criticized the expression 'theoretical saturation', suggesting that if researchers were fortunate to find the theoretical conjecture, justifying theoretical saturation required limited effort and examination (Bryant, 2017). For the constructivist grounded theory approach, it is highly possible that qualitative researchers dig new categories and themes from previous data again and again (Charmaz, 2014). Therefore, the sampling and analysing process may never be truly 'saturated'.

This research opened up the possibility that new findings may emerge in further analysis. This research, however, has argued for theoretical saturation based on the following criteria: (a) no new properties (regarding the subcategory of each theme) emerged from the analysis; (b) data collected from the verification stage could be contained into the framework. In other words, the framework enabled the prediction of residential decisions from new data; (c) the framework of this research was logically integrative; and (d) the framework fulfilled our research goal, that is, to explain why certain residential decisions (AIP or institutionalization) are made (O'Reilly & Parker, 2013).

4.3.4.5 Data verification (stage 4)



This stage sought to verify the theoretical framework formulated from the previous stage. It included techniques to enhance methodological rigour (as illustrated as below) and strategies to check the fitness (the extent of the research goal, participants' narrative and our analysis were integrative), generality (the extent to which our framework could be applied in multiple circumstances) and understandability (the extent to which our framework was understandable to people in this area) of our findings (Cutcliffe, 2005). These indicators were discussed and validated at a peer debriefing meeting and two conferences. It turned out that the fitness and understandability of this research were rarely challenged by peers, yet the generality of findings require careful examination in future research. Details are discussed in the final discussion chapter.

4.4 Enhancing the method rigor

This section elucidates methods and techniques to enhance the methodological rigour of this research. The definition of methodological rigor depends on how knowledge is defined and evaluated, and how the researcher is positioned (value-free researcher or value-infused researcher) in front of the produced knowledge (Denzin, 2009). Some qualitative research evaluated the extent to which the research process was systematic and coherent during practice (Mays & Pope, 1995); others criticized that the such criteria was origin from positivism paradigm, hence might not be appropriate on qualitative study (Cho & Trent, 2006). In line with the constructivist approach, this research refers to Cho's (2006) notion of *transnational validity* to check the methodological rigour. The methodological rigour and trustworthiness of the results depend upon techniques that ensure the validity of interpretation (e.g., triangulation, member checking) and the researcher's own reflectivity (Cho & Trent, 2006).

4.4.1 Triangulation

Triangulation accounts for the process where the researcher refers to other sources of information to verify the trustworthiness of a certain phenomenon/event (Cho & Trent, 2006; Decrop, 1999). In this study, for example, the meaning of *ku* and having someone at home was verified from the perspective of participants, village cadres and institution staff.



Triangulation was conducted during and at the end of the research process. The researcher interviewed the staff from institutions or village cadres, asking what their understanding was of *ku* and having someone at home. One village cadre verified that *ku* was widely used in dialect in (or perhaps beyond) Yunnan province. The ability to *ku* is influenced by one's physical, psychological and financial strength; it depends on the person's ability rather than the willingness to continue to *ku*. When it comes to the term 'someone at home', one staff from a rural institution used the term 'uncertain' to summarize the status. 'No matter how close a childless person might appear to others (their extended kin), everyone will eventually focus on their own family.'

An anti-poverty work team member, who had a close relationship with one childless participant, confirmed that the residential decision was a negotiation process. 'It usually takes one or two years for them to confirm the decision,' he suggested. 'Those self-sufficient individuals may be reluctant to move to institutions due to anticipatory declined autonomy.'

4.4.2 Member checking

Member checking refers to the process in which analytic categories, interpretations and conclusions were validated by members of groups whose voice being obtained from the original data (Cohen & Crabtree, 2008). This strategy was commonly applied to minimize the misunderstanding between researcher and participant, reducing the risks of data forcing (Goodwin & Horowitz, 2002). Four childless older adults, including one institution dweller and three community dwellers, were followed in a face-to-face interview. The researcher reviewed findings in front of the participants, asking for their feedback, comments or revisions. Four participants supported the findings of this research; one participant made supplement. This participant, who 'having no one at home to count on' during the first and follow-up interview, admitted that he had a potential wife to provide care. It was a newly fostered marital relationship and had only lasted for 20 days. This situation supplemented findings of interpersonal adaptation and negotiation (in Chapter Eight), revealing that the existence of significant other(s) was changeable.



Another participant questioned the purpose of this interview. Interestingly, although he claimed he understood the meaning of this research during the first interview, he doubted ‘whether this type of interview could change anything’ during verification. This question underlined, again, the possible power imbalance between researcher and participants and revealed how disadvantaged and powerless the childless older adults were. It also raised reflection and concern from the researcher that qualitative research practice may become a tool to exploit in some circumstances. More detail is illustrated in the reflective box of the researcher.

4.4.3 Peer debrief meeting

Peer discussion helped the researcher to clarify the methodological guidelines and the researcher’s optionality (Sharon Spall, 1998). In this study, the researcher took part in a monthly peer debriefing meeting with the researcher’s supervisor and two P.h.D candidates (Morse et al., 2002). The term ‘*ku*’ and having someone at home were especially discussed at the peer debriefing meeting. Participants of this meeting demonstrated great interest in such native expressions. One participant suggested the researcher should consider the anticipated prospect of having someone, that is, whether the current status of having someone influences one’s future perception of having someone). It enabled the researcher to extend his/her reading on prospect theory (on how individuals make decisions under assessment of future gain/loss) (Kahneman & Tversky, 1979). Further analysis supported this assumption, revealing that childless older adults attempted to accelerate their ‘negotiating leverage’ for not only current support but also anticipated care in the future.

At the same time, the supervisor of researcher underlined the importance of methodological design and rigour. This research initially confused the concepts of theoretical sampling – to fill gaps and properties of the initial framework by recruiting more samples – and verification, to verify findings in the follow-up interview (Barusch et al., 2011). It turned out that the seven follow-up interviews ought to belong to theoretical sampling instead of verification. Correcting this mistake in later research, this research conducted four more follow-up interviews to verify frameworks.



4.4.4 Reflexive notes

The problem of transformational validity begins with the ‘crisis of interpretation and presentation’ in qualitative research, that is, a socially constructed reality yields to multiple explanations from different perspectives (Cho & Trent, 2006). Therefore, methodological rigour cannot be achieved by listing verification techniques (e.g., member checking, triangulation) but needs to include participants’ reflective notes (Kvale, 1995).

Charmaz (2017) suggested that the reflective note could begin by questioning the researcher’s own social positionality in relation to the participants, so that the power imbalance was gradually crystallized (Charmaz, 2017). Following this argument, this note began by examining the researcher’s social location, then introducing a ‘new understanding’ of reflection.

I am an urban-born young female, who grown up in society where education is considered the most effective way for social mobilization. Education is inheritably associated with power – to be knowledgeable or only performing scholarly identity increase one’s cultural and social capital. The identity of ‘educated middle class women’ includes social assumptions such as ‘positive and resilient’, ‘work hard to follow the social expectation’; in some circumstance it encompassed the image of ‘being ignorant on rural life’, which is true. This socio-economic background differentiating between participants and me creates misunderstanding and relational distant on the one hand, paving the way for critical reflection on the other hand. I realized such difference as long as I entered the research field. Even I clarified the research purpose before interview, and even participants re-emphasized that they ‘understand’ the purpose, it still confused them why there was someone willing to care about their family life. The following section summarized three notes that sparked my thinking.

4.4.4.1 The ‘salience’ of participants

It has to be admitted that encouraging participants to talk about their life was not easy. Questions such as ‘what do you usually do each day?’ usually resulted in a friendly,



confused smile, or simple words such as ‘just do the usual thing’. I questioned his/her interview skill and the understandability of the research purpose he/she clarified at the beginning of the interview. As the research proceeded, I assumed that it may also be related to participants’ socially disadvantaged position, which rendered their day-to-day experience ‘too normal and unimportant to discuss’.

This phenomenon was found in anthropology literature among other socially disadvantaged groups such as immigrant women (Nagar-Ron & Motzafi-Haller, 2011; Puvimanasinghe et al., 2015). Anthropologists assumed that this silence (e.g., ‘there’s not much to say about my life’) was an extension of structural oppression and marginalization. When internalizing such an identity and idea, participants appeared confused (and surprised) when they were asked to discuss their ‘life story’ (Nagar-Ron & Motzafi-Haller, 2011). This was especially true given that some of the participants of this research had hardly met the others. Narrating their life experiences was unfamiliar in their daily life.

A growing body of literature calls for critical understanding of ‘silence’ (or inarticulation) in a qualitative interview. This was neither due to (but possibly related to) ‘poor interview technique’ nor ‘low willingness to participate’. Instead, it could be a way to detect disempowerment, oppression and social injustice (Puvimanasinghe et al., 2015; Tsalach, 2013). From the poststructuralism and feminist perspective, silence was a way for disadvantaged participants to take control of their narrative space and gain autonomy in deciding when to stop disclosing, and when to start again (Nagar-Ron & Motzafi-Haller, 2011). Both explanations seemed applicable to participants of this research. One of the pieces of evidence was that, when I applied the story-like interview guideline on stage two to invite participants to elucidate their family members, their willingness to share increased. This might have been because this type of conversation was more familiar and comfortable for participants.

However, one informant disagreed with my assumption that ‘silence was an extension of one’s social disadvantage’. The anti-poverty team member reminded me several times ‘not to trust everything they told you’. ‘It is possible that they are silently planning on something behind your back’, implying that they were not as vulnerable



and disadvantaged as I assumed. In a verification interview, one participant (an institution dweller) was invited to discuss his understanding of the findings of this research; he seemed reluctant to participate. He disclosed limited information in the face of questions such as ‘what do you think of our understanding?’ or ‘do you agree with our understanding?’, which was coded inarticulateness. As the interview processed, he asked me to reclarify the research purpose and suggested: ‘If the conversation did not do any good, it was pointless to continue.’ I realized that he had a story to tell – not related to the research question but important for some reason. Eventually, the man asked me to record his fear of, and uncertainty about, quarantine during COVID-19: ‘better to voice his needs to the township government’. If I agreed, he would continue to talk.

I didn’t follow the man’s order to ‘voice his needs to the township government’ because it might have disclosed the man’s identity. The man decided to participate in this research eventually. This made me feel regretful, guilty and powerless. I realized how limited I contribute to participants. This event also re-emphasized the need to consider participants’ narrative – the unspoken aspect in particular – from multiple perspectives. From the story of this participant, I am inclined to connect this silence (and inarticulation) to participants’ bargaining intention. He attempted to use his silence and inarticulation to bargain for my assistance. I had no intention of blaming or judging this man but began to understand how agency and resilience were applied and manipulated during such a constricted situation.

4.4.4.2 The resilient characteristic and struggling

On realizing the participant’s subtle agency and resilience, I re-examined his/her positionality and possible power imbalance between participants and researcher. I started to question my analysis of participants’ resilient character – a characteristic that was embedded in the socio-economic differentiation of participants and researcher, reflecting participants’ lifelong disadvantage and oppression. The research question that originally came from literature, that is, how participants struggled to adapt to residential difficulties, had been turned into a more critical concern of *how participants illustrated and performed their adaptation in front of me*. This reflection, again with no intention



to deny the resilient characteristic of participants, provided more critical awareness when analysing participants' narrative.

For example, when participants insisted on 'focusing on day-to-day wellness instead of making a long-term plan', is it possible that the immediate sense of wellness was the only experience where the participants could feel control? When participants expressed their pride at being a 'member of a progressive, advanced society', is it possible that this discourse enabled them to feel mastery, control and power? Previous research on everyday decisions among economically disadvantaged groups demonstrated a similar pattern. When individuals spend most of their savings on smoking or alcohol, it reflects a structural sense of powerlessness. When accommodating one's own experience into a grand discourse of progress and developing ('I'm proud to live in a progressive society'), it might release the suffering and sense of powerlessness a person has experienced in their life. Possibly, taking control of their wellness in everyday decision-making and in one's identity narrating enabled a sense of mastery and control – at least during the interview (Haushofer & Fehr, 2014). This realization shifted the attention from resilient character and adaptation behaviour to the struggling and messy side of adaptation. Details of such analysis are provided in Chapters Six, Seven and Eight.

4.4.4.3 Feminist perspective and placeless women

As a self-acclaimed feminist, I cannot avoid a feminist lens during data collection and analysis. I attempted to balance the number of men and women when recruiting; I was always aware that childlessness might be gendered in rural areas. However, having an understanding of gender issues and having an emotional connection with participants' oppressive expression were different stories. When one female participant illustrated her suffering from domestic violence and intensive domestic work in a casual, even humorous tone, I was shocked. When a female resident (a male participant's lover) was told to be 'silent' during the interview because her lover thought she was 'unable to express her feelings', I could not help but invite her to a separate interview to disclose her experiences and life stories. The woman cried during the interview, using words such as 'I cannot express', 'I don't know', 'I cannot make it clear' several times. I



became emotional too.

It was emerged that my understanding of feminist and women's suffering has long been blinded by a class lens. I was educated in a context where women don't have to rely on marriage for any life material or life achievement. But for female participants, the realization seemed a 'luxury'. Not everyone was equipped with the idea of gender equality or being able to 'deviate' from marriage. The woman stayed with her violent husband for several years to 'protect her young son'. She divorced her husband when her son had grown up, staying with her lover (a childless man and a participant of this research) and justified the 'lover' as a 'better option' than her husband. The lover told her to 'shut up' and 'keep quiet' several times during the interview; I understood her.

This realization allowed me to add a discussion on 'women's mastery and placeless experience' to this research. Rural women were traditionally 'reliable' to marriage and their husband. By 'marrying into men's family', women's connection to their own family was weakened. In other words, unlike rural men who were 'born to belong to the village', women's sense of mastery was closely tied to their offspring – usually a son.

To summarize, these reflections were rooted in the socio-economic differentiation between participants and me. Unexpected events happened during the interview, triggering my new understanding while paving the way for detecting social injustice in a way that the participants themselves may not realize. Fundamentally, it reminded me that conducting qualitative research might not be as empowering as I originally assumed, especially from the participant's perspective. One participant asked me several times 'how this conversation could change their current situation'. I was ashamed and felt guilty not to be able to provide any convincing answer. It reminded me that although all participants appeared 'supportive' and participated 'voluntarily' in this research, the structural disadvantage and oppression that remained a strong and unescapable social aspect of the qualitative interview always mattered.

4.5 Summary of this chapter



This chapter illustrated the paradigm stance, the epistemological foundation of this research and the research design. The qualitative inquiry and ground theory approach were selected to guide this research. Finally, the chapter argued that the constructivist grounded theory approach was applicable in terms of the epistemological stance, the research logic, the form of knowledge this research aimed to produce and the position of the researcher. Following the constructivist grounded theory guidelines, this research aimed to gain an in-depth and critical understanding of the residential reasoning process from the perspective of the childless.

This research is divided into four stages: (a) the initial stage to gain general understanding of the residential-decision experience; (b) the second stage to explore in-depth understanding; (c) the stage of theoretical sampling and formulating; and (d) verification. Each stage has its own strategies for data collection and analysis; the sampling process and data analysis in the next stage are guided by analysis results from the previous stage. Methodological rigour was enhanced by triangulation, member checking, a peer debriefing meeting and the self-reflection of the researcher. Analysis of data revealed that residential reasoning was an ongoing process in which mastery experience was centred.



Chapter Five

Childlessness in Rural China

Before introducing the residential reasoning process, this chapter introduces the life experience of participants, hopefully to enable readers to have contextual understanding of childless life in rural China. Childless older adults had experienced lifelong poverty in rural areas, witnessing great historical changes during the Cultural Revolution, the Collectivism Period and the rural-to-urban immigration trend in the 1980s. This chapter focuses on demographical features, childhood memory, the working experience, family relations and the reasons for becoming childless in later life. Overall, the findings of this chapter echo with previous findings on childlessness in rural China, that is, it is a disadvantageous and undesirable situation that requires adaptation.

5.1 Introduction of participants

As stated in the methodology chapter, all the participants of this research were childless, aged over 60 and living in a rural community or rural institutions. An introduction to each participant in terms of their residential arrangement, marital status and childless status is presented in the following Table 5.1. The demographic features of the participants were shown in Table 4.2 (Chapter Four).



Table 5.1***Introduction of participants***

Residential arrangement	Participants	Brief description of general situation
AIP group	TS	No.1. Male, 63, living with three brothers, never married
	SP	No. 2. Female, 76, living with daughter-in-law, loss of children
	TL	No.3. Male, 65, living alone, never married
	GG	No.4. Male, 73, disabled, living with grandson, separated from spouse
	AC	No.5. Male, 63, mentally disabled, living with brother, never married
	YZ	No.6. Male, 69, disabled, living alone, never married
	AB	No.7. Female, 81, mentally disabled, living with nephew, infertile
	CG	No.8. Female, 76, mentally disabled, living with granddaughter, infertile
	CC	No.9. Female, 67, living alone, never married
	BY	No.10. Male, 63, living alone, never married
	TTL	No.11. Male, 76, mentally disabled, living with brother, never married
	TTC	No.12. Male, 73, living alone, separated from spouse
	CN	No.13. Male, 62, living with brother, never married
	CL	No.14. Male, 68, living with nephew, never married
	JZ	No.15. Male, 64, mentally disabled, living with sister, never married
	SQ	No.16. Male, 75, living alone, never married
	GS	No.17. Male, 74, living alone, never married
Institutionalization group	DH	No.18. Male, 66, never married
	LD	No.19. Male, 68, disabled, never married
	YD	No.20. Male, 76, loss of children
	LF	No.21. Male, 74, loss of children
	DW	No.22. Female, 76, infertile
	HH	No.23. Male, 60, disabled, never married
	HM	No.24. Female, 83, disabled, infertile
	YY	No.25. Female, 76, infertile
	WL	No.26. Female, 69, infertile
	WJ	No.27. Male, 92, loss of children

Among all the participants of this research, there were 17 individuals living in a rural community and 10 living in rural institutions. Seven of the participants in the AIP group lived alone; 10 of them co-resided with extended kin. Ten of the participants from both groups had physical or mental disabilities, including blindness, stoop and one cripple. All participants were able to continue daily activities.

All participants were covered by the Extremely Poverty Assistance Scheme (*te kun bu zhu*) and the New Rural Pension Scheme (*xin nong he*). The ‘welfare package’ included cash subsidies from RMB 500 to 800 (about USD 78 to 125) per month, this varied across villages. Life materials allocated by the village committee such as rice, vegetables and clothes. As the anti-poverty scheme progressed, house modifications, medical expenses and funeral preparations were covered by the ‘package’ as well. For those living in rural institutions, the ‘package’ was directly allocated to the institution. Participants received RMB30 to RMB150 (USD5 to 25) for ‘pocket money’ – usually used for sweets, alcohol, cigarettes or playing poker, depending on the financial capacity of each institution. There was no expenditure required in rural institutions on food, clothing, medical support or funeral preparations.

5.2 Childhood experience

Participants of this research grew up witnessing constriction and some changes in rural China. Poverty is nevertheless at the centre of childhood memory. Poverty prevented participants from attending school, from settling in stable accommodation and/or from seeking appropriate medical care when necessary during childhood. Some of the negative impact has been accelerated in later life.

TS, for example, is a 63-year-old man living with his two single brothers. He went to primary school for three years until his teacher was captured during the Cultural Revolution for being ‘politically rightwards’ (*you qing*). The school TS had attended was established in a degraded local temple, and had approximately 50 to 60 students. The first and the third grade were combined in the same classroom. TS left school due to household poverty: ‘My family need a labour force to earn working points.’¹ TS left school around 1968, ‘bustling around’ farming, digging channels or building roads to earn working points:

I attended school for three years. After leaving school, it was run or rush everywhere. I worked (as an immigrant worker), digging channels, building roads...we earned working points instead of

¹ Working points were a way to calculate farmers’ ‘contribution’ to the village collective during the 1950s; the allocation of food, oil and fabric relied upon the working points of a household.

cash (during the 1960s). I had to earn working points for my family. It was not until the 1970s we had our own land and started to work for ourselves. I basically stayed at home and farmed, nothing more. (No.1. TS)

Education was an important way to diversify career choices. Some participants were hired by the village collective as village accountants; some benefited from the education they'd gained from childhood and worked as village cadres in middle life.² LF, a retired village cadre, expressed his regret at 'receiving limited education', which reduced his chance of promotion:

Back to my childhood, it was difficult to afford the education fees. I lived in a mountainous area instead of a plain area, which was poor and 'backward'. When I entered the fourth grade, the Cultural Revolution broke out. I stayed at school for only half a day and farmed the land the other half. The teacher attended the school quite randomly. I didn't learn anything. (No.21. LF)

Attending school in the 1960s imposed a great financial burden on rural households. Most parents would decide whether it made a 'great investment' for children. Such hesitation hindered girls' chance of receiving the same level of education as their male counterparts, as it was assumed they would 'become someone's wife' in the future. For most rural parents, it was an 'inefficient deal' to invest in girls' education compared to boys (Liu et al., 2007). Participants' demographical features reflected such gender discrimination in education, since none of the female participants had the chance to attend school. Among the male participants, was only one person, DH, 66, an orphan since childhood, remained illiterate for his lifetime. CC, 67, an illiterate woman, confirmed this phenomenon by stating:

My father died when I was young. Next year will be 60 years since his death. During the Collectivism Period we had a collective kitchen; if you didn't work (farm) there was no food for you. My mother held her baby (the participant) while carrying another baby (the participant's sister), earning working points at the same time. My mother was ONLY a woman, she had neither

² The common qualification for a village accountant or immigrant farmer was primary school. If a person didn't finish primary school education, they had to prove that they were able to read, write and do basic calculations.

the ability nor any money to afford us (the participant and her sister) going to school. (No.9. CC)

The poverty issue in rural China was usually successive from one generation to another (Glauben et al., 2012). A person who grew up in an impoverished household had to deal with layers of disadvantages from childhood until adulthood. DH's story demonstrated how 'growing up as an orphan' was miserable and full of suffering. DH's parents died in the 1950s, and since then he has constantly engaged in farming. His childhood was full of scarcity, turbulence and poverty: 'I ate whatever the neighbour gave me; I stayed wherever it was warm.' The village collective had provided him with food and clothing for several years, sending him to an adoptive family thereafter. It was not a pleasant memory, because the adoptive family was abusive. At the age of 18, DH approached the village collective to 'address the adoption issue'. The adoptive relationship was finished; DH was offered 'a basket, a quilt, a hoe and several grains' by the village collective, then he went back to his home. He decided to move to a rural institution in 2007. He commented:

When I was young, I spent most of my time in the corner of a wall (the participant grew up as an orphan). If any villager found me, they might give me something to eat. The village collective couldn't raise me, so they 'gave' me to a household (establishing an adoptive relationship from an oral agreement). They abused me. I went to town and the village collective fixed the problem. There was a person, Mr Wang, who was in charge of issues such as brawls and marriage conflict. He gave me a basket, an old quilt and some grains. I went back to my own home by myself, farming for myself thereafter. (No.18. DH)

Poverty and health inequality were intertwined in rural China. Some participants remained disabled from childhood, because 'there was no resources to gain any medical support when necessary'. LD, 68, relocated to a rural institution in 2010. His right leg is shorter than his left, which was the result of an accidental fall when he was three months old. His parents went out to earn the working points for the household, leaving him alone at home; he fell from the bed. His parents were too poor to afford medical assistance. LD remembered the experience with tears in his eyes:

My parents were incapable and weak, to be honest. They didn't leave me even a pair of chopsticks,



but they did make me disabled...when I was four months old, they (the participant's parents) went out to work, and I accidentally broke my leg [tears]. I didn't go to hospital, now I'm a disabled person. This is my situation: I have no house, no bowls or chopsticks. The older generation were...[sigh] (No.19. LD)

Most of the participants used the term *difficult* to describe their childhood memory: starving and turbulent, limited income, poverty. These stories and narratives corresponded to census data on poverty, in which accelerated household poverty and persistent poverty have always been issues in one segment of rural life in China. In 2014, there were still 70.17 million rural people living under the poverty line of RMB2,300 (USD 359) per capita annual income (Li et al., 2016). Lifelong exposure to poverty created layers of disadvantage and constriction for the participants, influencing their career path, family relationship, marriage and the reason for becoming childless. Poverty constitutes one of the main contexts and life themes for childless older adults.

5.3 Working experience

Farming was the most prominent occupation for all participants. The necessity and urgency of farming was embedded in the specific social and historical context of rural China, where farming was the only way to guarantee food supply since the participants' childhood (Pang et al., 2004). The heavy reliance on farming was derived from the Collectivism Period (1960s), when farming activities were quantified as types of 'contribution' to the village collective. For example, as AB's nephew, 53, a retired village cadre, remembered his childhood during the Collectivism Period:

During the Collectivism Period (1960s) we counted our working points to calculate the contribution. Everything in the village was planned. Now we have our own farmland and we can decide how to deal with it

³ *...When I was little, our family relied on food coupons to obtain grains. (No.7. AB)*

LF, 74, a village cadre during the Collectivism Period, remembered his experience

³ A Chinese farmer has the right of use of farmland instead of the ownership of farmland.

of farming:

During the Collectivism Period we combined three villages into one. It would take us three hours to walk from home to the collective farmland; four hours were spent on transportation every day. We arrived at the farmland before dawn, and it was all about farming. The farmland was mountainous, not flat. We suffered. So many people died from starving. (No.21. LF)

It was not until the 1970s that the Household Contract Responsibility System (HRS) initiated land distribution from the village collective to each household. Farmers were able to ‘work for their own pocket’ from then (Han et al., 2019). This land reform policy agenda shifted the farming pressure from the village collective to each household and individual, forcing farmers to ‘count on their own effort to gain grains, vegetables and other life-related material’. The narrative below illustrates HH’s (60, male) perception of being ‘self-sufficient’ after the reform:

After the Household Contract Responsibility System, every household had some land, more or less. If you didn’t farm, you didn’t have anything to eat. That was the reality for everyone. We didn’t have any commercial grains, not like now. It was just....self-reliance. I grew corns and peanuts. It was just about enough to eat. (No.23. HH)

After the Reform and Open-Up Policy in the 1980s, market expansion in the agricultural industry allowed farmers to divide the farmland into small sections while growing commercial crops (e.g., tobacco, beet, beans). Thus, for childless farmers, ‘starving was no longer common (GG)’. Childless older adults were encouraged to lease their farmland to local enterprises for commercial corps to increase their income. Accordingly, they tended to purchase grains at the local market⁴ and grow vegetables on small pieces of Residual Land (*zhai ji di*) near their house, in front of their yard. As GG (73, male) narrated:

I just stayed home and dealt with my yard. I grew vegetables in my yard. My farmland was rented to others (a village company) ...it was distributed to me in the Collectivism Period. They (the company)

⁴ 50RMB could purchase one bag of rice (10 kg).

paid me a little...just a little, not much. (No.4. GG)

All participants considered farming to be ‘the bottom line to guarantee daily living (TL)’, while some of them shouldered other types of ‘works’ in the village – mostly part-time jobs to diversify and increase income. For example, TS worked as a village cadre in his youth (when he was about 24 years old), ‘leading others to accelerate the working points’. LF, similarly, was proud to be one of the ‘main characters’ in ‘guiding others to work’. The narrative below elucidates a story when he ‘guided’ the villagers to build telegraph poles:

Back in the 1970s we grew tobacco. At that time, RMB600 or RMB1000 (USD 93 to 156) could buy the best stuff. I was the village cadre by that time. I received an order and sold our tobacco for RMB600. In 1984, the village didn't have electricity. I tried to work it out, asking the administrative centre of the town for help. We chopped wood for telegraph poles and connected electricity. It wasn't until 1989 that we finally finished the job. The township government gave us some wood, as I asked. It was difficult. Everybody else had electricity, except for our village. (No.21. LF)

Two participants of this research shouldered a full-time job, leaving the village to ‘work outside’. BY, 63, had worked as a security guard in town. BY was one of the most ‘educated’ participants of this research (he finished high school). By the time of his graduation, he had a chance to ‘transfer’ to urban citizenship, but he failed. The ‘recommendation quota’ for university was limited, according to him, and it contained corruption. BY stayed in the rural community until the 1970s, when an ‘old friend’ invited him to ‘leave’. BY was satisfied with the job as ‘it was less laborious than farming’. In 2013, he fell and injured his left leg. He planned to retire from the job and returned to the community to ‘continue my life’. He was regretful but understood that ‘it would be legally troublesome for the old friend to hire me’.⁵

Three participants had joined the military at a young age (approximately 16 or 17) and left the village. WJ was once a sergeant in the Communist Party but was ‘criticized’ and dismissed from the Party during the Cultural Revolution – his father was

⁵ BY is approaching the age of 65 – the legal retirement age in China. His hukou (戶口) was in his original rural area. If he continued to stay in the urban area, the company would address the pension issue.

categorized as a ‘landlord’. GS, 74, was in charge of the telegrams in the military; YD, 78, had participated in the Three Front Line Construction (*san xian jian she*).⁶ YD remembered his previous career achievements with a tone of pride:

They (the village cadre) ordered us (villagers) to participate in the programme. Everyone had to answer the call. Those who joined the army only had to stay in the military for three years. For us, we had to wait until the programme was finished, then we were allowed to come back. The western area of China didn't have a railway at that time...I joined the programme for five years. (No.20. YD)

The ‘part-time’ and ‘full-time’ job move beyond farming created a chance for farmers to get away from laborious farming activities while increasing their income, yet farming was central in participants’ narrative and experience. The ambivalent attitude towards farming is illustrated in the next chapter on residential assessment and adaptation.

5.4 Family relationship

Rural residents born between the 1940s and 1960s usually grew up in a ‘big’ family: the fertility rate was as high as 7.5 in the early 1950s, then sharply decreased after the 1970s and 1980s, around the time when China began implementing the One-Child Policy (Zimmer & Kwong, 2003). Rural couples believed in the idea of filial expectation and the prospect that ‘raising the children (usually son) could guarantee later-life wellness’ (Djundeva et al., 2018). Therefore, all participants of this research had at least one or multiple siblings. For example, CL (73, male) had two brothers living in the same village and three brothers ‘working outside in urban areas’; CN had two brothers living in the village.

All rural siblings had to experience the rite of household division (*fen jia*), where adult children (both men and women) were expected to leave their aged parents one by one, getting married and establishing their own core family (Zhang, 2004). During the household division, a contract regarding who would inherit the farmland, who would

⁶ The Three Front Line Construction program was initiated in 1960s to expend the infrastructure construction (e.g., the transportation, dam and electricity) in west and north area of China.

take care of the aged parents and who would take responsibility for disabled siblings (if applicable) was confirmed among siblings – legally or orally.

In the ideal scenario, it would be the oldest brother who inherited his parents' old house and farmland while taking care of the aged, fragile parents. However, the situation was far more complex in reality. Sometimes the adult son was 'too poor to shoulder the caring responsibilities'; sometimes the distant or conflictual relationship between a young wife and aged parents hindered the caring agreement.⁷ In rural Yunnan in our research field, it was common for aged parents to be *separately* assigned to the son's household to receive care. BY's narrative confirmed this phenomenon:

We had two boys in our family. During the house division, my father lived with my brother; my mother stayed with me. I didn't get married so the domestic chores needed some help. That's why I had my mother. I almost took full responsibility for both of my parents, but the village collective stepped in. They noticed my difficulty and stopped it. Now my brother took care of my father. He was satisfied with this arrangement, I guess. Even if he wasn't, he wouldn't say. My father could help him to pasture cattle. (No.10. BY)

Although such an arrangement might be renegotiated among siblings. CN's parents were originally 'assigned' to the oldest and the second brother, separately, but in reality, it was CN's youngest brother who shouldered their daily expense and 'sent them to the grave':

It was confirmed in that way (the oldest and the second brother raised their parents), yet in reality I took care of them both. My parents stayed with me until they died...they (the parents) didn't want to stay with my two brothers. They liked me. How can you reject this situation [laugh]? (No.13. CN)

The assignment of caring responsibility should correspond to a person's willingness and capacity – financial and physical – to provide care. After the house division, newly established households (with an adult son and his new wife) would disengage from their aged parents, 'focusing on the issues and problems of their own

⁷ This usually happened between a son and his young wife. The wife was not expected to shoulder the caring obligation for her own parents, as it was her brother's responsibility.

household’. In some circumstances, when siblings had a mental or physical disability – ‘when the parents realized the limited chance for a certain child to establish their own household’ (CN’s caregiver) – their brothers or sisters might adopt this person (who were most likely to become childless in later life) and provide long-term care ‘until the last day of their life’. The narrative of CN’s youngest brother confirmed this phenomenon.

When both of my parents were alive, they assigned my brother to us (other siblings). Whoever stayed in the village had to take care of him. I was the one who stayed. (No.13. CN)

To summarize, childless older adults were usually born in a ‘big’ family with at least one or multiple extended kin members living in the same village. Some siblings (and their next household generation) were assigned to take care of a disabled member of the family during the household division, while some childless older adults had to count on themselves to make a living. More detail on this topic is presented in Chapter Eight.

5.5 Becoming childless in later life

This research identifies four reasons for rural residents to become childless in later life: (a) *childlessness because of lifelong singles (usually for men)*; (b) *childlessness because of divorce/separation*; (c) *childlessness because of infertility*; and (d) *childlessness due to losing one’s children*. It was both embedded in and ‘deviant’ from the mainstream norm, that is, marriage and reproduction were closely related (Verdery et al., 2019). Overall, childlessness in later life was ‘unexpected’ and ‘undesired’; it was neither triggered by household poverty nor considered a ‘failure’ in terms of carrying the family line.

Men from impoverished households may have found it difficult to find a mate. Fourteen out of 18 male participants fall into this situation. The most common excuse for this situation was ‘mismatch’, although participants realized that the ‘true reason’ for such a situation was household poverty. As YZ (69, male, never married) summarized:



Because the person you liked didn't like you back, and those who liked you were not people you liked. Girls despised me for making so little money. 'Others earned a dozen yuan per month. How about you? Starving to death?' I don't know. There's nothing I can do. (No.6. YZ)

Family reputation and status within the village was another important consideration when establishing rural marriage. It was possible for some participants, still male, who 'failed to find any girl' to get married. TL is a 63-year-old man living in a rural community. He was also the only participant who attended high school. After graduating, TL stayed with his parents to farm. TL attributed his single status to two reasons: the 'bad history' of his parents and financial insufficiency. TL's parents were categorized as 'landlords' during the Land Reform (*tu di gai ge*),⁸ which was sinful under the Communist Party of China's (CCP) political agenda. Moreover, TL's household was too poor to afford his marriage expense:

My family had a bad reputation in this village. It was difficult to find any matchmaker to help me. Another reason is, there were too many brothers in my household, and the house was tiny. Gradually, when I grew old, it was even more difficult to find a mate. I have six brothers and four sisters. (No.3. TL)

The second situation that drove a person to become childless was separation or divorce. Two participants fell into this type. GG, for example, married a woman for four months until she 'left home for no reason' (when asked during the interview, GG refused to articulate the reason). GG attempted to find the woman on her own house, but she refused to 'come back'. GG was never married and remained childless thereafter. His life after his wife left centred around farming, and during the process a cousin of GG had 'provided intensive help'. In 2000, GG planned to repair his old house he had inherited from his parents. A step grandson of GG (the son of GG's stepbrother) offered to take care of GG until the 'last day of his life'. GG combined his old house and farmland with the man's land, receiving support and care since 2000. The caregiver and his wife worked in Shanghai in construction, in one of the biggest metropolitans in

⁸ One of the policy agendas on the Land Reform was to curtail the privatization of rural land. Hence, those who were categorized as 'landlords' were considered the 'political enemy' of the CCP and needed to be criticized or educated.

China; GG stayed in their new house to ‘take care of the house and keep the farmland fertilized’.

Thirdly, it was possible for rural couples, usually women, to become childless due to infertility. Five participants (all women) fell into this type. This research illustrates YY’s life experience as an example. YY is a 76-year-old woman living in an institution. The interview started with her first marriage – the most memorable event of her life. YY’s husband was a brutal man who always abused her for ‘unknown reasons’. YY married this man due to her mother’s disability (her mother was blind). YY obeyed her mother’s instruction to ‘marry a person, whoever he is’, to be settled in her village. YY attributed most of the suffering of her first marriage to her mother-in-law, a woman who ‘instructed her son to abuse me’. Their conflict was intensified during the house division, when YY and her husband attempted to build their own house but ‘received limited help from the husband’s family’. Twenty years later YY sued her husband for divorce.

The lawsuit was a struggle and ‘no legal’ the first time, because YY’s husband didn’t attend court. YY sued again, won the case and went to stay with her sister. She had attempted to adopt a boy, but their relationship turned into conflict as ‘he (the boy) hated me for not affording his tuition fee’. Several years later, YY’s sister died. The brother-in-law (YY’s sister’s husband) sent a matchmaker to YY’s house. YY married the man and the marriage lasted for 18 years, before she moved to a rural institution after the man died.

YY was infertile. According to her narrative, she was ‘unable to bear children’. The reason given for moving to an institution was: ‘unreliable...all of them’ – the adopted son and the four sons of the second husband (YY’s stepson, also his nephew):

I had married, yes, but I cannot have any children. I cannot have children anywhere (the participants had been married twice) ... The villager cadre asked me several times to move to an institution. I pondered, well, I have no place to go. When my husband died, his children (the stepchildren of the participant) were unreliable. He (the village cadre) asked me several times, then I told him I would like to move. (No.25. YY)



The final reason for childlessness is loss of one's children. Four participants fell into this type. For example, DW, 76, female, relocated to an institution in 2016. DW had a boy who tragically died from malaria when he was two weeks old. DW has been infertile since then. According to her, this was due to 'wetness' left on her body. DW has been married twice; both husbands were 'good' to her. The first husband died from a disease (DW didn't know what it was); the second husband died in his 60s. Since then, DW has stayed with her nephew, the son of her brother. Her narrative confirmed the story:

I had a son who died when he was two weeks old. It was dysentery. He had diarrhoea and vomiting, then he died.... The doctor in the village gave me some medicine, but it didn't work. I cannot have children. It was said I have some 'wetness' left in my body when I had my son. I didn't have any children in my life thereafter. (No.22. DW)

Overall, childlessness is gendered in rural China, although its negative consequence are undesired and impose layers of disadvantages for men and women. For men, entering into marriage was perhaps constrained by the financial capacity of the household, while for women, marriage did not guarantee 'children's care' in later life. In general, women are less likely than men to be childless due to lifelong singleness (Gu et al., 2007). One village cadre confirmed our argument by suggesting:

If a woman lowered her standards, it was easier for her to find a mate than for men. In my generation, a man has to have a house and a car to get married. If the (financial) standard was not achieved, it reduced the odds (of getting married). This village is poor and has a bad transportation system. If a person (a man) is not laborious, he won't be able to get married.

5.6 Summary of this chapter

Reviewing the participants' autobiographies will help the reader to gain a contextual understanding of childlessness in rural China. Several characteristics of childlessness need to be specified in this chapter:

Firstly, childlessness in later life is a rare and undesired situation for most rural



residents. China has had the tradition of ‘raising children in middle life to guarantee wellness in later life’ (*yang er fang lao*). The idea was deeply rooted in filial piety – one of the most mainstream cultures in rural areas. For thousands of years among older adults in rural China, filial piety had been socialized and practised for generations of farmers, to ‘carry on the family line’ (especially for men) and to ‘set a pledge for general wellness in later life’. Lifelong singleness, infertility or loss of children in rural areas was eventually ‘deviate’ from the mainstream idea of filial norm, remaining a long-term and mostly negative influence on their farming activities, as well as their social connections and long-term residential decisions.

Secondly, childlessness was gendered in rural China. Although male participants tended to describe their childless status as a ‘mismatch of mate finding’ or ‘bad timing’, it was essentially caused by their constrained financial capacity to afford wedding expenses, to establish a new house and to maintain the identity of the ‘breadwinner’. For female participants, the main reason to become childless was biological infertility or loss of children. Only one female participant of this research was never married (CC) and became childless in later life. Such a gender imbalance in childlessness reflected the rooted discriminative idea that, ironically, women were the ‘potential wife’ of another household. While men were socially expected to ‘afford the expense of finding a wife’ and ‘carry on the family line’, women were expected to ‘carry on the family line’ *for* their husband. This led to the paradoxical situation that finding a mate and having a marital relationship was more difficult for men than women. Men were more likely to be lifelong singles and childless in later life; women were more likely to be childless within a marital relationship.

Thirdly, the causes and consequences of childlessness intersected with the rural and household poverty that had accelerated for generations. Poverty rendered the childless older adult ‘reliant’ on farming activities to gain food and other living materials; it hindered the educational opportunities and the chance of finding a mate, especially among men. For female participants, household poverty constricted them in finding a ‘better’ mate and forced them to stay within ‘undesirable’, even violent marriage relationships to secure their survival needs. Poverty intensified the negative influence of childlessness in later life; the participants had limited options but had to rely on



formal sectors when advancing needs emerged.

Becoming childless older adults in rural China came along with layers of difficulties and individual resilience. The overreliance on farming activities fostered a sense of self-sufficiency and mastery; the lack of children's care forced them to reach out for support from other sources (e.g., from extended kin, neighbours or friends). Overall, this chapter has set the stage for understanding childlessness from a contextual and critical perspective: revealing the cultural and material constriction, and the deeply rooted gender discrimination in rural China while underlining participants' individual agency. The next chapter applies the residential reasoning model to demonstrate how childless older adults adapt during AIP and how they make long-term residential decisions.



Chapter Six

Residential Assessment and Ambivalent Mastery

This chapter investigates the way the residential situation during AIP was assessed by the participants. Living in rural areas encompassed simultaneously the negative experience and individual resilient character (as reviewed before) where childless older adults struggled to maintain self-reliance on the one hand, constrained by the uncomfortable experience of individual disability, social disengagement or stigma while seeking help from formal sectors. The term *ambivalence* was applied to describe the experience. This chapter first introduces the definition of ambivalence, then illustrates how participants' residential assessment was ambivalent in the context of farming, the interpersonal dynamic and seeking help from formal sectors. The final section of this chapter highlights how the ambivalence of residential assessment was gendered among participants. Overall, this chapter concludes that, in regard to the question of 'how do you perceive the current living situation?' ('how did you perceive the previous living situation?' for institution dwellers), mastery experience was the sufficient condition to achieve residential congruence among childless older adults in rural China, although it came along with the negative experience.

6.1 Defining ambivalence

The psychological definition of ambivalence refers to the extent to which people's separate rating of positive and negative reactions to a topic/phenomenon/event reflects a mixed evaluation (Luttrell et al., 2016). In the context of residential assessment, this research uses the term to describe the mixed positive and negative feeling about the current living situation. Participants of this research demonstrated ambivalence when they were asked to 'describe the current living situation' ('the previous living situation' for the institutionalized group): some aspects of rural living were masterful and competent, others were uncomfortable, uncertain and full of suffering. Before revealing the ambivalent feeling of the residential environment, this chapter provides a general



description of the residential circumstances of participants.

The warm weather of Yunnan province enables the villages ‘pleasant’, according to BY (63, male), ‘the moutons was green, and the water was lovely’ (*shan qing shui xiu*). At the same time, the southwest mountainous landscape rendered transportation and large-scale agricultural industry difficult to develop. Villagers in Yunnan tended to separate their farmland into small pieces (approximately 400–600 square metres for a childless farmer) to farm. In most of circumstances, rural residents, including both childless and parental groups, had to resort to physical strength, ‘working to survive’ and ‘working until they drop’. Such geographical characteristic created a mixed feeling of mastery for participants: the urgency to be ‘self-reliance through farming’ or there was limited chance to increase income, and the appreciation on their lifelong resided rural area.

The most common residential environment comprised a single house built from wood or concrete, a yard to raise chickens and residual land to grow vegetables and grains. The distance from one’s house to the residual land was usually less than a 30-minute walk. Tables, chairs, a closet and a stove were the ‘standard’ equipment for all participants, regardless of their financial capability. Some who were better off financially would purchase a TV and a sofa for their house. In most circumstances, the house was rebuilt with government assistance – which was included under the Extreme Poverty Assistance Scheme.

For institutional dwellers, the living situation was slightly better: the water supply and electricity were stable, and a TV and table tennis table were available at any time. Rural institutions were usually situated near a village (approximately 20 to 30 minutes by bus), which allowed institutional dwellers to ‘visit home’ and ‘go to the market’ at any time as long as they notified the staff. Childless older adults were assigned to a single or double room, with a shared bathroom located on each floor, with the men and women being separated. Some rural institutions had a large yard where residents could grow vegetables, ‘to remain physical active’. Labour participation was voluntary. The vegetables the older adults grew were packed and sold at the market to cover their institution expenses; each person could receive RMB5 to 10 (USD 0.9 to 1.5) as a



‘reward’.

The option of institutionalization was informed by the village cadre. As the targeted anti-poverty program continued in every village, those who were registered as Three-no older adults (in some villages, the Extremely Poor Household) was monthly visited by village cadre, with their welfare ‘packages’ being notified during the process. The ‘package’ included the amount of pension subsidies, the subsidies for poverty and the ‘alternative’ residential options that allows the childless older adults to make decision on ageing in place and relocation. This decision was flexible. If the childless older adults changed their mind and decided to relocate to institutions, the village cadre were obligated to make arrangement. The ‘arranging results’ and satisfaction of ‘targeted group’ was included in the evaluation report of village cadre.

Relocating to rural institutions enabled ‘improvement’ of the (physical) environment. Nevertheless, participants demonstrated a strong desire and ability to sustain mastery during AIP. Ageing in a rural community encompassed positive (mastery) and negative (discomfort) experiences simultaneously. Specifically, it was noticed that the mastery experience was expressed in the context of ‘farming’, ‘managing the interpersonal relationship’ and ‘acquiring support from the village collective’. Each gender had a different sense of mastery, which was rooted in the idea of gender discrimination in rural areas. The next four sections of this chapter show how sense of mastery came along with ambivalence in different contexts.

6.2 Mastery of ambivalence during farming: balancing between self-reliance and stress

As illustrated in the previous chapter, the daily life of childless older adults was centred around farming, which enabled simultaneously a sense of self-reliance, stress and hassle. On the one hand, participants showed great pride in being able to rely on their physical effort to make a living. As BY (63, male) suggested:

I can cut trees for firewood. The electricity company gave me 15 kilowatt hours of electricity on discount. I'm planning to buy a bag of rice for RMB60, saving over RMB 300. The salt was RMB1



per bag, I grow vegetables by myself. It's not necessary to move to institutions. (No.10. BY)

Similarly, TS (63, male) rejected the proposal of moving to an institution because of his endeavouring to maintain self-reliance and dignity:

You have to have some dignity. If you cannot count on farming for food, that's another story. But think about the previous time (the collectivism and household responsible period). In the old days, even if you were willing to farm, there might be no chance. Now you farm for your own pocket. (No.1. TS)

On the other hand, farming in most areas in rural China involved intensive physical effort. Farmers had to grow grains and vegetables, raised chicken and gather firewood from the forest. Despite the market expansion and economic growth since the 1980s, farming was always the prominent method of guaranteeing food while economizing everyday expenses. One informant used the phrase 'all you see is work' to summarize the day-to-day experience of Chinese farmers. TL (65, male) supported this argument, commenting:

Farming is.... when you have work to do you had to spend the whole day on the farmland. The work are varied and unlimited. Farmers don't have time to rest. I have to clean up the weeds to make sure my grain grows well next season. Whenever there is work, you have to go to the farmland. (No.3. TL)

DH (66, male), a person who defined himself as a 'solo person'(*du ren*), summarized the suffering aspect of farming:

It is hard being a solo person in a rural area. When I was young, I couldn't guarantee food by myself. I was starving. I had to farm, to clear weeds on the land, to chop the firewood and raise pigs... (No. 18. DH)

Moreover, farming activities was susceptible to health condition and old age. For most participants, farming in such (physically and mentally) declined condition was hardly comfortable. Almost all the participants of this research reported more than one



chronic condition (e.g., high blood pressure, bone fracture, post-stroke symptoms) during the interview. Residential mastery and self-reliance were described under the (hypothetical) condition of ‘capable’ and the subtle fear of declining health:

S: So, you spend most of your time on your own?

TL: Yes.

S: Is that comfortable? I mean...

TL (laughing): Only if I'm not sick. The gross income from farming is less than RMB10,000 (USD 1600) per year. You have to live, buy fertilizer and prepare for sickness. If you don't get sick you could save RMB4,000 to 5,000 (USD 666 to 830). But if I have a disease, this money is nothing like enough. (No.3. TL)

GG (73, male) revealed how visiting the hospital was an ‘unaffordable intervention’ in farming, and how old age prevented any mastery or ambitious imagination:

If you get sick you have to visit the hospital. Farmers cannot afford to have a serious disease, which may force you to leave the farmland for a long time...I don't have any plans for tomorrow. I'm old, it is beyond imaginable. An old man is...you never know when you have to meet the 'end'. I can't imagine anything about tomorrow. (No.4. GG)

This section highlights how ageing intensified the conflict between self-reliance and the stressfulness of farming, rendering the residential mastery full of mixed and ambivalent feelings. Farming activities were extremely labour-intensive in China and other developing Asian countries, as formal support for elderly care and farming activities was rare (Jansuwan & Zander, 2021). Farmers had limited resources but their labour force to count on to make a living. The situation deteriorated when the younger generation of rural residents relocated on a massive scale to urban areas for employment, so that new technology became more diffident to be involved (O'Meara, 2019). This section concludes that, although farming enabled a sense of self-reliance, the overall assessment of the situation was that it was not as comfortable and congruent as the residential normalcy expected (Golant, 2011).

6.3 Mastery of ambivalence on interpersonal relationship:

experiencing the connection and constriction

Maintaining social connection was another important element of daily rural living, and also represented an important opportunity for participants to manifest a sense of mastery. When visiting community dwellers at home, it was noticed that some participants were sitting around with friends and neighbours in public village squares, next to trees and ‘having small talk with their peers’. Among the institutionalized group, although some described how their ‘memory had faded’, ‘having small talk with peers’ and ‘mutually helping friends and neighbours’ were frequently mentioned. Childless older adults cultivated a sense of belonging and connection through visiting friends, or reaching out for support from extended kin, although again, the feeling of mastery was full of constriction. For example, SP (76, female) described how visiting extended kin was an important way to ‘kill time’:

I just came back from my grandson's home. Yes, I visited them quite frequently. I just cannot sit here (in my house), I'm restless. (No.2. SP)

Similarly, TS was proud of being ‘welcomed’ by villagers, which was intricately related to his life achievement:

I feel like I'm welcomed by everyone. Both children and adults in this village like me, whether they're 80 years old or three. (No.1. TS)

Some participants did not intend to be ‘welcomed’ by villagers, yet the ‘society of acquittance’ (*shu ren she hui*) of rural areas still provided a variety of opportunities to meet neighbours, friends and extended kin. The ‘random small conversations’ that happened between villagers – what has been labelled the ‘naturally occurred social network’ in some literature (Van Dijk et al., 2015) – allowed participants to be supported instrumentally and emotionally; all positive feelings enhanced a sense of mastery when staying in a rural community. As TL summarized:

We (me and my neighbours) meet on farmland and talked – talking about basically everything, not important things. It can't be concluded that we are ‘close’ or ‘intimate’. We sent our greetings to



every person we meet on the farmland, providing information regarding farming and fertilizing. I get along well with every person in village. (No.3. TL)

Receiving support and care from friends, neighbours and extended kin came along with mixed feelings. While literature on social connection and rural voluntarism tends to highlight how rural residents demonstrated high willingness to engage in community activities in order to cultivate a ‘sense of community’ (O’Meara, 2019; Winterton & Warburton, 2011), this research tends to shift the attention to the reciprocity and hierarchy of the informal social network in rural China. Specifically, childless older adults realized how receiving support from an informal network – whether from friends, neighbours or extended kin – was never a ‘free meal’. GG described the experience of ‘labour exchange’ with his cousin, a significant figure in his life story, in helping him to build a house and with farming:

Labour exchange... you helped me for several days and I offered my help in exchange. It has to be fair. If he (the participant’s cousin) is too busy I could help, me the same. It’s a good arrangement, I think. (No.4. GG)

BY complained about the ‘indifference’ of his brother’s household:

I have a brother who married an ‘aggressive’ woman. She’s a Yi person (Yi ethnicity) ... Anyway, when my brother attempted to be nice to me, she was unhappy. I decided to rely on myself. I don’t want to count on them. They are living in that direction (pointing to north) ... 4 kilometres away from my house. We don’t engage a lot normally except for any ceremony (e.g., the Spring Festival). It’s just the rural issue, you know. If you don’t live in a rural area, you don’t know the complexity of the rural issue (here referred to as the interpersonal dynamic in a rural area). (No.10. BY)

As the interview progressed, BY suggested that the whole society was governed by such reciprocal relationships: “When you ask for help from others, you have to pay. It is the ‘society of money’”. Similarly to findings in Thailand and Belgium, the apparent familiar and supportive atmosphere in rural areas contains a hidden ‘price’ and is constrained by one’s social and financial resources (Jansuwan & Zander, 2021; Volckaert et al., 2020). In other words, the mastery that a person experienced on an

interpersonal level was mixed and constrictive.

Another factor influencing the extent of the interpersonal level of mastery was relational closeness. Childless older adults carefully categorized the source of support from the informal network and identified the most appropriate figure to provide support. In most circumstances, support from extended kin carried heavier weights than support provided by friends and neighbours. BY and one informant applied the term ‘outsider’ to distinguish the ‘insider’ to describe the situation. In the narrative below, the sense of mastery was under threat from the unreliability and unsustainability of support that a person gained from friends and neighbours.

BY: Relocating is a double-edged sword (here he referred to relocating the whole village from a mountainous area to a plain area). The air is fresh, the place is quiet. But the new place is far away from my brother's house. Although I have a good relationship with my neighbours, they are still the 'outsiders'.

SC: What do you mean by 'outsiders'?

TC: Only a kinship relationship can have such a close connection. Those outsiders (friends and neighbours) could provide help and their concern, to some extent; but they are not as close as your own 'family'. (No.10. BY)

For SQ (75, male), the support and care a person gained from extended kin was uncertain when the bloodline carried on into the next generation:

It is hard to draw conclusions (on whether extended kin might become reliable in the future). If you're referring to siblings, they might provide their help when they are capable. If you're referring to the generation of nephews, they are more distant (than siblings). My siblings are getting old and distant. (No.16. SQ)

The sense of ambivalence on the interpersonal level might be intensified by ageing and physical decline. When a person grows old, the chance of managing connections and receiving support from an informal supportive network decrease. AB's (81, female) nephew described the declining supportive network for his childless aunt:



SC: Does anyone come to visit her (the participant)?

Nephew: They are probably dead. My mother (the participant's sister) died; her friend is over 80. The friend is healthy, I heard, but she's unable to walk very far. I think she's the only person in this village of this age (the participant is 83 years old). (No.7. AB)

Similarly, GG described how difficult it was to carry on the interpersonal relationships he had fostered from childhood. He had a friend who played trumpet when he was a teenager. It was becoming increasingly difficult to visit the 'old peers', although this friend was only 30 minutes' walking distance away from GG's house:

SC: Do you still have any interaction with your friends?

GG: Well, I'm just here (in my house). I have to raise chickens for my nephew and feed the pigs.

SC: Does that mean you don't engage a lot?

GG: We have had some visits. But this year I have hardly ever visited him. I feel dizzy when I get back (the participant has high blood pressure). (No.4. GG)

To summarize, residential mastery could be sustained on an interpersonal level by cultivating social connection and a sense of belonging. However, the mastery experience was under constant threat from one's financial ability, the relational closeness and ageing process. The findings of this research pay specific attention to the ambivalent feeling of mastery, instead of simplifying or flattening it. In fact, the split feeling of connection and constriction confirmed the residential normalcy model, which regards the residential congruence as a complex and split experience that deserves adaptation (Golant, 2011).

6.4 Mastery of ambivalence when voicing one's needs: being practical and feeling disempowered

Moving beyond the individual and interpersonal level, residential mastery was ambivalent in institutions. Some participants mentioned the feeling of ambivalence when voicing their needs in front of the formal supportive sectors (the village collective). They admitted being 'disadvantaged' and 'needing help' but were reluctant to deal with the administrative staff from the village collective. The narrative below



from BY demonstrates the feeling of ambivalence when attempting to balance his self-esteem with practicality:

If the village committee has 'good emotional status' they will probably give me some rice. Sometimes if they don't...the rural area is not like urban places, sometimes if they are not happy, it is hard to tell (laughs). Now if they inform me or give me anything, I will come to the village committee. If they don't, I will count on myself. That's it. (No.10, BY)

One reason why the participant was reluctant to deal with the village collective is that any help that came from the village collective may come with a stigma. The narrative below demonstrates this situation. The village collective visited BY's house to carry out a means test, and during the process the choice of institutionalization was brought up by one staff member. Conflict emerged when BY was uncomfortable with the 'tone' of the staff member (XD):

XD: Another option is, if you'd like to move to an institution. We call this 'centralized support'. It might be better than your current situation.

BY: No, thanks, I have my own house now.

XD: If you insist on staying in your house, like I said before, you have to do what you can. The government provides every kind of support they can, even the house...

BY (becoming angry): I built my own house, they just gave me money.

XD: But you still have to have some gratefulness and be self-sufficient

BY: Sufficient? Yes, I will have some gratefulness, but self-sufficient? I pay my respect to the Communist Party, I do. But it is unreasonable to be 'sufficient', as you say? (No.10. BY)

The conflict was attributed to the narrative of the administrative staff member, who positioned institutionalization as being opposite to self-reliance (doing things by yourself) and subtly questioned BY's capacity and willingness to be 'self-reliant'. BY pointed out that his house had been built 'with his own hands', with some material support from the state. He was outraged when the staff member told him to be 'sufficient', implying that he realized how difficult it was to stay in a rural community. He mentioned the term 'grateful' to counterbalance the term 'sufficient' to demonstrate the insistence on mastery.



Additionally, there might exist misunderstanding and distrust between the childless and the village committee. GS (74, male) complained about the ‘uncertain and irregular’ provision of the state pension and assumed that there existed some ‘dirty secret’ among the village committee about ‘transferring his pension to others’:

When I went to the bank with my card, I had no idea how much money I had left in my account. Sometimes it might be there, sometimes it disappeared. I asked the village committee: why has my money gone? They said: why you come (to withdraw cash) at this moment. I don't know. Later on, when I did my investigation, I noticed that my money had been transferred to others. (No.17. GS)

According to the village committee, it was because YG was a retired soldier, who enjoyed two pension schemes: the Rural Pension Scheme and military assistance. The two schemes belong to different governmental departments, so the distribution of the allowances was in different forms (the rural pension was distributed monthly while the military assistance was distributed seasonally).

From the perspective of the village collective, neither of these situations was ‘intentional’. One village cadre revealed how difficult it was to ‘deal with those folks’ (the childless older adults), which reflects how the participant was ambivalent when ‘dealing with people from formal sectors’:

It is not easy to deal with these ‘special groups’. Sometimes when you want to say something good (to inform them about the choice of institutionalization) and they have a different opinion, they remain silent. You have to pay multiple visits to their house to figure out what they really want.

One staff member from a rural institution confirmed that the providing of community-based service was still in the developmental stage. The proliferation of a ‘targeted poverty-releasing programme’ required the village collective to visit each household of the village, identifying the most ‘poor’ and ‘needy’ households to provide (material and non-material) support. The process was complicated, and sometimes could be slow in some villages. The administrative staff believed that the formal sector would provide ‘better help’ and more vacancies for childless older adults in rural institutions:



We expands our advertising (of rural institutions) in recent years. Especially coming along with a targeted anti-poverty agenda, the village committee knock on every person's door to investigate the household situation. For those who have fallen into poverty due to disease, disability or loss of caregivers, we try to inform them and persuade them to take the option of relocation. Once they meet the criteria and are willing to come, we are very welcoming. The number of beds in this institution are gradually increased. When we have our new district (of institution) developed, I believe there would be more and more childless people coming.

This section echoed with previous evidence identified in rural Australia; that is, the strong insistence on mastery and individual capacity may prevent older adults from voicing their 'true needs' (Winterton & Warburton, 2011). Participants of this research were ambivalent about maintaining a sense of mastery under the dominant social norm of self-sufficiency and self-reliance. Dealing with a village collective might hinder such a sense of mastery and intensify their powerlessness, as some participants were unsure and sceptical about formal sectors. On the other hand, when 'needs' emerged (e.g., when a person needed to apply for a rural pension), being practical became the predominant strategy to gain support from the village collective.

6.5 Gendered mastery and placeless women

Although all participants – men and women – were subject to the feeling of mastery, women's perception of residential mastery was different to that of men. Female participants were more likely to experience multiple relocations because of marriage, which rendered them more reliant on their husband's household to gain resources, such as farmland and a rural house. Ambivalence was driven by women's dependency on marriage and the struggle to avoid patriarchal marriage and to be self-reliant.

When a young couple established a new household in rural China, it was usually the bride who relocated to the groom's household. Most of the female participants (except for GG, who had never married and inherited her parents' house and farmland) had married and experienced multiple relocations: from their own home to their husband's home; and some of them had been relocated several times due to divorce/separation and remarrying. AB, 83, married a disabled yet brutal man who



always ‘beat her badly’. AB’s sister was outraged, taking AB home to stay with her and her three boys. AB’s nephew, the third son of her sister, became AB’s caregiver after that. AB was mentally disabled during the interview; her nephew confirmed this situation:

She (the childless participant) married a dummy who abused her. After she came back to her home, she stayed with us. My oldest brother took care of my father, the second brother took care of my mother. The village committee suggested I take care of her, my aunt. I think it’s a good idea. (No. 7. AB)

Marrying the ‘wrong man’ imposed great suffering on rural women, triggering their feeling of powerlessness and an attitude of resistance at the same time. The narrative below illustrates YY’s story about ‘suing for divorce’:

YY: I’ve been married for over two decades, and he (the ex-husband of the participant) just followed whatever his mother ordered. He sold my pig; his brother took my firewood. When we established our own household (from the house division) we had nothing. We had to borrow rice to feed ourselves. My mother-in-law despised my mother (the participant’s mother is a blind woman), so she told her son to abuse me. There was one time when I sold a pig, he took the money and handed it to his mother. I went to pee, he followed me and beat me... brutally, all the way until I ran out into the street.

SC: Why?

YY: He thought I had stolen the money. I told him I knew nothing about the money, and he beat me. I ran home in tears, then he found the money himself. He hid the money from me and forgot, I was innocent. I was furious, I was... the time I spent with him was full of suffering and anger. (No.25. YY)

Several informants illustrated the old saying of ‘a woman ought to follow her husband no matter what his lot is’ (*jia ji sui ji jia gou sui gou*). When getting married, rural women were excluded from inheriting the rural house and farmland of her own family. Consequently, women had to be extremely ‘reliant’ on their husband’s household to gain resources. Two village cadres confirmed the discriminating succession rule during their interview:



Village cadre one: This is our rule (of allocating farmland): a married girl could inherit some land from her parents, but she cannot establish her own household... you have to combine your household certificate with your relatives, usually your parents or husband. The farmland allocation depends on the amount of people in each household.

Village cadre two: Yes, because the land belongs to the village collective. The total amount of farmland is stable, the population is expanding. If you get married outside the village, for example, inheriting the farmland from the original village (from the girl's parents) would be difficult to calculate...because you are not here (meaning, because the girl has to stay with her husband).

Because women were not allowed to establish their own household, their farmland had to be inherited from either her parents' household or husband household. At the same time, because rural parents tended to leave their properties to their son rather than their daughter – to let the 'male inheritor carry on the family name', women had limited options and relied on their husband and offspring – preferably their son to enhance their status (Djundeva et al., 2018). If they became childless unexpectedly in later life, rural women who were excluded from property inheritance were more vulnerable than their male counterparts. HM, a woman who was biologically infertile, described her experience of farming alone, with tears in her eyes:

SC: Before relocating to the rural institution, how did you arrange your life?

HM: The government gave me food and cash subsidies. RMB200 or 300 per month; 100 kg of rice per year. Sometimes they gave me 5 or 6 kg meat at the Spring Festival...our working team has only 300m² of farmland per person. I have to deal with the land, all by myself. (in tears) I stayed in my house, (the suffering) was hard to describe. (No.24. HM)

When applying a feminist and critical lens, this research reveals how the sense of mastery was different for childless men and women, even though both groups faced multiple layers of oppression such as poverty, a lack of support and ageing. The structural discrimination against women rendered them overly reliant on the domestic domain, preferably their husband and son, to gain support and care. This research uses the term *placeless* to describe this situation; that is, a woman was culturally and legally deprived of property inheritance and accelerated wealth. When lacking their children's



care – one of the only resources rural women could count on in later life – women were more likely to be powerless than men.

6.6 Summary of this chapter

This chapter has explored how childless older adults assess their current residential condition through the lens of residential mastery and comfortable. Living in a rural area is a mixed experience encompassing simultaneously self-reliance, stress, constriction, disempowerment and suffering – what has been labelled *mastery of ambivalence*. Besides, the (ambivalent) mastery experience could be conceptualized into three levels. On individual level, even farming activities enabled sense of mastery, the experience encompassed uncomfortable aspect such as stress, suffering and the fear of losing one's farming capacity due to age-related decline. On interpersonal level, the social support participants received from villagers was accompanied with feelings of constrain, which was further susceptible to one's financial resources and the relational closeness with the person who provided help. On institutional level, the ambivalence emerged when childless older adults attempted to seek for instrumental support from village committee. They worried the situation of 'disadvantage and 'in need' could intensify stigmatization of uselessness, but the practical needs for instrumental support was too tangible to be suppressed. Overall, childless older adults struggled to maintain mastery while constraining by a matrix of individual and rural disadvantages such as ageing, poverty, insufficient development of the market economy, reciprocity on interpersonal relationships and a shortage of formal support. This findings challenged the assumption of the residential normalcy model, which assumes that residential congruence is achieved when older adults fall into the comfort *and* mastery zone (Golant, 2012). This research concludes that, for childless older adults living in constrictive rural area, residential mastery was the core experience during the reasoning process.

Specifically, this chapter provides three arguments to understand the (ambivalent) sense of mastery among participants. Firstly, residential mastery is a sufficient condition for achieving residential congruence. Although this research confirmed that residential mastery and comfort were distinct experiences during residential assessment, sense of mastery is the key to achieving residential congruence. This argument did not



imply that the residential comfortable experience was not important. Instead, it was adaptable. Participants recognized that some aspect of rural landscape could contribute to comfortable experience, such as the ‘warm weather’, ‘mountainous view’ and ‘quietness’ when living alone. However, living in such pleasant circumstances did not smooth the tension, the fear and the stress of surviving, that is, ‘a person has to farm to gain food’. A person has to take adaptive strategies to ensure (even the minimal level of) residential comfort through manifestation the mastery experience. The adaptation strategies would be elaborated on next chapter. This finding support Golant’s (2012) argument of, ‘sense of control lays in the central position of residential congruence’ (Golant, 2012). In constrictive rural area such as Yunnan, the extend childless older adults could maintain mastery, to feel control and to sustain daily life was prioritized over other experience.

Secondly, the sense of mastery that childless older adults struggled to maintain encompassed the feeling of ambivalence, which was further embedded in the disadvantageous position of childlessness in rural China. For example, farming in constrictive rural areas involved with intensive engagement with physical capacity, which was stressful and full of suffering. Reaching out for support from villagers was constrained by the expectation of reciprocity, which was further shaped by the physical and financial abilities of childless older adults. The stigma of uselessness, which prevented participants to voice out their needs reflected a cultural discourse that ‘Chinese farmers has to be self-reliance to demonstrate their dignity’. Taking the critical perspective, this chapter emphasized that, the ambivalent aspect of mastery – the stress, suffering, constrain and disempowerment was interlocked with the structural disadvantage of rural ageing in China’s society.

Finally, the mastery experience was gendered. Although both men and women were subjected to a matrix of uncomfortable experiences, childless women were more vulnerable than men while staying in a rural community. They may encounter domestic violence and multiple relocations as a result of marriage. They gained limited resources from their own household, and hence had to rely heavily on the domestic realm – preferably the husband’s ‘kindness’ and the son to guarantee later-life wellness. Such structural discrimination against women was embedded in the patriarchal expectation



of rural China that a girl was ‘born to become someone’s wife’ (Zhang, 2007). When they became childless for any reason (usually biological infertility or loss of children), childless women usually had fewer material resources, such as farmland or a rural house, to count on and to use to negotiate support from kin. In other words, rural women may become ‘placeless’ if they become childless. The mastery experience had different meaning for childless men than for women.

Residential reasoning was a negotiation process that encompassed residential assessment, adaptation and decision-making. The next two chapters provide detail on how participants adapt in order to AIP, as well as converse cases when participants struggled to adapt and ended up in rural institutions. In conclusion, this chapter has argued that residential mastery is the key experience and a precondition in sustaining congruence for childless older adults. It eventually echoes the findings from the previous chapter that childlessness in rural areas is an undesired and disadvantageous situation that requires adaptation.



Chapter Seven

Enduring *Ku* and Residential Decisions

The previous chapter revealed that mastery experience was central during residential assessment. This chapter focuses on how childless older adults adapt to achieve residential mastery and how such adaptative effort influences their residential decisions. Overall, participants applied the term *ku* to explain their choice of residence and the process of *enduring ku* to describe their adaptive efforts. This chapter first introduces the meaning of *ku* in narrative form, then illustrates how childless older adults struggle to endure *ku* in order to choose AIP. This chapter identifies three levels of adaptation specifically, which respond to the behavioural, psychological and existential level of adaptation, respectively: (a) *routinizing ku*, (b) *positively reappraising ku*; and (c) *transferring the narrative of ku into toughness identity*. Eventually, this chapter concludes that if a person can endure *ku*, the choice of AIP is more desirable than institutionalization.

7.1. Defining *ku* in narrative form

Participants referred to the term *ku* to describe the choice of AIP or relocation. This concept was firstly brought up by TS (63, mle), the first interview participant, and was repeatably mentioned by the upcoming participants. As the data analysis continued, this research revealed that the narrative form of *ku* has two layers of meanings: (a) *ku* as a verb to describe the behaviour of ‘doing stuff’ such as farming, raising chickens or doing housework; and (b) *ku* as an adjective to describe the negative emotional status of being stressful and suffering. For example, as LF (74, male) suggested:

This is how farming activities work, ku...works are endless. I don't have any time for rest. (No.21. LF)

Similarly, AB's nephew confirmed that:

If you don't ku, what do you eat? Ku enables food. (No.7. AB)



When applying *ku* in a narrative context, it contains the negative feelings of stress and suffering. LF remembered his experience of suffering during the collectivism period:

In the collectivism period we combined three villages into one. It took two hours from one village to another...four hours of walking per day. I arrived at the farmland before sunrise and farm. it was difficult. People starved and dead. It was ku (suffering). (No.21. LF)

Similarly, YY (76, female) elucidated her experience when staying in a rural community:

I was ku (suffered) everywhere. I was ku since I was young. When I moved to stay with my second husband, I ku like hell. I did basically everything. Raising chicken, pigs, cooking...I did everything. (No.25. YY)

Ku means ‘doing stuff (mainly farming but not limited to farming) in a suffering and stressful way’. At the same time, it represented a mastery experience where childless farmers were eager to depend on their own physical and psychological strength to make a living in a rural area. The narrative form of *ku* encompassed the negative emotional status of being stressful and suffering – an experience of discomfort that required adaptation.

A person has to endure *ku* to make a living in a rural area. It is an ongoing process of negotiation, adaptation and struggling. Childless older adults demonstrated strong resilient characteristics to endure *ku* – to manifest residential mastery. At the same time, they were also constrained by physical, psychological and social resources. The next section illustrates in detail how participants struggled to endure *ku* in order to achieve residential mastery.

7.2 Enduring *ku* as ongoing negotiation

7.2.1 Routinizing *ku* in a flexible way



Farming and farm-related activities were overall routinized and controllable. Childless older adults engaged with daily activities such as cleaning the house, farming and raising chicken to fulfil their everyday schedule. Relocating to rural institutions, on the other hand, may result to interruption of their routine. TS described his daily schedule as follows:

I have a cow to take care of near my farmland, and I need to harvest grass (to feed the cow) ... I go to the farm every day by walking. I leave home at 7 a.m., eat lunch at about noon, leave at 1 or 2 p.m. and come back around 8 p.m. This is basically my everyday schedule. (No.1. TS)

The routine *ku* was routinized yet not rigid. The schedule was implemented in a flexible way in which participants made decision on when to move, what to do and when to stop *ku*. The anticipated institutionalization was pictured as a prospect of losing autonomy. TL stated:

If there's not a lot to do on farmland, I could go home.... If I relocate (to an institution), the institution wouldn't let me even go out. I'm used to farming on farmland, not being regulated. (No.3. TL)

Similarly, CC (69, female) rejected the proposal of institutionalization and 'working outside the village' (as an immigrant worker) in order to sustain her autonomy:

If the sun is extremely hot, I can stay at home. If I work outside the village, there is no way you can avoid the sun...I get up extremely early, going to bed before 12 in the moonlight. (No.9. CC)

Conversely, if the routinized *ku* is interrupted by unexpected events (usually a decline in health), childless older adults are usually left with no option but to relocate to an institution. HM (83, female) was once sent to a rural institution by his nephew, and it was 'difficult to get used to it'. HM returned to her house to continue *ku*. An accidental fall interrupted her routine *ku*, after which she was sent to a town hospital by her niece. HM still attempted to *ku* after returning from hospital, but the sequelae of the fall were troublesome and disturbing. The village cadre noticed the situation and suggested that HM should relocate to an institution. HM hesitated for a few days,



eventually deciding to move. The choice of institutionalization was not absolutely voluntary because the fall was ‘not in the plan’. Life after institutionalization was ‘barely acceptable’ and routine:

I get up before sunrise, have breakfast when the staff are ready to prepare it. After eating it is TV time or sleeping time. If I feel good, I will walk around or go to the market. (No.24. HM)

Everyday routine may include ‘unhealthy hobbies’, which were also regarded as ‘a small everyday treat’ by participants. Activities such as smoking, and alcohol consumption constituted an important aspect of daily life and a ‘good reason to stay’. CC revealed her reluctance to stop drinking alcohol:

I have cholecystitis, I can't drink alcohol or eat chicken or eggs. But...well, occasionally, I drink some alcohol (laughs). I buy the local-made alcohol from the local grocery. (No.9. CC)

This theme illustrates how childless older adults took their behavioural strategy from routinizing *ku*, implementing *ku* in a flexible way and maintaining ‘small treats’ to gain a sense of mastery and control. Conversely, when everyday routine was interrupted by unexpected change (e.g., a fall), childless older adults had to relocate to institutions for advanced support. The process may have possible (long-term) negative consequences, such as maintaining ‘small, unhealthy hobbies’. This research does not suggest that childless older adults should bear the blame for their choice. Rather, lifelong exposure to a constrictive environment may reinforce the sense of loss and amplify the pleasantness of an ‘immediate treat’ (Bruch & Feinberg, 2017). It is possible that participants applied such a ‘treat’ to regain their mastery.

7.2.2 Reappraising the negative aspect of *ku*

Childless older adults transferred the uncomfortable experience into the new idea of: ‘the dissatisfied aspect was unimportant’. In such a perspective, they release the tangible feeling of stress, suffering and discomfort, focusing on the day-to-day immediate *ku* to avoid the anticipated anxiety of farming activities. Residential congruence was achieved when the overall perception of positive and mastery were



fulfilled.

Normalizing the negative aspect of *ku* as a ‘way of living’ became one of strategies of participants. They reappraised the environmental expectation as ‘unimportant’, hence the negative experiences such as a degraded house, leaking roof, distant farmland or poverty were not tangible. For example, as TL (65, male) suggested:

If we’re discussing about basic material, such as food and clothes, I would say ‘barely enough’. My life is not extremely difficult, but I have difficulties. I just get used to them. I cook the things I grow; if I don’t have food, I’ll go to the market. (No.3. TL)

Similarly, SP (76, female) suggested:

You just ku (farm) for whatever is available and eat it. (No.2. SP)

The residential aspiration was reappraised because of financial constraints and increasing age. Any improvement in life (e.g., starting a new hobby or visiting distant friends) became unrealistic. Childless older adults had to take control over their present life to maintain mastery. When invited to describe his future plan, TL responded:

SC : What’s your plan for the future?

TL : I’m too old to have any ‘plan’. Even if I wanted to, I cannot. I don’t have the energy (to implement it).

SC: Like what, for example?

TL: (laughs) Oh no, I want a lot of things but I cannot. I just need to focus on my farmland and guarantee my life. (No.3. TL)

At the end of his interview, TL admitted that he would like to raise his roof so that his house could contain more firewood. ‘Other than that, no way,’ he insisted. ‘I don’t think that I have the resources to do more.’

The residential discomfort was unimportant when it became an ‘common issue among villagers’. Comparing with others – especially the imaginative figures who were



‘experiencing the same level of suffering’, their own *ku* was less tangible. As TS (63, male) suggested:

It's difficult to say.... If you say, 'It is bad' ...to be honest, there are places that are worse than here. But we do live better than those who live in the foothill areas...We are just too poor....everyone is ku (suffering) in the village. We are worse off than some, better off than many other individuals.
(No.1. TS)

The sense of constriction overshadowed the whole interview process, shaping the participants’ narrative. Most of the childless participants described how they focus on the day-to-day *ku* to avoid making a long-term, unrealistic residential plan. The life attitude of ‘making everyday count’ was fostered to maintain control and competence. For example, BY (65, male) commented:

I don't have any plan. I'm old and sick. Let's be practical. I will cherish my health and taking care of my body. I'll live for as long as possible. That's the plan....if you say so. I'm 65 years old already.
(No.10. BY)

Similarly, TTC (73, male) chose to focus on the day-to-day *ku* due to the uncertainty of death:

I'll die soon, what's the point of relocating? ...I'd rather die in my own house. Or maybe I'll die tomorrow, who knows? I've stayed in this house for quite a long time. Why do I bother to move to institutions? (No.12. TTC)

Overall, the narrative demonstrated how adaptation was full of uneven process, and how it was susceptible to increasing age, material constriction and uncertainty. When reappraising the discomfort aspect as unimportant, participants did not deny that the stressfulness, suffering and discomfort of *ku* were inchoately related to, and embedded in, rural ageing. This argument is in line with previous studies, which suggested that older adults tended to marginalize their negative experience during residential assessment (Hillcoat-Nallétamby & Ogg, 2014). For those who didn’t regard enduring *ku* as being solvable, the choice of institutionalization remained a way to escape. As LF

(74, male), an institution dweller, remarked:

It's just what farming activities look like, ku. The work is never ending; having any rest is impossible. I'm done with ku and farming. I'm over 70 years old, I don't have such strength. I'm sick. (No.21. LF)

7.2.3. Transferring the narrative of ku into toughness identity

Childless older adults struggled to make meaning from *ku* (farming and conducting farm-related activities), which we called *existential strategies*. The term ‘existential’ referred to issues about meanings and values in life triggered by the life trajectory (Lloyd et al., 2016). Participants from this research transferred *ku* as a positive identity of toughness and hardworking, legitimating the choice of AIP to sustain such identity. They also accommodated such identity into the narrative on social progress to sustain individual and autobiographical mastery. Through adaptation, enduring the negative side of *ku* had its ethical appropriateness.

Enduring *ku* signified a ‘surviving strategy’ where a person had to dedicate substantial labour strength to guarantee food and basic surviving. ‘*Ku* enabled food’ or ‘to *ku* to guarantee life’ was frequently mentioned by participants. Following this argument, anticipated institutionalization was perceived as ‘harmful’ to one’s self-reliance, laborious and tough identity. BY justified his insistence on AIP as follows:

A tree can be cut for firewood. The electric company provided me 15 kilowatts of electricity for free. I can buy one bag of rice for RMB60 (USD9) and save RMB300 (USD45). I grow vegetables by myself (Interview 1). I think the most important thing in this society is to be self-reliant. I don't want to rely on any person. A phoenix in distress is worse than a chicken (Interview 2). (No.10. BY)

Interestingly, mastery narrative of *ku* was emancipatory. Participants connected their own autobiographical *ku* with the discourse of social progress economic and growth and, then legitimating their experience of *ku* as a culturally appropriate identity (*ku* as a ‘good farmer’). Since the 1950s, the People’s Republic of China has advocated



the political movement of *suku* ('talking about bitterness') to mobilize low-income peasants to share their stories of misery in China's former society, which, in their narrative form, contrasted the present society that the Chinese Communist Party has established (Zhou, 2015). *Ku* remained a linguistic framework for farmers to organize, and to vocalize their (suffering) experience. In line with these findings, participants of this research accommodated their autobiographical *ku* with the discourse of economic growth and social development:

The village had a shortage of water supply during the collectivism period (1960s). Farmers need to dig channels and threshed our grain. We just had firwoods; electricity was beyond imagination. Even if you were willing to farm, there was limited chance that you may feed yourself. Now you ku for yourself, farming for your own pocket. It's a good time. (No.1. TS)

From this perspective, the negative aspect of *ku* was gradually reduced by social development and economic growth. By living in such a 'developing' society, participants were able to gain a sense of mastery. The choice of AIP, therefore, reflected the cultural appropriateness of toughness and laborious farming:

The more I ku (farm), the more confident I am. I ku to improve my life. Only lazy people would consider moving to rural institutions. (No.1. TS)

There was one participant estranged himself from this grand discourse of social development. He relocated to a rural institution with the perception of being 'flawed' and 'incompetent' someday:

The economy is good, and money is easy to earn. But I'm a flawed person. I planned this moving long several years ago. (No.23. HH)

HH (60, male) was born with stoop. Years of labour and harvesting enabled him to be 'barely feed the stomach'. The social development and economic growth were tremendous, as farmers could 'have some grains left' every year. However, the choice of relocation remained a way to minimize the anticipated risk of ageing and his physical disability: "I'm here because I am old and disabled". The discourse of economic growth



did not enrich the meaning of *ku*.

Despite the age-related difficulties, participants struggled to make meaning from *ku*. YZ (69, male), as stroke survivor, endured *ku* with great pain: “I didn’t even know the date today... *curse ku!* I cannot do anything.” YZ also rejected the relocation plan, as the anticipated relocation may render him ‘disgraceful to others’, especially the staff in institutions:

If I move to institution, the house will belong to the village collective, the land will belong to the country. No. I would rather have an eye on my degraded and leaking house... If I move, I'd be regulated. It would be a disgrace. (No.6. YZ)

YZ’s and HH’s examples revealed how the mastery narrative was sustained in an uneven, struggling and messy way. On the one hand, enduring *ku* transcended into positive identity of toughness and was reinforced by the socially constructed discourse of economic growth and social development. On the other hand, such a narrative prevented them from seeking advancing support when needed, binding them to the rural community with all its potential risks and challenges (Golant, 2015; Winterton & Warburton, 2011). Accepting one’s prospect of incompetence took courage and effort. For some participants, it could take months or years of adaptation until the choice of institutionalization became ‘urgent’ and unavoidable (Koss & Ekerdt, 2016).

7.3 Summary of this chapter

This chapter explored how childless older adults struggled to adapt in order to achieve residential mastery and how such adaptation leads to different residential decisions. Residential decisions were made based on the perception of *ku* – that is, to conduct farming and farm-related activities. If childless older adults could endure *ku*, the choice of AIP was more likely to be preferred. Conversely, if a person perceived him/herself to be overwhelmed by *ku*, institutionalization was more desirable.

This chapter incorporated three main findings. Firstly, it brought up the term ‘*ku*’ to summarize the experience of childless older adults and their way of making



residential decisions. The narrative of *ku* in a rural context covers a variety of farming and farm-related activities, which were under threat of environmental constrictions, increased age and functional decline. *Ku* in narrative form reflects the ambivalent feeling of farming, covering self-efficiency, self-reliance and stressfulness, suffering and discomfort. Essentially, *ku* represented a form of mastery that childless older adults were eager to preserve.

Secondly, enduring *ku* in a rural place required substantial effort and adaptation. This chapter identified three levels of adaptation for childless older adults to achieve residential congruence and mastery. By routinizing everyday farming activities, reappraising the negative experience as being less important and giving meaning to farming activities, childless older adults were able to maintain congruence and wellness, preferably choosing to stay. Conversely, if adaptation did not sustain residential mastery, it was more likely that participants would choose to relocate. This finding also underlined that enduring *ku* and making the residential decision was a gradual, negotiation and adaptive process rather than momentary event. Some participants may seemingly not be endurable to the intensive farming activities due to age-related decline, but they applied a variety of strategies to adapt to such difficulties until more urgent and advancing needs emerged. Some may be physically and mentally able to endure *ku* but still choose to move to rural institutions in preparing for the upcoming health decline and support deficit. This research did not attempt to encompass the complex experience of enduring *ku* and the residential decision-making into linear relationship. Instead, enduring *ku* or not being able to endure *ku* was a subjective, complex and dynamic experience for childless older adults to evaluate, to justify and to reason their residential decisions. Echoing with the core argument of residential reasoning model, ‘the residential assessment, adaptation and decision is an ongoing negotiation process’ (Granbom et al., 2014).

One interesting finding of this research was that participants seemed to attach the capacity to *ku* to individual value and historical continuity. Participants accommodated their autobiographical mastery into the grand narrative of economic growth and social development in China, seeking the meanings and moral appropriateness of their farming behaviour. This phenomenon was attributed to the specific socio-economic



context of rural China, where economic growth has been rapid (Liu et al., 2017). The ‘progress discourse’ sharpens the comparison between past *ku* and the contemporary *ku*, legitimizing the mastery narrative of contemporary society while adding meaning and strength to farmers’ positive identity.

Finally, adaptation and structural constriction coexisted. This chapter paid particular attention to the struggling and uneven side of adaptation, unfolding how resilient characteristics of childless older adults were created, shaped and constrained by their disadvantage. The findings of this research echoed with the argument of the previous chapter, that is, adaptation was ambivalent. For example, when the participants focused on the immediate, day-to-day *ku* to gain a sense of control, it reflected a broader concern in which ageing in the constrictive rural context was inherently uncertain and constrictive. When manifesting available (cognitive, behavioural and existential) resources to adapt, the mastery narrative reflected resilience on the one hand, rendering the disadvantage of childlessness and their voice of struggling invisible at the same time.

Overall, this chapter concludes that childless older adults decide where to grow old based on the perception of *ku* – a mastery narrative that is related to farming and its related activities. Being able to endure *ku* requires substantial adaptation. Childless older adults struggle to apply the psychological, behavioural and existential strategies to sustain such a mastery narrative.



Chapter Eight

Having Someone at Home and Residential Decisions

The previous two chapters revealed how the residential mastery experience was centred around residential assessment and decision-making. This chapter pays special attention to the interpersonal level of residential mastery and its impact on residential decisions. In line with the basic assumption of the residential reasoning model, receiving support and care from the significant other(s) was an ongoing negotiation process that encompassed both resilient characteristics and struggling experience. This chapter first reveals the identity of the significant other(s) for childless older adults – in the narrative form of participants, ‘someone at my home’. Thereafter, this chapter reveals three types of negotiation between childless older adults and their significant other(s), which enabled the childless older adults to feel certain and mastery in order to choose to AIP. In conclusion, this chapter argues that ‘having someone at home’ was a special situation that differed from what previous literature identified as ‘receiving support and care from informal network members’. It indicated a caring relationship that guarantees the intensiveness and durability of care when the childless older adults need it, and the status of ‘having someone’ was dependent on one’s financial, social and cultural resources while being shaped by the mainstream idea of filial piety.

8.1 ‘Having someone at home’: revealing the significant other(s) of participants

The significant other(s) was found crucial when deciding where to grow old – what was framed as ‘whether a person has someone at home’ by participants. ‘Having someone’ is different from ‘receiving support from neighbours, friends or any other kin members’ – an experience commonly illustrated by almost all participants (see Chapters Five and Six). Instead, it is a special situation that guarantees the intensiveness and durability of care from extended kin.



Out of all 27 childless older adults, 13 of them received support from the significant caregiver and choose to AIP; four of them chose to AIP but didn't specify the significant caregiver. Among the institutionalization group, one of the participants had received support and nine of them perceived themselves as 'having no one at home'. Noticeably, the extents of enduring *ku* and 'having someone at home' were not mutually exclusive. Receiving care from the significant other(s) could reduce the magnitude of *ku*, that is, the stressful and suffering side of farming and its related activities. It was also possible that a person could attribute the residential decision to the extent to which he/she could endure *ku* while relying on 'someone' to get daily care (e.g., SP). This research defined the status of 'having someone at home' as when either the childless participants or the significant other(s) mention the caring relationship as '(at least) one reason to stay put'. Otherwise, for those who did not mentioned the figure of 'someone', this research regarded this person as 'having no one at home'.

This section reveals two sources to require long-term care from 'someone': (a) the significant other(s) from a vertical kinship tie, (b) the significant other(s) from a horizontal kinship tie;

8.1.1 Designating the significant others from vertical kinship tie

It was possible when childless older adults received support and care from their niece/nephew or in-law (usually daughter-in-law). Eight participants fell into this category.

In rural Yunnan, it was customary for aged parents to be 'assigned' to their adult children's (usually son's) household, *separately*, to receive long-term care, which spread the caregiving burden to each child and maximized the division of labour with aged parents (see Chapter Five). Based on this arrangement, it is possible when one adult child (usually the third son) was free from any care responsibility for his own parents, thus could provide long-term care to their childless uncles or aunts. GG (73, male) fell into this situation:

My caregiver's own father stayed with his old brother; and his mother stayed with the second brother.



He is the third son of the family. He persuaded me several times: you're all by yourself now. Me and my wife would like to repair your house so that you could live with us. (No.4. GG)

AB, similarly, was taken care of by her nephew (her sister's third son). AB married a disabled yet brutal man, who constantly abused her. AB's mother was furious about her daughter's suffering, persuading AB to come back home after a few years of marriage. The prospect of AIP was confirmed by AB's nephew:

We had three older members in our family: my parents and my aunt. My two brothers took care of my parents, and I would take care of my aunt. ... We also prepared the coffin and grave for her. (No.7. AB).

Only one participant counted on in-laws for support. SP, 76, co-resided with her daughter-in-law since her only son had died from an accident. The prospect of caring was confirmed by SP's daughter-in-law:

Of course, we (the in-law and her new husband) would take care of her...she's our family member. If it's not us, then who would do such a thing? (No.2. SP)

'Having someone at home' was not synonymous with 'receiving support from an informal supportive network'. As revealed in Chapters Five and Six, it was common for participants to 'pay some visits to neighbours, friends or extended kin' or 'offer and receive help to and from extended kin' on tasks such as farming, digging channels or exchanging farming tools. Having someone at home specified a situation where the long-term care that significant others (usually extended kin) provided was intensive and durable; it covered tasks ranging from provision of food, clothing and medical support when needed and funeral preparation. The following narrative demonstrates how BY (63, male) and the village cadre clarified the caring boundary of 'someone', which he referred to as 'our own people' and 'outsiders' – people who may provide help but may not be reliable when needs emerged:

Although I have a close relationship with my neighbours, at the end of the day they are the 'outsiders' ... If I put it straight...everyone is busy on their farmland. Even if something unexpected



happened to me, they might not answer the phone. (No.10. BY)

8.1.2 Designating significant others from horizontal kinship tie

Six of the participants specified their siblings or cousins as their significant caregiver. Five participants received care and support from the significant caregiver due to a ‘taken-for-granted family obligation’; one participant had resigned a legal contract with the significant caregiver to designate care obligations and rights. TS (63, male), a never-married man, co-resided with his two never-married brothers for decades. When his younger brother and sister got married, TS and his third and fifth younger brother inherited their parents’ house. TS and his two single brothers supported each other with farming, household chores and medical needs. He even planned to count on his two brothers when his health declined:

I have five siblings; three brothers among them didn’t have the chance to find wife. My sister married into another village...I will rely on my three brothers for the rest of my life. (No.1. TS)

Similarly, JZ, 64, a mentally disordered man, had been living with his sister (68 years old) since he was born. From the narrative of JZ’s sister, taking care of the disabled brother and ‘burying him when he dies’ was her ‘taken-for-granted’ responsibility:⁹

He didn’t learn to walk until the age of 10. He didn’t have any teeth. He was the only son and inheritor in our family. We (the caregiver and her husband) raised him until he was 64 years old...as for the future, I’ll feed him whatever we have. If I die, my children will deal with his issue. It’s their responsibility. They have to shoulder it. (No.15. JZ)

Some participants even had a legal contract with the significant caregiver to guarantee the prospect of AIP. TTC, 73, a never-married man had signed a legal contract with his extended cousin, whom he considered was financially accountable and ‘biologically related’. According to the contract, TTC’s cousin would provide care

⁹ In rural areas in China, ‘arranging one’s funeral when he/she died’ signified the most filial and responsible behavior the caregiver was expected to provide.

‘until his dying breath’. In exchange, TTC’s land and rural house would belong to the cousin when TTC died. The cousin confirmed this situation:

Some people in this village encouraged me to take care of him (TTC)...and he chose to stay with my family ...it was confirmed by the legal department of this village. When he died, his land and house will belong to us. (No.12. TTC)

8.2 The ongoing negotiation: How ‘having someone at home’ influence the residential decision

Having someone at home was an ongoing negotiation process that involved a complicated interpersonal dynamic between childless older adults and their significant caregiver. The next section presents detailed accounts on how receiving support from the significant caregiver influences residential decisions. There were three ways in which the significant caregiver could establish a long-term relationship with childless kin, hence ensuring the prospect of AIP: *(a) continuing the relational closeness, (b) formalizing the caring relationship; (c) implementing the filial responsibility.* Conversely, if participants didn’t fall into either type of negotiation, they were more likely to choose to relocate.

8.2.1 Continuing the relational closeness

Two participants perceived ‘having someone at home’ and chose to AIP due to the close relationship with the significant other(s). The intimate relationship may have been established for decades, and was intensified due to day-to-day interactions with, and assistance from, the significant other(s). For TS, for example, the prospect of AIP was *apparent* as he had to take care of his two single brothers. Mobilizing his brothers for farming, raising pigs and picking wood constituted one segment of TS’s life that enabled a sense of connection and mastery. When invited to describe future expectations, TS mentioned the term ‘solidarity’ several times:

Nothing is worth worrying about when I stay at home. My brothers will take care of our cow. All I have to do is watch TV or do nothing, depending on my wish... I call this situation ‘solidarity’.



'Taking care of my own family', this is my definition of solidarity... I'm old. There's nothing I can control. I would only consider my brothers. (No.1. TS)

CC (67, female), similarly, had established a close relationship with her sister's household since the 1950s. After her father died, CC's mother raised CC and her sister under 'extreme difficulty'. CC's mother died from an accident; her brother-in-law (sister's husband) provided help with the funeral arrangements. CC was illiterate, so she had to count on her niece to manage her bank account, withdrawing cash and addressing the pensions for her. The prospect of 'having someone at home and staying in the rural community' was also articulated, although with a subtle sense of uncertainty encompassed within her narrative:

SC: If you have any disease, what's your plan?

CC: My niece will take care of me.

SC: Have you discussed your plan with her?

CC: She bought me clothes and food. She always visits me. I believe she would take care of me.

SC: Have you told her what you expect?

CC: No...but even though I haven't (told her), she would take care of me. She visits me every Spring Festival, buying me food and clothes. She also invited me to stay with her family...She took care of me when I was sick...she buys three sets of shoes every time, one for me, one for her father, and one for her mother... She is concerned about me (No.9. CC)

Echoing the findings from rural Belgium (Volckaert et al., 2020), this category revealed how relational proximity remained an important indicator to evaluate the extent of 'having someone at home'. Although not all participants revealed their expectation (e.g., CC), the close relationship cultivated from daily interactions enhanced one's sense of mastery and confidence, thus increasing the odds of choosing to AIP over institutionalization. Conversely, if the relationship with (potential) significant other(s) was distant or complicated, institutionalization became more likely. WL, 69, relocated to a rural institution with her husband. She was disappointed in her husband's kin – the mother-in-law who accused her of stealing their wood; and the brother-in-law who provided limited help to her household when needed. The familial conflict has a long history; it was deduced that it might be related to WL's infertility.

To avoid further interaction, WL and her husband moved out after five years of co-residing with the husband's family. When they turned 60, the village cadre encouraged WL and her husband to move to an institution. Her nephew, who once promised to take care of the childless couple, changed his tone and encouraged them to 'take the chance'.

DH's (66, male) narrative revealed a similar sense of disappointment. Growing up as an orphan, DH was outraged by his older brother, who 'abandoned DH and his younger brother for his own wellness'. When the village cadre persuaded DH to move, he agreed, saying that he had no one at home:

I'm not getting along well with my own brother! We are so different! My brother has three kids, but we don't talk...His family are mean... they never care about me...When the village cadre encouraged me to relocate to the rural institution, I agreed. (No.18. DH)

8.2.2 Formalizing the caring relationship

Some participants offered their residual land and rural house to the significant caregiver in exchange for long-term support, formalizing the caring relationship on legal confirmation or oral agreement. Four of them fell into this category. The prospect of AIP was ensured when they realized or assumed that the 'one' would guarantee their upcoming wellness in the rural community, no matter how age-related challenges may undermine such a prospect. For example, TTC signed a legal contract with his cousin to secure the prospect of AIP (see the narrative above). SQ, 75, considered his farmland as 'leverage' to acquire the prospected long-term support and care from his nephew. The village cadre explained the arrangement as follows:

It is common sense in rural areas; that is, if you stay in a rural community (instead of relocating to an urban area), you should take care of the elderly members. Relatedly, you could have their house and land. This arrangement was clear; it wouldn't usually involve any complicated conflict. If you provided care, arranged the funeral for kin, you would have the property.

As the interview progressed, this village cadre acknowledged that a complicated personal dynamic might emerge:



Sometimes people go to court to deal with such conflict. According to inheritance law, personal property of older adults can only be passed on to the caregiver if they provide care. That means, at least arranging the funeral (for the childless person's kin).

Some participants only assumed that the (potential) caregiver would *provide help when needed* – yet any revealing of such an expectation or assumption may appear complicated or inappropriate. YZ, 69, considered his farmland and house as the ‘leverage’ to acquire support from his nephew’s family. The relationship between YZ and the significant other, the nephew’s family was ‘terrible’, as ‘he (YZ’s younger brother) took basically everything during house division’. YZ was unsatisfied when he had to compromise as the older brother, letting the ‘younger do as he wished’. In 2017, YZ’s functional capacity was undermined by a stroke. He was unsatisfied with his nephew’s ‘irregular help’. Nevertheless, YZ was confident about the ‘leverage’ he had and certain about the prospect of receiving support:

I'll have my brother and nephew to take care of me if I get sick. I told my nephews, 'If I become weak for someday, if any of you could provide care to me, my house and land would belong to you.' That's why I don't want to move to institution. (No.6. YZ)

Many participants mentioned how such a conversation on long-term caring expectation happened in an informal and casual situation, meaning that the prospect of receiving care and AIP was eventually uncertain. The ‘leverage’ they occupied may only increase their subjective sense of mastery and confidence in AIP while the true effect of one’s ‘leverage’ may remain unknown. Nevertheless, for those who had limited ‘bargaining leverage’, relocating to rural institutions seemed inevitable. HM, 83, was ‘sent’ to an institution by the village committee. Before relocating she stayed in a ‘shack’ near to her nephew’s house. HM had been kidnapped and sold to a family as a ‘young wife’¹⁰ (*tong yang xi*) when she was little. She was rescued by the police in 1949 and returned to her own village. HM moved to a ‘shack’ – an extremely degraded living situation after the husband died. HM’s narrative expressed a deep sense of

¹⁰ Some rural parents might purchase a female infant as their potential daughter-in-law for their son. They raised the infant until she grew up – usually at the age of 15 to 16, then married the girl to their son. This behavior was confirmed as illegal in 1949.

powerlessness and disappointment in her nephew, who charged her RMB600 (USD 93) rent for the ‘shack’:

Before moving, I stayed at my nephew’s shack. I had to give him RMB600 (USD 100) per year. Every year the village committee would come to my place for the rent (The rent was covered by the village committee). It was RMB600. (tears) I can’t talk about it, I’m speechless. (No.24. HM)

Overall, whether a person had the ‘leverage’ to negotiate for intensive and durable care from the significant other(s) remained an important concern when making the residential decision. Those who could offer their personal property (e.g., farmland, rural house) as exchange seemed to have more confidence and mastery – albeit assumedly – to choose to AIP over institutionalization. For some childless older adults, due to the lifelong and accelerated disadvantage (e.g., poverty, infertility and ageing), the odds of acquiring care may be constrained by their limited resources and disadvantageous social position (e.g., HM’s story).

8.2.3 Implementing the filial responsibility

In the third situation the participants demonstrated neither sufficient ‘leverage’ nor any relational closeness to the caregiver’s household. Instead, receiving intensive and durable care was on demand of the caregiver’s parents; from the perspective of the caregiver, it signified ‘implementation of the filial obligation’.

CN (62, male), a mentally disabled man, had received a wide variety of care from his youngest brother (53, male). CN was born deaf and with mental illness, so he never had the chance to attend school, nor the opportunity to find a mate. Before CN’s parents died, they ‘assigned’ the caring responsibility to the youngest brother due to his geographical closeness and financial capacity. CN’s brother confirmed this situation:

My parents told all the siblings in this family that, those who stayed in this village had to take care of this deaf person (CN). I stayed in village... Me and my wife wash his shoes, clothes and quilt. At any time when we have something to eat, we share with him. (No.13 CN)



Similarly, JZ (64, male) had stayed with his oldest sister (68, female) since he was born. JZ was mentally disabled and extremely thin (only 35 kg at the time of the interview), which imposed great pressure on his sister (and her husband) when caring for the ‘only inheritor’ of the family. The narrative of JZ’s sister was full of ambivalent feeling: being proud when ‘whipping the younger brother to school’; feeling troubled and stressed when ‘preparing every meal for him’. The sister has had rheumatism for decades, going through a dangerous surgery and almost dying in 2019. Regardless, she planned to transfer the caring responsibility to her daughter, whom she regarded as ‘tender and obedient’:

Sister: He (JZ) always behaved like this (mental disability). We grew up in an extremely poor and difficult situation; even the grass we picked from the mountain we gave it to him (as food). We can’t let him starve. He’s the only inheritor of our family. My mother died when she was 36; after that, I took care of him...I tear the food into pieces to feed him, chewing the food before feeding him (when he was young)¹¹. This was how I raised him. After he became an adult, I was ill. I’ve been sick for over two decades. I cannot even think of how I got through all these years.

SC: What’s your future plan?

Sister: I told my daughter to take care of him. If I die, he will become my daughter’s responsibility. We made it clear.

SC: And they agreed?

Sister: Agree? How can they refuse? (No.13. JZ)

According to the narrative of JZ, the prospect of staying with his sister was ‘fortunate’, albeit slightly challenging:

SC (to JZ): What’s your plan?

JZ: I will just behave well. I will do whatever they tell me and...ignoring the complaints they (his sister) make about me.

SC: What do you mean behave well?

JZ: Just let them complain. Sometimes they complain.

Sister (interrupting): When do we complain? My husband (78 years old) has never accused him

¹¹ This is a rural expression meaning ‘this person was under intensive care and received substantive love from their caregiver’.

(JZ). He manages him so well. Sometimes when he goes out, my husband will prepare some warm food and wait for him to come back.

The narrative illustrated above revealed the subtle and complicated interpersonal dynamic during negotiation. On the one hand, a person was seldom able to reject demands from his/her own parents, or he/she might be considered ‘unfilial’. On the other hand, interestingly, because taking care of childless kin was not as normative as taking care of one’s own aged parents, there always existed power imbalance between childless kin and the significant other(s).

One characteristic of such a power imbalance between caregiver and childless kin was that the caregiver intended to underline their ‘moral contribution’ and ‘virtuous identity’ when implementing caring tasks. As TTL’s (67, male) brother (75, male) illustrated:

We raised him (TTL) from the 1970s until now. When my parents died, it was me who held the whole family together. The village committee told me to send him to a rural institution, if taking care of him was too stressful; and I rejected it. I would lose face if I did that...dating back to the 1970s, I earned RMB40 per month while feeding seven persons in my family (meaning, how difficult can it be to feed TTL now?). (No.11. TTL)

Similarly, by taking care of the childless aunt (AB, 83), the caregiver, AB’s nephew (53 years old), could foster a filial image in front of his own son and the village public:

The village cadre tried to persuade me to send her (AB) to an institution, my son insisted they let her stay with us... When talking about ‘raising the old person’ ...to be honest, I’ll be old someday. Every person will be old. My child is filial, giving her food and care...I’ll tell you that, if there are any old persons dying in this village, I’ll definitely provide my help (to arrange the funeral¹²). Especially during the funeral arrangements, I’ll spend my whole day in that house. (No.7. AB)

¹² In rural areas, the funeral has to be implemented with collective help. The household that arranges a funeral for their family member will invite friends, neighbours and extended kin to ‘contribute their labour force’. How many people this household could invite was considered one indicator of this household’s social capital and social status in this village.

Another characteristic of the power imbalance was that the childless kin had to demonstrate strong willingness to offer help to the caregiver's household when they 'had someone at home'. Such help included a wide variety of tasks including watching the house, fertilizing the farmland, raising chickens or cleaning rooms, depending on their physical capacity. From the caregiver perspective, interestingly, the attitude towards such help was complicated. Some complained that the childless kin made only a minimal contribution to this family, such as JZ's sister:

He can do nothing at home...not even cook for himself. He also won't go to the rural institution. He said he would rather stay at home. He eats what we eat at home. (No.13. JZ)

Some caregivers considered the labour involvement unnecessary or even troublesome, such as AB's nephew:

SC: Is there any conflict between you and your aunt?

Nephew: No...sometimes we tell her to take some rest, but she insists to pick woods and raise chickens....stuff like that.

Nephew's wife: Yes, she does her stuff here and there, even we tell her not to. She's half blind, we are so worried that she falls again. (No.7. AB)

These two examples, echoing the previous argument of this research, suggested that the negotiation between childless kin and the significant other(s) was complicated, and embedded in the filial norm of rural society. Filial piety established the normative expectation where adult children have to be respectful, obey, please and offer material and non-material support to their aged parents (Chou, 2011). Taking care of one's childless kin, on the other hand, was related to not falling into the filial norm, and hence was secondary to filial obligation.

DW's story confirmed the argument that, when taking care of childless kin was contradict with taking care of one's 'true' parents, no matter how intimate the relationship may appear, it was more appropriate for the caregiver to fulfil his filial responsibility. DW (76 female) had received 'considerable' care from her nephew before institutionalization. Her only son died as an infant, then her nephew, the son of



her husband's youngest brother, insisted on residing with DW. DW was close to her nephew, who had 'torn the chicken into small pieces to feed her due to her bad teeth'. A quarrel with the nephew's mother emerged in 2016. The nephew's mother accused DW of 'wasting the grain', implying that DW was burdensome to the nephew's family. In return, DW relocated to an institution to 'save some dignity':

She (DW's mother) said it was a great waste to give her grain to the chickens and the dog (implying the participants were wasting her son's grain). I was furious. I said, I won't waste your grain anymore, and I leave (laughs)...it was just one time. Other than that, me and his mother got on well. We visited each other at the Spring Festival. (No.22. DW)

This section has illustrated that, apart from providing one's personal property or continuing the relational closeness, childless older adults may choose to AIP due to the strong filial obligation of the caregiver. Filial piety was the mainstream caring expectation and discourse in rural China, shadowing the caring relationship between childless older adults and their caregiver while governing what types of support they may gain from extended kin and what 'price' they had to pay. Whether through conceding from conflict (e.g., JZ) or contributing labour force (e.g., AB), this phenomenon suggested that, in most circumstances, childless older adults were in a disadvantageous position when negotiating long-term care. As they understood how childlessness was 'deviant' in rural society, acquiring support from the significant other(s) always required justification and compensation.

8.3 Summary of this chapter

This chapter has explored how significant others influence the residential decision among childless older adults in rural China. Two subquestions were explored: (a) who the significant other(s) of the childless older adults in the context of long-term care was and (b) how the significant other(s) influenced the residential decisions. All identified significant others fall into the network of extended kin, in which participants used the term 'having someone at home' to describe their situation. 'Having someone' was not synonymous with 'receiving support, such as helping with farming, digging channels or watching farmland for villagers'; instead, it signified a situation where the adoptive



relationship with the significant other(s) could guarantee the intensiveness and durability of long-term care. Once participants ‘have someone at home’, the chance of choosing to AIP increased; conversely, ‘having no one at home’ was an important reason to relocate. This chapter has argued that, essentially, ‘having someone at home’ enabled a sense of mastery on an interpersonal level, and thus encouraged childless older adults to choose to AIP.

Specifically, this chapter has highlighted three characteristics of ‘having someone at home’ in the context of long-term care arrangement. Firstly, the caring relationship and supporting tasks have to be intense and durable; it was available for the current and for the future. For childless older adults in rural China, the intense and durable care a person may receive was usually limited to extended kin – who happened to have the willingness and the ability to provide help. Otherwise, the support from neighbours, friends or kin members other than ‘significant kin’ was not reliable enough to influence such a huge decision in later life.

Secondly, ‘having someone at home’ was an ongoing negotiation process that was full of resilient characteristics as well as struggling and messiness. Childless older adults might refer to the relational closeness, providing their property (farmland, residual house), or relying on filial obligation to gain and to sustain care from the caregiver, depending on what resources the childless older adults had. This suggested that, according to Grenier (2017), the chance of receiving support and care from the significant other(s) was dependent on, and became the extension of, one’s social position and resources (Golant, 2015; Grenier et al., 2017). If a person didn’t have these resources during negotiation, he/she might be unable to choose to AIP, even if he/she had such desire.

Relatedly, childless older adults were more likely to fall into a disadvantageous situation during negotiation. Because taking care of one’s childless kin was not strictly part of, but may be related to, filial piety, the prospect of ‘having someone at home’ was not as normative as taking care of one’s own aged parents, and hence always required compromise and compensation. Childless older adults either appeared reluctant to reveal their expectations, or they had to actively contribute their labour force to the



caregiver's household to avoid the label of 'useless'. From the caregiver perspective, although not all of them valued momentary gain, the assentation of a 'victorious' and 'filial' image was a good reward. Essentially, this chapter argues that receiving care from extended kin rather than from one's children was culturally deviate, hence has to offer price to compensate.

It should be noted that the chance of receiving support from the significant other(s) is gendered. Women are more likely to be considered a 'future wife' than successor of a rural household, and hence are less likely than men to inherit the property from their parents. Marriage forces them to encounter multiple relocations, decreasing the chance to accumulate 'negotiation leverage'. When in need of support someday, they usually refer to relational intimacy with (potential) significant other(s) rather than turning to bargaining 'leverage' for anticipated support and care, which might be problematic if the caring relationship becomes less stable (e.g., HM and DW's story).

Overall, this chapter has concluded that 'having someone at home' – receiving support and care from a significant other(s) – remains an important reason when deciding where to grow old. It signifies a prospect where a childless older adult is able to acquire intensive and durable care from a significant other(s) – usually extended kin. It is a special and complicated situation that involves ongoing negotiation with the caregiver, which further depends on one's financial capacity, relational closeness and position regarding the filial norm. Essentially, such negotiation reflects the fact that, as 'deviant' members of a rural society, childless older adults have to 'make certain compensation' to the caregiver's household in the context of the residential reasoning process.



Chapter Nine

Discussion and Conclusion

This research has aimed to gain in-depth understanding of how childless older adults make long-term residential decisions (AIP versus institutionalization) and why. The question responded to gaps in the current literature, which tended to simplify childlessness as one category of demographic features, marginalizing the voice, life experience and subjectivity of the childless group (Ivanova & Dykstra, 2015; Verdery et al., 2019). This situation is especially prominent in rural China due to the constrained public service system and the overreliance of elderly care responsibility on family members (Yang et al., 2020). In other words, a person who does not benefit from children's long-term care in rural China faces layers of challenges that deserve a policy response.

The term *residential reasoning* was applied to guide the research process. Defined as the ongoing negotiation process of residential decision-making, this concept was found to be appropriate for the childless group in rural China. As findings illustrated, childless older adults in rural China demonstrate strong resilient characteristics and make multiple adaptive efforts before making their residential decision (of AIP or institutionalization). Further, the process of reasoning was embedded in the general experience of rural ageing, which encompasses a matrix of difficulties such as poverty, cultural stigmatization and gender discrimination. Overall, this research has revealed that the residential reasoning process of childless older adults is centred around mastery experience. As long as childless older adults could gain mastery on an individual and interpersonal level through adaptation – albeit in a struggling and messy way – they were more likely to choose to AIP than relocate to rural institutions.

A constructivist grounded theory approach was applied to guide the method design. Confirming and extending the residential reasoning model, this research revealed that there were three main findings that emerged from data analysis. Each argument



corresponded with one key concept of the model, respectively: residential assessment (in terms of mastery or comfort), residential adaptation and the significant other(s). The final concept could be regarded as an extension of residential adaptation on an interpersonal level (Koss & Ekerdt, 2016).

(1) Childless older adults assessed their living situations in rural areas based on the mastery experience, albeit in an ambivalent way. As long as residential mastery was fulfilled: participants was self-reliant, socially connected and their needs were fulfilled from formal sectors, the residential congruence was achieved.

(2) Achieving residential mastery required ongoing negotiation and adaptation. On an individual level, childless older adults engaged with cognitive, behavioural and existential strategies – in a struggling and messy way – to convert residential discomfort into a mastery experience. Once a person could achieve such mastery, according to the narrative of participants, ‘to endure *ku*’, he/she tended to choose AIP over institutionalization.

(3) ‘Having someone at home’ was another reason for childless older adults to choose to AIP rather than relocate to rural institutions, which could be considered mastery and certainty on an interpersonal level. The process was negotiated as well, since childless older adults had to offer their property, referring to relational closeness or relying on filial piety to sustain the status of ‘having someone’.

Extending key concept of the residential reasoning model, the upcoming section of this chapter discusses the findings of this study. The first section provides a contextual understanding of participants’ life experience, hopefully to draw readers into the context of rural China and to have a look at how participants struggled to make a living for lifetime. Three characteristics of childlessness are highlighted and discussed in the first section. In the second section of this chapter, this research applies the term ‘mastery of ambivalence’ to respond to the question of how the residential situation is assessed by participants through the lens of the residential reasoning model. Thereafter, two levels of adaptation – enduring *ku* and having someone at home – are discussed. Specifically, because childlessness is gendered in rural China, a discussion on how



female and male participants differ is presented afterwards. Finally, this chapter summarizes the theoretical and policy contribution of this study and provides an overall conclusion.

9.1 Childlessness in rural China

In chapter Five, the life experience of participants in terms of their childhoods career path, family relations and the pathway to becoming childless in later life was reviewed. It provided the contextual understanding childlessness in rural China from an individual and structural perspective, underlining how childlessness is, in the end, an undesired, disadvantageous and deviant situation in the rural context. There are three characteristics of childlessness in rural China worth mentioning, namely: (a) the involuntary character; (b) the individual/structural constriction; and (c) the cultural deviation.

Firstly, childlessness in rural China is not voluntary. The findings of this research identified four pathways to becoming childless in later life, namely: (a) lifelong singleness and childlessness; (b) divorce/separation without children; (c) biological infertility; and (d) loss of children, none of which is intentional. This finding is in line with anthropology studies conducted in other developing countries, indicating that ageing without children is usually driven by unexpected life trajectories (Djundeva et al., 2018). Unlike the Western society in the 1960s where the decline of the infertility rate signified ‘a shift from Maslowian basic surviving needs to higher-order needs of self-actualization and individual autonomy’ (Zaidi & Morgan, 2017), in rural China, childbearing remained a momentous investment strategy to guarantee later-life wellness (Cai et al., 2012). Two factors accounted for this argument. First of all, while the state had attempted to promote agricultural modernization since the 1949s, farming production still relies largely on small households (Day & Schneider, 2018). Secondly, despite the rising coverage of pensions, the long-term care system in rural China still relies heavily on family – namely, adult children and spouses (Wang et al., 2020). Previous research revealed how older adults in rural China have to rely heavily on their offspring for monetary, informational and emotional support (Djundeva et al., 2018; Wu et al., 2009), and how the absence of children’s care is dependent on physical, social



and psychological unwellness (Feng, 2018; Zhang, 2007; Zhang & Liu, 2007). This research supported the argument of Lesthaeghe (2010), who regarded the decline of infertility as one of the results of economic growth, modernization and change in the social norm of marriage and childbearing in the 1960s – what he called the ‘second demographic transition’ (Lesthaeghe, 2010; Zaidi & Morgan, 2017). In rural China, where the modernity level is still ‘immature’ (Lai, 2016), it is not attractive for residents to choose to become childless, especially when their productive capacity will drastically decline during ageing.

Secondly, any review of childlessness in rural China has to pay specific attention to filial piety – the cultural tenant that has established the rightful form of long-term care arrangement among parents and children for thousands of years (Zhang, 2004). Having offspring, especially male inheritors, caters to the filial expectation and patriarchal family tradition that promoted a patrilineal transfer of assets from one generation to the next (Hsu & Shyu, 2003). Consequently, a person who becomes childless is considered to have ‘failed’ to carry on the patrilineal line, and hence might experience simultaneous stigmatization and sympathy from the village public. The narrative of most (male) participants, who justified their childless status as ‘unmatching of mate’, expressed their complex perception. Supporting the mainstream idea of ‘filial’ while externalizing the childless status as an ‘unexpected and unintentional result’, childlessness was positioned on the spectrum of acceptance of the undesired reality and the willingness to defend one’s dignity. Such experience is different from how men illustrate their (involuntary) childless status in Western society. Although becoming childless shares to some extent a ‘deviation’ from the normative expectation – getting a job, establishing a family and having offspring, to be well-off in later life (Hadley, 2018; Ng et al., 2020) – what needs to be highlighted in the Chinese context is that under the discourse of filial piety, childlessness is more of not only an ‘individual failure’ but also a betray of patriarchal and familial expectation that deserves justification.

Thirdly, and most importantly, childlessness demonstrates the intertwined influence of individual disadvantage and structural constriction in rural China. By using the term ‘structure’, sociologists referred to the ‘tendency or pattern or relations that are not reducible to individuals and are durable enough to withstand the whims of



individuals who would change them’ (Kazyak & Park, 2020). Specifically, there were two structural constrictions accounted for this issue: (a) household poverty; and (b) gender. Household poverty remains a cause of childlessness while intensifying the negative consequence of childlessness, especially when entering the stage of being ‘in need’ (Zhang & Liu, 2007). This is possible when a man who cannot afford the wedding expenses remains involuntarily single and childless for his entire life (e.g., TS); or a person born with a certain disease is unable to seek medical assistance due to household poverty, leaving him/her with a lifelong disability and ‘unlikely to find any mate’ (e.g., JZ and HH). Despite receiving government subsidies (approximately RMB500 to 800 per month for childlessness), none of the participants pondered ‘having the luxury’ to find a mate, especially with advancing age and the of losing their farming capacity.

Gender is another important dimension in understanding childlessness in rural China. In societies where skewed sex ratios and a preference for sons have created an increasing number of ‘involuntary single’ men, such as rural China and India, the pathway to childlessness is different between men and women (Nie, 2020). For female participants, childless status basically results from biological infertility or outliving one’s children; only one female participant had never married. Ironically, a skewed sex ratio in the demographic structure may increase women’s marriage autonomy on the one hand, reproducing gender discrimination by manifestly selecting the gender of newborns and a substantive preference for sons over daughters on the other (Jin et al., 2013). Discrimination against women was evident in great detail in the participants’ narratives, in regard to attitudes on marriage, family and offspring, and from the perspective of informants. For example, when describing biological infertility, participants and informants tended to use the phrase ‘unable to bear children’, implying that biological infertility is a ‘women’s problem’ (Bell, 2013) or an ‘imperfection of motherhood’ (Hollo, 2019). When narrating the experience of losing one’s children, informants intended to emphasize how it is ‘pathetic’ and ‘unsettling’ – particularly for women as they had ‘no one to rely on’. More detailed discussion on gender and childlessness can be found in the following section of this chapter. This research echoes and extends Verdery’s (2019) study, which attempted to record the kinlessness rate across developing and developed countries and regions and suggested that childlessness predominantly affects males in China and some European countries (Ireland, Denmark,



Slovenia, German) (Verdery et al., 2019). This research attributes this phenomenon to the deeply rooted (discriminative) idea in areas where the filial norm is proliferated, especially in rural China, that only men have the responsibility to ‘carry on the family line’ and that women ‘were born to be someone’s wife’ and ‘carry on the family line for others’ (Jin et al., 2013).

This section specifies that becoming childlessness in rural China are disadvantageous, so that making the long-term residential decision are full of adaptation. The next section shifts our attention to the residential reasoning process of childless older adults, revealing how they assess the residential circumstances of rural living, and how they have struggled to adapt to difficulties in making their residential decisions.

9.2 Mastery of ambivalence: how childless older adults assess their residential circumstances

Informed by key concepts of the residential normal and reasoning model, the question of, how participants assess their living situation through the lens of mastery and comfort (subquestion (a)) was investigated (Golant, 2011). The findings of this research revealed that, when assessing the current (or previous, for the institutionalization group) residential circumstances, the sense of mastery was weighted heavier than the residential comfortable experience, entailing the precondition of residential congruence. This research has challenged the residential normalcy and reasoning model, revealing that, at least for childless older adults, living in a constrictive rural community was hardly comfortable but could be mastery.

Golant’s model (2011) valued the subjective perception from the resident’s own standpoint rather than from that of the social worker, policymaker or person who conducted ‘needs assessment’. It attempted to formulate a framework to predict the level of residential congruence – that is, when people’s goals and needs are satisfied in order to explain relocation decisions from an emotional-based perspective (Golant, 2011; Granbom et al., 2014). It distinguished two levels of residential experience (mastery and comfort) to capture the emotional complexity of environmental surroundings, usually one’s own home (Golant, 2015). It was initiated in response to



Lawton's model – which considered the driven force of relocation as 'unfit for one's environmental needs' and 'the loss of one's competence', which simplified the complex and dynamic residential experience from Golant's perspective (Golant, 2018; Perry et al., 2014). This research supported this view and found his argument overall applicable to participants.

What differentiating the residential normalcy and reasoning model from this research shows is that residential congruence was enhanced by a mastery experience rather than a comfortable experience. In other words, it is unnecessary for childless participants to fall into both the mastery and the comfort zone to achieve congruence, given that living in a constrictive rural community comes along with a matrix of discomfort such as stress, suffering and powerlessness. Specifically, there were levels of discomfort experience were found from the participants' narratives.

Firstly, farming is a complex experience that brings stress and suffering – the negative aspect of *ku* with it. Although farming activities represent a form of self-reliance in the mainstream rural discourse – similarly to farmers' narrative' in Australia, Canada and Africa (O'Callaghan & Warburton, 2017; Rutagumirwa & Bailey, 2019; Winterton & Warburton, 2011), what remains unignorable is that farming activities have the function of catering for basic material needs in rural China. It wasn't until the 1980s that farmers could use modern machines and fertilizers to liberate themselves from intensive labour demands (Schneider, 2015); prior to that, the stressfulness of 'getting food or otherwise starving' and the suffering of 'farming on extremely dry and warm days' filled the participants' narratives. The situation is more prominent in mountainous areas such as rural Yunnan, where the use of agricultural machinery is constrained by geographical disparity and farmers dividing the farmland into pieces (approximately 400–600 square metres for a childless farmer) to obtain grains, vegetables and commercial agricultural products (e.g., beans). Pang's (2004) description of Chinese farmers is appropriate in summarizing the participants of this research: 'working to survive' and 'working until they drop' (Pang et al., 2004; Zhang, 2007).

Secondly, in terms of the interpersonal relationships of participants, the sense of



mastery came along with constriction and negotiation. Sociologist Fei Xiaotong admired the ‘society of familiarity’ and its ‘friendly, connected, trusted’ social atmosphere of rural China (Lou & Ng, 2012); others criticized its ‘invisible and unjust oppression’ of disadvantaged social members (Bian et al., 2015). This research found both arguments partially applicable to participants’ narratives. On the one hand, participants illustrated their experience of ‘visiting neighbours’ and ‘helping friends’ – an experience that is similar to what Van Dijk et al. (2015) called the ‘naturally occurred social network’ (Van Dijk et al., 2015). On the other hand, for childless older adults, seldom would they expect their neighbours, friends and extended kin to respond to their emergent and intensive needs, such as declining health. Specifically, participants illustrate two constrictions of interpersonal support: the common expectation of reciprocity (e.g., ‘I might not be able to pay them back if I receive such intensive help’, as expressed by GG) and relational closeness.

Chinese sociologist Bian (2018) used the term *renqing* to explain such complicated interpersonal negotiation. A person providing help to others could accelerate his/her *renqing* – a form of social capital and the ‘debt’ that the help recipient potentially owed to the help provider. Once the ‘debt’ exceeds its anticipatory value, which depends on the subjective assessment of each party, it will render the help recipient morally subordinate to the help provider – ‘owing a dreadfully large amount of *renqing* to the support provider’ (Bian, 2018). The amount of ‘debt’ depends on the relational closeness of each party and the social status of each party (Bian et al., 2015). This could be used to explain the findings of this research. There existed an atmosphere among villagers that acquiring instrumental and support was common. However, as (socially, financially and physically) disadvantaged as childless older adults, reaching out for support may mean a large amount of *renqing* is owed to the support provider. The simultaneous sense of connection and constriction among villagers is similar to what Volckaert (2020) found in rural Belgium among older residents (Volckaert et al., 2020), only its cultural root is different. As BY, one of the participants, concluded: in such a ‘society of money’, every kind of ‘help’ has its price.

Thirdly, dealing with the formal sector (e.g., village committee or anti-poverty work team) also encompassed complicated feelings of mastery and powerlessness:



participants had to rely on their village committee to apply for government assistance, while the experience was unpleasant and full of powerlessness. BY's interaction with the village cadre expressed such tension. This research attributed this phenomenon to the stigma over 'lazy' and 'reliance' in rural discourse, which are in the opposite spectrum of 'laborious', 'hardworking' and 'self-sufficient' (Li & Walker, 2018; Schneider, 2015). Voicing one's needs in front of the formal sector signified a compromising of dignity and, similarly to older farmers in Australia, 'an undermining of one's masculinity' (O'Callaghan & Warburton, 2017; Söderberg et al., 2013).

Taking the individual, interpersonal and institutional level of residential experience into consideration, this research applied the term *mastery of ambivalence* to summarize the way childless older adults assess their (current and previous) residential circumstances of rural living. Ambivalence refers to the extent to which people's separate rating of positive and negative reactions to a topic/phenomenon/event reflects a mixed evaluation (Luttrell et al., 2016). For childless older adults living in rural China, feelings of mastery – being self-reliant during farming, feeling connected in interpersonal interactions and being 'realistic' in front of the formal sector – came along with a matrix of discomfort experiences such as stressfulness, suffering, constriction and powerlessness. More importantly, the ambivalence reflects intersected layers of (individual and structural) disadvantage a childless person encountered in rural area. This finding detailing the question of 'how childless older adults assess their residential circumstances through a lens of mastery and comfort' expands the residential normalcy model to the 'non-parental' and 'non-prestigious' group. Essentially, it points to the fact that what Golant (2011) defined as 'comfortable' in the residential normalcy model – feeling pleasure, hassle-free and memorable may seem 'too luxurious' to the childless group (Golant, 2011).

What needs to be specified is that this research did not mean to disregard the importance of residential comfort. Although most of the narratives from the participants were more closely related to residential mastery, some illustrated 'the enjoyment of the rural landscape' or 'satisfaction with the mountainous view' – the comfortable experience a person perceived during rural living. However, this research argues that sense of comfort is secondary to mastery during residential assessment. Sense of control,



autonomy and self-reliance are too monumental to be compromised when a person has to rely on their own physical, mental and social resources to gain food. This could be partially attributed to the lifelong exposure to environmental constriction, as most of participants still had the vivid memory on the ‘great hunger’ on 1950s. Experiencing the extreme poverty on childhood and the tremendous socio-economic change on adulthood, rural residents’ of 1940-1960s cohort, both childless and parental group had internalized the importance of ‘hardworking’ and ‘enduring’ (Hershatter, 2011). Prioritizing instrumental needs (‘to feed one’s stomach’, as suggested by BY) and marginalizing the ‘individual enjoyment’ remains a practical surviving strategy as well as personal virtue for most of participants. Similarly to qualitative findings in degraded urban neighbourhoods and rural areas in the US (see, Finlay et al., 2020; Ness et al., 2014), this research emphasizes that, childless older adults transfer their preliminary attention away from the irreversible and structural disadvantage to sense of mastery, control and individual autonomy (Haushofer & Fehr, 2014). The aspect of individual enjoyment, pleasant and comfort are not being eliminated but being marginalized – ‘comprisable and adaptive’ for some participants – to ensure mastery and self-reliance.

It has to be notified that the ambivalence of mastery is not a clear-cut of residential assessment but an ongoing negotiation process. It is possible when childless older adults who are seemingly loss of control still choose to stay in their own home; or some participants relocated to rural institutions in advance for the upcoming age-related decline. This finding did not overturn the fact that residential mastery is the preliminary condition of residential congruence but pointing to the necessity to understand the residential mastery, adaptation and decision-making from dynamic perspective. Because childless older adults could always change their mind on the residential decisions, they could stay in their own home until their mastery experience are not able to continue. This services flexibility also encourages the residential decisions to take as long time as the childless older adults needs, which in line with argument of residential reasoning model: residential mastery lays on the centre position of residential reasoning process (Granbom et al., 2014).

Shifting from residential assessment to adaptation, the next section would elaborate how childless older adults respond to difficulties when staying in rural community, and



how such adaptation influenced their residential decisions.

9.3 The residential decision as ongoing negotiation process

Other than capturing a method of residential assessment, the residential normalcy model considered the relocation decision as an ongoing negotiation process full of adaptation (Golant, 2015). Both the residential normalcy and reasoning models were proved to be applicable in the empirical world, especially in regard to adaptation and resilient characteristics (Koss & Ekerdt, 2016; Stafford, 2017). However, when applying this concept to the group of childlessness, one concern that cannot be ignored is the uneven and messy side during the process, which are further dependent on their disadvantageous social positions. The next two sections respond to the subquestion (b) (How childless older adults adapt to the residential difficulties). It focuses on two levels of negotiation and their struggling experience: (a) enduring *ku*; and (b) having someone at home. Both levels of adaptation represented important reasons to influence residential decisions. Overall, two findings point to the core argument of this research, that is, the decision of where to grow old as an ongoing negotiation and the adaptation process for childless older adults in rural China.

9.3.1 Enduring *ku* and residential decisions

The first aspect that influences residential decisions was the extent to which a person can endure *ku*, meaning whether a person could endure the stressful and suffering aspect of farming. If the answer was positive, participants tended to choose AIP; otherwise, if they were overwhelmed by *ku*, institutionalization became more likely.

Since the term '*ku*' is a local term deriving from participants' own narrative, this research first aimed to identify its definition. The term '*ku*' contains a complex emotional status, which, on the one hand, refers to the behaviour of 'conducting farming and farm-related activities'. On the other hand, the literal meaning of *ku* is 'bitterness', implying that farming and farm-related activities covered layers of negative feeling such as stressfulness and suffering. The term '*ku*' has its historical and socio-contextual



root in rural China. For example, since the 1950s, the People's Republic of China has advocated the political movement of *suku* ('talking about bitterness') to mobilize low-income farmers to share their stories of misery in China's former society. In its narrative form, the suffering experiences of *ku* contradicted the present, 'well-off' and 'sweet' society (Zhou, 2015). Since then, *ku* was a linguistic framework for farmers to organize and vocalize their (mostly suffering) experience and autobiographical identity. Childless older adults in rural China had to endure layers of physical and social difficulties during farming (Pang et al., 2004). This research argues that the narrative of *ku* in rural China came along with the practical concern that living in a rural area has to involve prioritizing survival needs, e.g., 'farming for food, even in a stressful way'.

When residential choices had to be made, a person would assess the farming capacity – the extent to which one could endure and continue *ku* – to decide where to grow old. Moreover, enduring *ku* was an ongoing negotiating process that was full of adaptation and uneven aspect. Childless older adults routinized everyday *ku*, reappraised the negative side of *ku* and making meaning from *ku* to achieve residential congruence. The adaptation strategies corresponded with Golant's (2011) model, which emphasized how adaptation involved with a wide range of behavioural, cognitive and existential efforts. Specifically, this research highlighted how adaptation was shaped by the individual and structural constrictions. When participants struggled to 'focus on the present', it reflected a deep anxiety of the long-term prospects of rural ageing (Haushofer & Fehr, 2014; Lofqvist et al., 2013). This research draws attention to how structural inequality infuses into everyday adaptation practices, rendering the process struggle and messy.

Interestingly, the narrative of *ku* is emancipatory. Enduring *ku* become the spiritual strength for childless older adults to respond to their existential concern (Peace et al., 2011) and to legitimize their (past) discomfort and suffering experience. In other words, sense of mastery is tied to contemporary achievements and future development of China's society. By sharpening the comparison between the past *ku* – the hunger, anxiety and uncertainty that participants had experienced since childhood – and the contemporary, endurable *ku*, residential mastery is legitimized by living in the developed, progressive Chinese society.



There are three situations where childless older adults may perceive *ku* unendurable and preferred to relocate to institutions: when daily routines are interrupted, when the residential suffering is too prominent to harm sense of mastery and when participants fail to make meaning from *ku*. This finding suggests that, echoing with argument of Golant (2011), moving to elder-care institution is the last solution for most of older adults to gain mastery and sense of control (Golant, 2011). Specifically, those who had been interrupted by unexpected health decline seemed to be risky (e.g. HM's story). Studies conducted in Canada recognized the importance of notification and preparation before moving to elder-care institutions. It suggested that those who engaged with involuntary and unexpected moving were more likely to feel out of control and feel disengaged with new environment (Sussman & Orav-Lakaski, 2020). Further research may consider exploring the residential mastery experience among involuntary movers, investigating their hesitations, needs and adaptation strategies in new surroundings – putting by Golant's (2012) discourse, 'the re-gain of mastery' in elder-care institutions.

Similar with ambivalence of mastery, enduring *ku* is not a momentary event or a clear-cut of evaluation; it is a dynamic and continuous process that might take month, or even years to live with. It is possible when participants, even if they seemed capable of *ku* for the current choose to relocate to rural institutions to avoid the upcoming prospect of *ku* in rural community (e.g. LF). This finding did not contradict with our framework but extending the residential decision-making from continuous perspective. Qualitative studies conducted in U.S. suggested a research potential to treat the residential decision as a forward-looking evaluation, that is, to consider the future residential needs and physical capacities instead of the needs for the current situation (Koss & Ekerdt, 2016). This research also underlined that the residential decision-making of childless older adults is continuous, thanks to the flexible accommodation services of long-term care.

9.3.2 *Having someone at home and residential decisions*

On the second level of negotiation and residential decisions, this research specified the importance of 'having someone at home' – another narrative that derived from the local narratives of participants. The findings of Chapter Eight clarified (a) the identity



of the ‘one’, a figure that was specified as a ‘significant influencer or other(s)’ in the residential normalcy model and (b) how it influenced residential decisions. In conclusion, this research has revealed that when ‘having someone at home’, the intensiveness and durability of long-term care is guaranteed, so childless participants intended to choose AIP rather than institutionalization. ‘Having someone at home’ is an ongoing negotiation process that is dependent on the individual and structural constriction of childlessness in rural China. When perceived ‘not having someone at home’, it was more likely for participants to relocate to rural institutions for future and advancing care.

Only kin members have the potential to become the ‘significant other(s)’ of participants, yet not all kin members *would* turn to this situation and shoulder the weight of influencing one’s long-term care residential decisions. This finding corresponded with the argument from Chapter Six that interpersonal support and care among villagers are common yet constricted. Providing a certain amount of labour assistance and emotional comfort is possible; guaranteeing the intensiveness and durability of care is another story. This finding challenged the proliferated applied definition of ‘receiving support’ in quantitative studies, which tended to regard the situation as binary and clear-cut (Wang et al., 2020). This research argues that, even when older adults, especially the childless group who had experienced a lifelong support deficit, admitted receiving support currently, such support didn’t point to its anticipatory effect and intensive needs: for example, when age-related challenges hindered daily activities (Ivanova & Dykstra, 2015). Although rural areas were known to have an atmosphere of being ‘supportive’ and ‘familiar’ (Ryan & McKenna, 2013), a person would not expect their neighbours, friends or kin members to fulfil emergent needs (Volckaert et al., 2020). In sum, the long-term care residential decision is only influenced by the presence of a significant other(s); otherwise, ‘receiving support from an informal support network’ did not shoulder such weight to influence the mountainous decision in later life.

‘Having someone’ is a negotiation process rather than a momentary event (Granbom et al., 2014; Koss & Ekerdt, 2016); it was the sense of mastery and certainty of significant others that influenced the long-term residential decisions. When the intensiveness and durability of care are confirmed by significant others, legally or orally,



it enables childless older adults to have the confidence to trust their judgement and choose to age in place (Kahneman & Tversky, 1979; Tversky & Wakker, 1995).

There were also three situations where participants felt distrust and uncertain to ‘someone’ – the potential significant other(s) living in the same village. Childless older adults may have conflictive relationship with their close kin in village, so that the option of relocation was a way to ‘make peace’ (e.g. WL’s story). They may perceive themselves unable to provide material ‘leverage’ for the future care, hence choosing to relocate to institutions to avoid become a ‘burden’ to their loved one (e.g. LD’ story). On a rare situation when childless older adults had been taking care of by niece and nephews who also have their own aged parents alive, the caring relationship may become culturally inappropriate. All these situations are perceived as ‘not having someone at home’ and lead to the choose of relocation. These findings supplemented the existing literature on social support and institutionalization, who suggested that childless older adults living in rural area had to turn to formal sectors when there were (prospectively) no one available to provide care (Wenger, 2009). This research revealed that the potential caregiver may exists in different form (as ‘distant relatives’ or ‘nice person in village’). Policymakers may consider encouraging this significant person(s) to provide long-term care to the childless older adults while providing (instrumental or cultural) reward to them. Afterall, ‘having someone at home’ is driven by sense of mastery and certainty, which are further confirmed by instrumental, relational or cultural assertion. The caring relationship with the significant other(s) are more likely to be interrupted than parent-children relationship due to its cultural illegitimacy.

Further, sense of relational mastery is dependent upon one’s social resources and social position, intertwined with the disadvantage of childlessness and having a lifelong impact on later life during the residential decision-making process. Two phenomena account for this argument. Firstly, childless older adults usually have to exchange their individual property, specifying the relational closeness with the ‘one’ or relying on the ‘last wish of parents’ to gain long-term care from a significant other(s). In other words, there is always a ‘price’ to pay during negotiation, even if the ‘price’ is not necessarily in a momentary form. Secondly and relatedly, because the ‘price’ is usually shouldered by childless older adults, it is more likely that the caregiver will fall into a



disadvantageous situation during negotiation. They whether being reluctant to reveal needs and expectation on the future residential arrangement (e.g., YZ), which could be interpreted as a signal of powerlessness in literature (Nagar-Ron & Motzafi-Haller, 2011), or they have to effortfully engage with farming activities to demonstrate their valuable contribution. They could be accused of being ‘useless’ – a severe criticism that contradicts the mainstream ideas of ‘laboriousness’, ‘hardworking’ and ‘self-reliance’ in rural areas (e.g., JZ and DW) (Schneider, 2015). This indicates that the significant other(s) and their household has the power to decide which behaviour deserves encouragement and which needs to be condemned. In other words, once a long-term caring relationship with a significant other(s) is initiated, the power of this person is over the childless.

On investigating the reason why the significant other(s) has power over childless older adults, this research attributes it to the proliferated idea of filial piety. Filial piety established the rightful form of care that aged parents deserved from their adult children, creating the opportunity for the childless group to acquire long-term care from kin, yet constraining such care in an ‘accepted’ and ‘appropriate’ range. Because childlessness is eventually deviate from filial expectation, acquiring long-term care from non-children (one’s siblings, nephews or nieces) is accompanied by compromise and compensation.

The notion of ‘accountability’ seemed particularly appropriate to explain this phenomenon. According to Hollander (2013), accountability refers to the process where (a) a person is aware of the social expectations in any given social category (e.g., filial piety), (b) a person when interacting judges themselves and others on whether they meet the expectation (e.g., how the childlessness and parental groups differ under filial piety), and (c) he/she responds to this judgement (e.g., negotiating for support as childless older adults) (Hollander, 2013). Accountability is a term to describe how social norms, especially the dominant social expectations, are produced in interaction and how inequality is reproduced from day-to-day social interactions (Kazyak & Park, 2020). In the context of this research, both childless older adults and caregivers are held accountable by filial expectation through interaction. When a caregiver underlines their ‘virtuousness’ in taking care of childless kin, or when childless older adults have to



withdraw from the existing caring relationship to avoid interrupting the ‘true filial responsibility’ of a caregiver, it reflects a common understanding among both parties that taking care of one’s childless kin is ‘deviate’ from taking care of one’s aged parents. As summarized by one informant, ‘it falls into a grey space of filial norm’. This may explain, at least partially, why childless older adults have to make an effort (instrumentally or emotionally) to compromise or compensate for caregivers.

In conclusion, the findings from Chapter Eight reveal how ‘having someone at home’ – receiving support and care from a significant other(s) – became an important reason when deciding where to grow old. This finding is in line with the core argument of the residential normalcy model, which suggested that the decision to relocate is made under the influence of a significant other(s) by enhancing one’s sense of mastery (Golant, 2015). This research argues that such an influence remains the extension of one’s social and cultural position (Connidis, 2015). When childless older adults are suffering from a shortage of (financial, relational or cultural) resources during negotiation, they are more likely to fall into a disadvantageous situation.

9.4 Gendered reasoning, placeless women and precarity

One unneglectable facet of this research is that the residential reasoning process are gendered. It intersects social attributes such as household poverty and ageing, remaining a powerful lens to understand the residential adaptation and decision-making process among childless older adults in rural China. The pathway to becoming childless when living in a rural community is gendered, which leads to strategy differentiation on their adaptation strategies. It further reflects a deeply rooted discrimination against women, rendering them more vulnerable and oppressive than their male counterparts. This section elucidated two keywords to discuss this issue: placeless and precarity.

As argued before, due to the skewed sex ratio in rural areas, with a strong preference for sons over daughters, women are more likely to find a mate, and thus less likely to become childless in later life due to lifelong singleness (Djundeva et al., 2018; Zhang, 2006). The cultural source of the preference for sons has followed the patriarchal expectation and filial norm for thousands of years, claiming that only men,



and not women, have the responsibility to carry on the family line (Jin et al., 2013). Women are ‘born to be the wife of another household’, and thus have limited opportunity and expectation to provide long-term care to their aged parents (Djundeva et al., 2018). Correspondingly, daughters are less likely to obtain resources from their own rural parents and the village public (Zhang, 2006).

Once rural girls enter into a marital relationship, it is usually the bride that leaves her own family and relocates to her husband’s house. The estrangement of a girl from her own family undermines her chance of increasing her resources (Miller, 2004). Women may ‘marry out’ to other villages while men are usually expected to stay in the local village to ‘contribute to its development’ (Han et al., 2019), so the discriminatory rule of land allocation worsens women’s status and their property rights. Rural land unproportionally benefits men rather than women in the name of ‘protecting the entire property and overall wellness of the local village’; women have fewer quotas than men on farmland (see Chapter Six). In places where land resources are extremely limited, it is possible that women have no allocated land (e.g., HM). Consequently, rural women have to overly rely on domestic resources – their husband and offspring, preferably their son – to gain and guarantee resources in terms of farmland, a rural house and monetary support.

Becoming childless in later life is seldom intentional, since it imposes layers of suffering on the life of rural women. As suggested by HM, farming alone and enduring *ku* was ‘a memory of tears’. Lacking ‘negotiation leverage’ (farmland or rural house) intensified women’s suffering and powerlessness, and as DW demonstrated, the close relationship with one’s caregiver (her nephew) might not be reliable when conflict emerged. Once the caring relationship was interrupted for any reason, childless women would have few options but to relocate to rural institutions. This finding extends previous studies on institution admission in China, which revealed that, contrary to Western society, institutionalization dwellers are mostly males rather than females (Gu et al., 2007; Qian et al., 2017). This demographic phenomenon results from the greater number of childless men compared to childless women in Chinese society: women are less likely than men to become childless due to lifelong singleness in later life (Qian et al., 2017). Most importantly, the findings of this research unfolded the paradox that,



when becoming childlessness for an unexpected reason – usually biological infertility or loss of children – women are more likely than men to relocate to rural institutions. Their limited financial and physical capacity, along with age and health-related challenges, constricted them to endure *ku* and to bargain with extended kin for long-term care. When advancing needs emerge, the public sector may become their only option to count on.

This research established the term *placeless* to describe the multiple relocation of rural women, who are driven from marriage, remarriage and possibly later-life institutionalization for their entire life. Further, the conception of ‘precarity’ was found to be appropriate to explain the placeless situation. Defined as a ‘structural sense of uncertainty and unsureness’, precarity was broadly used to describe how being in ‘need of care’ rendered older adults, especially women, dependent on, and vulnerable to, others. In the Western context, women born in the 1940s cohort are more likely to be involved with domestic, caring work for their entire life rather than pursuing their own career and increasing their own resources (Grenier et al., 2017; Portacolone, 2013). This research has argued that, at least for women living in rural China (and possibly other restricted areas), the overreliance on marriage and domestic care renders women extremely uncertain and powerless. If becoming childless for any reason, a woman usually has limited physical resources to support her own life and to negotiate with other possible social resources for long-term care.

9.5 Contribution, policy implications and limitations

By investigating the long-term residential decision (AIP versus institutionalization) among childless older adults in rural China, this research has revealed that the choice of where to grow old is an ongoing negotiation process. This section specifies the theoretical contribution and policy implications of this research, then illustrates its limitations.

9.5.1 Contribution to literature on long-term residential decisions

This research has explored the residential experience of childless older adults, who



are usually simplified as one demographic indicator of the quantitative-dominated research tradition regarding long-term care residential decisions (Ivanova & Dykstra, 2015; Medeiros et al., 2013). The qualitative design of this research has enabled an ‘emergence of complexity’ in regarding to participants’ voice, experience and subjectivities, unfolding the structural inequalities that infuse into the individual level and influence one’s subjectivity (Charmaz, 2020; Creswell et al., 2007). This research indicates that, when making long-term residential decisions, the financial, relational and cultural disadvantage of childlessness, which further interlocks with poverty, gender discrimination and ageing, imposes great challenges on participants. It points to the need to pay attention to the childless group on the issue of long-term care, regarding them as active agents who ‘create and construct their own place and ageing experience’, and who, at the same time, are constrained by structural disadvantage and oppression (Andrews et al., 2013; Lincoln, 2010; Wiles et al., 2012).

9.5.2 Contribution to residential reasoning model

This research has confirmed and extended the residential normalcy and residential reasoning model in regard to four facets. According to Golant (2015), only when an individual falls into both the mastery *and* the comfort zone will he/she be able to achieve residential congruence (Golant, 2011), but the findings of this research challenge that argument. This research argues that the mastery experience is the precondition and primary concern for childless older adults in achieving residential congruence. As for the comfort experience, although participants value its importance to some extent, it is a ‘luxury’ experience that could be compromised.

This research has applied the term *mastery of ambivalence* to describe the fact that childless older adults in rural China are struggled to achieve residential mastery and congruence while at the same time experiencing stress, suffering and powerlessness. More importantly, the negative aspect of residential experience reflects the broader and structural disadvantages of rural living, such as poverty and gender inequality, which are further intensified by ageing and childlessness. The adaptation effort and its messy side correspond to this argument, demonstrating how adaptation and resilience are embedded in individual and structural constrictions. This general observation contains



three arguments, and each of them supplements one aspect of the residential reasoning model.

Firstly, enduring *ku* (conducting farming-related activities) requires the behavioural, psychological and cognitive effort to engage in; it is not a momentary achievement for individual who had been living in materially constricted environment for almost entire life. Previous literature tended to regard self-reliance as the key reason for ageing in place for rural residents (O’Callaghan & Warburton, 2017; Winterton & Warburton, 2011); this research added another layer of experience – the uneven, struggling and stressful side to the discourse of self-reliance. Relatedly, because enduring *ku* is such a complex experience that having the simultaneous positive and negative side, childless older adults intend to make (positive) meanings from *ku*. This behaviour is different from the notion of ‘place attachment’ in residential normalcy and reasoning model which underlined the importance of the past and the ‘lifelong continuity and attachment to one’s living surroundings’ (Golant, 2011; Granbom et al., 2014). Idealizing the memory of living circumstance and having emotional attachment to it are reasonable in contexts such as the US or Europe: the 1940s–1960s cohort had experienced the ‘golden age’ of social development in their younger period (Roth et al., 2012). However, rural residents living in China have witnessed the rapid development and progress in the last three decades and a sharp contrast between their ‘past suffering’ and the current ‘material prosperity and sufficiency’. Enduring *ku* is driven by the grand discourse on social development and economic growth in Chinese society. This finding calls for further exploration of, and reflection upon the residential-decision framework in contexts other than the West.

Secondly, acquiring long-term care from a significant other(s) is a process of negotiation, including steps of compensation, compromise and/or even disempowerment. For both the residential normalcy model and its followers, the ‘significant influencer’ of the residential decision is considered a figure whose role is to ‘influence the residential decision’ or ‘create a social expectation of where older adults should resided (Golant, 2015; Koss & Ekerdt, 2016). This research has underlined three conditions to guarantee the intensiveness and durability of care from significant others (i.e. relational connection, reciprocal exchange and cultural



acquiescence). This research further brought up the importance of power imbalance during the residential co-reasoning process, highlighting that the concern and struggle over where to grow old are the extension of one's social and cultural positions. This phenomenon is especially prominent in the childless group and other groups of individuals who are socially and culturally considered 'deviant' (e.g., the LGBT group). Intentionally or unintentionally, acquiring care and support from an informal social support network might be bound with one's class, gender and other social attributes, rendering those 'disadvantaged groups with double or triple jeopardy' more vulnerable in care (Kazyak & Park, 2020; King & Dabelko-Schoeny, 2009).

Thirdly, the residential reasoning process is gendered – at least in contexts such as rural China. Although childless men and woman suffer similarly from a lack of support in later life, bargaining for long-term care from extended kin is easier for men than for women. It is driven by a specific cultural tradition and discrimination in rural areas, and is probably similar in contexts such as rural India (Jin et al., 2013). Although it is found that older women cultivate social connections more easily than man (Leach & Joseph, 2011; Ness et al., 2014; Ryan & McKenna, 2013), evidence from the Western context requires careful examination as to its generalization. In rural China at least, the connected social atmosphere of the rural community may not be sufficient to guarantee women's wellness, especially when they lose their children – the only resources they can count on. Future research might expand this argument and explore the differentiation between women and men in terms of the residential assessment, adaptation and negotiation process.

Overall, confirming the importance of mastery and control during residential decision-making – the core argument of the residential normalcy and reasoning model (Golant, 2012; Granbom et al., 2014) – this research adds that the mastery experience is full of ambivalence. Most importantly, the ambivalence reflects the individual and structural disadvantage of childlessness in rural China. This research criticizes the residential normalcy and reasoning model for positing 'residential congruence' and 'individual adaptation' as an ideal situation that enables AIP, rendering the uneven side as well as its structural influencer invisible. In fact, there might be a number of older adults who live in degraded circumstances and choose to AIP for a variety of reasons.



It would be problematic to regard the residential decision as ‘a voluntary choice’, or to simplify their status as ‘adaptive’. Ageing in one’s own home may make it possible to make voluntary choices but it also creates anticipatory risks and challenges for groups that are socially vulnerable (Finlay et al., 2020).

It has to be notified that, methodologically, this research aimed to gain in-depth understanding on the residential decision-making process by seeking for analytic generalization instead of statistical generalization of findings. The analytic generalization of qualitative findings, according to Curtis (2000), is the theoretical findings, insights or perspectives that applied to wider theory on the basis of how findings fit with general constructs (Curtis et al., 2000). In other words, applying the findings of this research did not attempt to apply to wider populations (e.g. all rural residents) on the basis of representative statistical samples. This research criticized and revised the residential reasoning model by underscoring the importance of residential mastery, and by adding the layer of ambivalence, unevenness and messiness on such residential mastery experience. Future research which sought to apply this finding (e.g. the notion of *ku* or having someone at home) to other populations requires careful comparison and analysis.

9.5.3 Policy implication

This research contributes to the long-term care policy agenda in China by pointing out the lived experience of the childless group. It provides an explanation for why 80% of childless older adults choose to AIP rather than move to a rural institution, regardless of the dismissed financial concern (a person could endure *ku* and have someone at home). Specifically, when choosing to stay in a rural community, there are four considerations of childless older adults that deserve a policy response.

Firstly, the needs for rural institutions are expending. Even most of childless older adults express the preference for ageing in place, the risk of age-related decline and support deficit exists. Since the Chinese government are attempting to implement the 90-7-3 structure of long-term care system, the ‘vulnerable group’ who fall into the 3% category of the structure – childless older adults and other socially and physically



disadvantageous group requires policy attention. Rural institutions may consider getting prepared for the upcoming increasing numbers of admission for the next decades, since the number of unmarried rural residents has been rising (Jin et al., 2013). Chinese government may also get prepared for the increasing financial budget on the long-term care system, including the budget for rural institution and related medical assistance for childless older adults who has worse health status than their parental counterpart, and who would needs intensive care when health declined.

Secondly, even most of childless older adults intended to endure *ku* in rural communities, the health-related challenges remain. This research suggests a trusted relationship between childless older adults and village committee – the closest resources of formal support for most of rural residents. As the targeted anti-poverty program continues, the village committee may continuing establishing regular visiting scheme with the childless residents, paying attention to their health status and keeping records. The village committee may also continue to inform the choice of institutionalization to villagers, including childless older adults and their parental peers to legitimize and normalize such choices. It is important to express the message of, ‘relocating to rural institutions is not contradict with the moral appropriateness of enduring *ku*’.

Thirdly, although ‘having someone at home’ seems a feasible and preferable way to manage one’s long-term care needs, such a caring relationship requires compensation. Policymakers may consider increasing the ‘negotiation leverage’ of childless persons – to protect one’s individual property or increase monetary subsidies to prompt their negotiation mastery. The village committee could also play role in helping the childless older adults and their significant other(s) to establish legal contract of care. Because some (potential) caregivers and childless older adults are hesitating on the compensation and succession legal term when establishing long-term care relationship, the village committee and village cadre could clarify the legal obligations and rights to both parties to initiate caring relationship.

Finally, because women are more reliant on offspring to secure later-life wellness, childlessness renders them extremely vulnerable and disadvantaged. Their



disadvantage is evident when acquiring long-term care from extended kin, since women may not have the negotiation ‘leverage’ that men do. Policymakers may consider providing extra assistance to rural women. Some places have attempted to increase quotas of farmland distribution to rural woman, and this has proved to enhance their autonomy in regard to marriage (Han et al., 2019). This research has suggested that, although becoming childless in later life is unforeseeable, securing one’s ‘leverage’ still plays a role in contributing to mastery and wellness.

9.5.4 Limitations of this research

Several limitations of this study should be pointed out. Firstly, the findings of this research were based on a small selective sample in Yunnan, which may limit the transferability of findings to other provinces and areas. For example, the warm weather of Yunnan province may release the stress of farming, and the dispersed geographical allocation of villages may increase the difficulties to access to further medical support in town. This research argues for the analytic generalization of findings – the extend findings could provide coherent and in-depth understanding on participant’s experience, not the statistical generalization that aiming to generate linear relationship between two variables. However, future research may consider applying these findings to other rural areas (e.g. plain area or coastal areas) for further investigation.

Secondly, using the residential reasoning framework as sensitizing concept may forcing empirical data to existing concepts and frameworks. It is possible that the analysis of this research is influenced, even if not biased because multiple methods were conducted to ensure rigorousness, by the residential reasoning framework. Future research may benefit from adopting other theoretical framework or other method design (e.g. narrative analysis, life story approach) to analysis participant’s narrative. It would be inspiring to investigate, for example, how childless position their sense of control into the discourse of rural poverty.

Besides, as regards the research design, the question ‘How was your previous living situation before moving?’ was retrospective for the institutionalization group. This may have been biased by participants’ current circumstances and their mixed



memories (Grossoehme & Lipstein, 2016). Future research may take this method deficit into consideration and collect longitudinal data to track their residential reasoning process over time.

Finally, it should be noted that the researcher differs socio-economically from the participants. For example, I noticed that the participants tended to report positive aspect of their physical, emotional status or residential circumstances. I also noticed that female participants were more likely to regard their experience ‘unimportant’ during interview or being interrupted by their younger family members (e.g., nephew or husband) if this member participated in interview process. Without any intention to blame or disregard participant’s positive experience, I inclined to connect such narrative, perceptions and behaviour to the lifelong disadvantageous situation of participants, which further embedded in the structural discrimination against childless person, especially childless women in rural area (details were illustrated on the final section of chapter Four). Notifying this positionality of researcher and analysis process enabled the transparency of findings (Charmaz, 2020). However, it still needs to be specified that alternative explanation of participant’s narrative and experience are possible in future research. Although several efforts were applied to enhance method rigour, the findings of this research are still open to criticism and further investigation.

9.6 Future studies

Based on findings of this study, future studies may consider pursuing topics such as:

1) The experience of urban childless residents on the process of residential reasoning. When urban residents do not rely on physical and mental strength to farm to secure daily life, the evaluation and adaptation experience may be different from their rural counterpart. It would be beneficial to gain more qualitative data from urban participants and to compare the broader differentiation and inequality of urban-rural long-term care system in China’s society.

2) The experiential differentiation between *shidu* group (childless older adults who



lost their only child) and childless group who had never have children. This research did not separate the *shidu* group with other types of childless older adults because all situations are deemed support deficit in later life (Gu, Dupre, et al., 2007). However, *shidu* group and other childless older adults are different in terms of functional ability and psychological wellness (Feng, 2018). Future research may consider comparing two groups and investigate, for example, how *shidu* group narrated and positioned their *ku* experience, how such experience intertwined with the fact of losing one's children.

3) Women's experience during residential reasoning process. As argued before, childless women are more vulnerable than man in terms of property acceleration and exchanging support from kin. Future research may pay attention to such gender inequality and its interlocked effect with rural poverty. It would also be inspiring to compare the narrative of *ku* among childless women with childless man, as literature suggested that women's *suku* (telling the individual suffering experience) provided a lens to understand women's oppression (Zhou, 2015).

4) How childless older adults regain mastery on rural institutions. Applying the comparative method, this research underlined the experience of AIP group and the retrospective experience of institutionalized group. This left the space to investigate the voice of institutionalized group after relocation. Future research may consider expanding the sample size and focus on, for example, how childless residents adapt after relocating and what is the adaptation consequence. It would enable the policymakers and staff of institution to improve the services quality in rural institutions.

9.7 Summary and conclusion

This research has explored the residential reasoning process among childless older adults in rural China, investigating why certain residential decisions (regarding AIP and institutionalization) were made and how. The summarized finding of this research is shown on following figure:



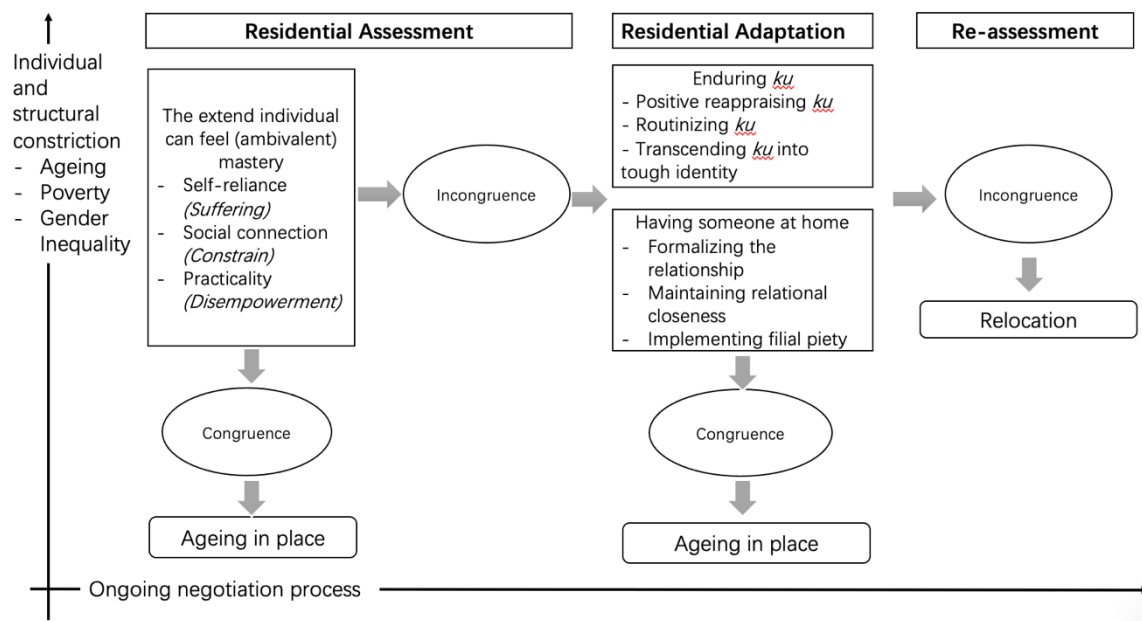


Figure 9.1 The residential reasoning process of childless older adults in rural China

Guided by the constructivist grounded theory approach, this research concludes that the reason why certain residential decision was due to sense of mastery. The mastery experience in rural areas encompassed simultaneously the resilient characteristics of self-reliance and negative experiences such as stressfulness, suffering and powerlessness on the individual and interpersonal level.

Sustaining mastery resulted from ongoing negotiation. Two levels of negotiation were identified in this research: (a) enduring *ku* – conducting farming and farm-related activities; and (b) negotiating for someone at home – reaching out for support from significant others. When a person could sustain mastery on either level, that is, when he/she could endure *ku* or he/she perceived ‘having someone at home’, the choice of AIP was preferable to institutionalization. Otherwise, when he/she was overwhelmed by the stressfulness and suffering of *ku*, or he/she did not have someone, relocating to a rural institution became more likely.

Notably, the individual and structural constriction of childlessness overshadowed the whole reasoning process, infusing into the process of residential assessment and adaptation, rendering childless older adults in rural China disadvantaged and vulnerable. This manifested in two facets: (a) farming in constrictive rural areas was full of

stressfulness, suffering and uncertainty; (b) negotiating for long-term care and support from extended kin came along with a variety of compromises and compensation. This was especially deteriorated with intersected impact from ageing, poverty and gender inequality. Childless older adults had limited resources but their own physical strength to count on, which was especially under threatening during ageing. Poverty increased the chance to become childless while intensify the negative influence of childlessness in later life, raising the stressfulness of farming and the difficulty to negotiate for long-term care. Childless women were more disadvantage than their male counterpart on conducting farming activities and negotiating for long-term care from extended kin, because they were physically more vulnerable and occupied fewer resources to negotiate for kin support. This suggested that the residential assessment and adaptation reflect the overall disadvantage of childlessness, which is further intersected with poverty, gender inequality and ageing in rural China.



Reference

- Adriansen, H. K. (2012). Timeline interviews: A tool for conducting life history research. *Qualitative Studies*, 3(1), 40–55. <https://doi.org/10.7146/qs.v3i1.6272>
- Ahn, M., Kang, J., & Kwon, H. J. (2020). The concept of aging in place as intention. *The Gerontologist*, 60(1), 50–59. <https://doi.org/10.1093/geront/gny167>
- Albertini, M., & Mencarini, L. (2014). Childlessness and support networks in later life. *Journal of Family Issues*, 35(3), 331–357. <https://doi.org/10.1177/0192513X12462537>
- Aldwin, C., & Igarashi, H. (2012). An ecological model of resilience in late life. *Annual Review of Gerontology and Geriatrics*, 32(1), 115–130. <https://doi.org/10.1891/0198-8794.32.115>
- Allen, R. E. S., & Wiles, J. L. (2013). How older people position their late-Life childlessness: A qualitative study. *Journal of Marriage and Family*, 75(1), 206–220. <https://doi.org/10.1111/j.1741-3737.2012.01019.x>
- Allen, R. E. S., & Wiles, J. L. (2014). Receiving support when older: What makes it OK? *Gerontologist*, 54(4), 670–682. <https://doi.org/10.1093/geront/gnt047>
- Anderson, E., Larkins, S., Beaney, S., & Ray, R. (2018). Should I stay or Go: Rural Ageing, a Time for Reflection. *Geriatrics*, 3(3), 49–63. <https://doi.org/10.3390/geriatrics3030049>
- Andrews, G. J., Cutchin, M., McCracken, K., Phillips, D. R., & Wiles, J. (2007). Geographical gerontology: The constitution of a discipline. *Social Science and Medicine*, 65(1), 151–168. <https://doi.org/10.1016/j.socscimed.2007.02.047>
- Andrews, G. J., Evans, J., & Wiles, J. L. (2013). Re-spacing and re-placing gerontology: Relationality and affect. *Ageing and Society*, 33(8), 1339–1373.



<https://doi.org/10.1017/S0144686X12000621>

- Andrews, G. J., Milligan, C., Phillips, D. R., & Skinner, M. W. (2009). Geographical gerontology: Mapping a disciplinary intersection. *Geography Compass*, 3(5), 1641–1659. <https://doi.org/10.1111/j.1749-8198.2009.00270.x>
- Arksey, H., & O'Malley, L. (2005). Scoping studies: Towards a methodological framework. *International Journal of Social Research Methodology: Theory and Practice*, 8(1), 19–32. <https://doi.org/10.1080/1364557032000119616>
- Atkins, M. T. (2018). “On the move, or staying put?” An analysis of intrametropolitan residential mobility and ageing in place. *Population, Space and Place*, 24(3), 1–14. <https://doi.org/10.1002/psp.2096>
- Aykan, H. (2003). Effect of childlessness on nursing home and home health care use. *Journal of Aging & Social Policy*, 15(1), 33–53. https://doi.org/10.1300/J031v15n01_03
- Bacsu, J., Jeffery, B., Abonyi, S., Johnson, S., Novik, N., Martz, D., & Oosman, S. (2014). Healthy aging in place: Perceptions of rural older adults. *Educational Gerontology*, 40(5), 327–337. <https://doi.org/10.1080/03601277.2013.802191>
- Banaszak-Holl, J., Fendrick, A. M., Foster, N. L., Herzog, A. R., Kabeto, M. U., Kent, D. M., Straus, W. L., & Langa, K. M. (2004). Predicting nursing home admission: Estimates from a 7-year follow-up of a nationally representative sample of older Americans. *Alzheimer Disease and Associated Disorders*, 18(2), 83–89. <https://doi.org/10.1097/01.wad.0000126619.80941.91>
- Barry, A., Heale, R., Pilon, R., & Lavoie, A. M. (2018). The meaning of home for ageing women living alone: An evolutionary concept analysis. *Health & Social Care in the Community*, 26(3), e337–e344. <https://doi.org/10.1111/hsc.12470>
- Barusch, A., Gringeri, C., & George, M. (2011). Rigor in qualitative social work



- research: A review of strategies used in published articles. *Social Work Research*, 35(1), 11–19. <https://doi.org/10.1093/swr/35.1.11>
- Bell, K. (2013). Constructions of “infertility” and some lived experiences of involuntary childlessness. *Affilia*, 28(3), 284–295. <https://doi.org/10.1177/0886109913495726>
- Bian, Y. (2018). The prevalence and the increasing significance of guanxi. *The China Quarterly*, 235, 597–621. <https://doi.org/10.1017/S0305741018000541>
- Bian, Y., Zhang, L., Yang, J., Guo, X., & Lei, M. (2015). Subjective wellbeing of Chinese people: A multifaceted view. *Social Indicators Research*, 121(1), 75–92. <https://doi.org/10.1007/s11205-014-0626-6>
- Black, K. (2008). Health and aging-in-place: Implications for community practice. *Journal of Community Practice*, 16(1), 79–95. <https://doi.org/10.1080/10705420801978013>
- Bolin, J. N., Phillips, C. D., & Hawes, C. (2006). Differences between newly admitted nursing home residents in rural and nonrural areas in a national sample. *Gerontologist*, 46(1), 33–41. <https://doi.org/10.1093/geront/46.1.33>
- Booth, T., & Booth, W. (1996). Sounds of silence: Narrative research with inarticulate subjects. *Disability and Society*, 11(1), 55–70. <https://doi.org/10.1080/09687599650023326>
- Bruch, E., & Feinberg, F. (2017). Decision-making processes in social contexts. *Annual Review of Sociology*, 43(1), 207–227. <https://doi.org/10.1146/annurev-soc-060116-053622>
- Bryant, A. (2017). *Grounded theory and grounded theorizing: Pragmatism in research practice*. Oxford University Press.



- Bryant, A., & Charmaz, K. (2007). Grounded theory in historical perspective: An epistemological account. In A. Bryant & K. Charmaz (Eds.), *The SAGE Handbook of Grounded Theory* (pp. 31–57). SAGE.
<https://doi.org/10.4135/9781848607941>
- Bureau of Statistics of Yunnan Province. (2020). *Yearly Sensus Data*.
http://stats.yn.gov.cn/tjsj/tjgb/202103/t20210326_1048207.html
- Byrnes, M., Lichtenberg, P. A., & Lysack, C. (2006). Environmental Press, Aging in Place, and Residential Satisfaction of Urban Older Adults. *Journal of Applied Sociology*, 23(2), 50–77. <https://doi.org/10.1177/19367244062300204>
- Cai, F., Giles, J., O’Keefe, P., & Wang, D. (2012). *The Elderly and Old Age Support in Rural China*. The World Bank. <https://doi.org/10.1596/978-0-8213-8685-9>
- Cantor, M. H. (1979). Neighbors and Friends. *Research on Aging*, 1(4), 434–463.
<https://doi.org/10.1177/016402757914002>
- Castora-Binkley, M., Meng, H., & Hyer, K. (2014). Predictors of long-term nursing home placement under competing risk: Evidence from the health and retirement study. *Journal of the American Geriatrics Society*, 62(5), 913–918.
<https://doi.org/10.1111/jgs.12781>
- Centers for Disease Control and Prevention. (2013). *Healthy places terminology: Aging in place*. <https://www.cdc.gov/healthyplaces/terminology.htm>
- Charmaz, K. (2008). Constructionism and the grounded theory method. In J. A. Holstein, J. F. Gubrium, P. Alasuutari, V. Amit, & P. Atkinson (Eds.), *Handbook of Constructionist Research* (pp. 397–412). Guilford Press.
- Charmaz, K. (2014). *Constructing grounded theory*. Sage.
- Charmaz, K. (2017). The power of constructivist grounded theory for critical inquiry.



Qualitative Inquiry, 23(1), 34–45. <https://doi.org/10.1177/1077800416657105>

Charmaz, K. (2020). “With constructivist grounded theory you can’t hide”: social justice research and critical inquiry in the public sphere. *Qualitative Inquiry*, 26(2), 165–176. <https://doi.org/10.1177/1077800419879081>

Chaudhury, H., & Rowles, G. D. (2005). Between the shores of recollection and imagination: Self, aging, and home. In Graham D. Rowles. & H. Chaudhury (Eds.), *Home and identity in late life: International perspectives* (pp. 2–18).

Chen, L. (2015). Deciding to institutionalize: caregiving crisis, intergenerational communication, and uncertainty management for elders and their children in Shanghai. *Journal of Gerontological Social Work*, 58(2), 128–148. <https://doi.org/10.1080/01634372.2014.925026>

Chen, L. (2017). Power and ambivalence in intergenerational communication: Deciding to institutionalize in Shanghai. *Journal of Aging Studies*, 41, 44–51. <https://doi.org/10.1016/j.jaging.2017.03.004>

Chen, S. H. Z., & Lou, V. W. Q. (2021). Residential reasoning: how childless older adults choose between ageing in place (AIP) and institutionalisation in rural China. *Ageing and Society*, 1–19. <https://doi.org/10.1017/S0144686X2100074X>

Cho, J., & Trent, A. (2006). Validity in qualitative research revisited. *Qualitative Research*, 6(3), 319–340. <https://doi.org/10.1177/1468794106065006>

Chou, R. J.-A. (2010). Willingness to live in eldercare institutions among older adults in urban and rural China: a nationwide study. *Ageing and Society*, 30(4), 583–608. <https://doi.org/10.1017/S0144686X09990596>

Chou, R. J. A. (2011). Filial piety by contract? The emergence, implementation, and implications of the “family support agreement” in China. *Gerontologist*, 51(1), 3–16. <https://doi.org/10.1093/geront/gnq059>



- Chui, E. (2008). Ageing in Place in Hong Kong—Challenges and Opportunities in a Capitalist Chinese City. *Ageing International*, 32(3), 167–182.
<https://doi.org/10.1007/s12126-008-9015-2>
- Cloutier-Fishera, D., Joseph, A. E., Cloutier-Fisher, D., & Joseph, A. E. (2000). Long-term care restructuring in rural Ontario: Retrieving community service user and provider narratives. *Social Science and Medicine*, 50, 1037–1045.
[https://doi.org/10.1016/S0277-9536\(99\)00353-6](https://doi.org/10.1016/S0277-9536(99)00353-6)
- Cohen, D. J., & Crabtree, B. F. (2008). Evaluative criteria for qualitative research in health care: Controversies and recommendations. *The Annals of Family Medicine*, 6(4), 331–339. <https://doi.org/10.1370/afm.818>
- Coldwell, I. (2010). Retracted: masculinities in the rural and the agricultural: A literature review. *Journal of the European Society for Rural Sociology*, 50(2), 171–197. <https://doi.org/10.1111/j.1467-9523.2009.00504.x>
- Coleman, T., & Wiles, J. (2020). Being With Objects of Meaning: Cherished Possessions and Opportunities to Maintain Aging in Place. *Gerontologist*, 60(1), 41–49. <https://doi.org/10.1093/geront/gny142>
- Connidis, I. A. (2015). Exploring ambivalence in family ties: progress and prospects. *Journal of Marriage and Family*, 77(1), 77–95.
<https://doi.org/10.1111/jomf.12150>
- Costa-Font, J., Elvira, D., & Mascarilla-Miró, O. (2009). “Ageing in place”? Exploring elderly people’s housing preferences in Spain. *Urban Studies*, 46(2), 295–316. <https://doi.org/10.1177/0042098008099356>
- Creswell, J. W., Hanson, W. E., Clark Plano, V. L., & Morales, A. (2007). Qualitative research designs: selection and implementation. *The Counseling Psychologist*, 35(2), 236–264. <https://doi.org/10.1177/0011000006287390>



- Curtis, S., Gesler, W., Smith, G., & Washburn, S. (2000). Approaches to sampling and case selection in qualitative research: Examples in the geography of health. *Social Science and Medicine*, 50(7–8), 1001–1014.
[https://doi.org/10.1016/S0277-9536\(99\)00350-0](https://doi.org/10.1016/S0277-9536(99)00350-0)
- Cutchin, M. P. (2003). The process of mediated aging-in-place: A theoretically and empirically based model. *Social Science and Medicine*, 57(6), 1077–1090.
[https://doi.org/10.1016/S0277-9536\(02\)00486-0](https://doi.org/10.1016/S0277-9536(02)00486-0)
- Cutcliffe, J. R. (2005). Adapt or adopt: Developing and transgressing the methodological boundaries of grounded theory. *Journal of Advanced Nursing*, 51(4), 421–428. <https://doi.org/10.1111/j.1365-2648.2005.03514.x>
- Dalmer, N. K. (2019). A logic of choice: Problematizing the documentary reality of Canadian aging in place policies. *Journal of Aging Studies*, 48, 40–49.
<https://doi.org/10.1016/j.jaging.2019.01.002>
- Day, A. F., & Schneider, M. (2018). The end of alternatives? Capitalist transformation, rural activism and the politics of possibility in China. *Journal of Peasant Studies*, 45(7), 1221–1246.
<https://doi.org/10.1080/03066150.2017.1386179>
- Decrop, A. (1999). Triangulation in qualitative tourism research. *Tourism Management*, 20(1), 157–161. [https://doi.org/10.1016/S0261-5177\(98\)00102-2](https://doi.org/10.1016/S0261-5177(98)00102-2)
- Deindl, C., & Brandt, M. (2017). Support networks of childless older people: Informal and formal support in Europe. *Ageing and Society*, 37(8), 1543–1567.
<https://doi.org/10.1017/S0144686X16000416>
- Denzin, N. K. (2009). The elephant in the living room: Or extending the conversation about the politics of evidence. *Qualitative Research*, 9(2), 139–160.
<https://doi.org/10.1177/1468794108098034>



- Djundeva, M., Emery, T., & Dykstra, P. A. (2018). Parenthood and depression: Is childlessness similar to sonlessness among Chinese seniors? *Ageing and Society*, 38(10), 2097–2121. <https://doi.org/10.1017/S0144686X1700054X>
- Dobner, S., Musterd, S., & Droogleever Fortuijn, J. (2016). ‘Ageing in place’: experiences of older adults in Amsterdam and Portland. *GeoJournal*, 81(2), 197–209. <https://doi.org/10.1007/s10708-014-9613-3>
- Downey, H., Threlkeld, G., & Warburton, J. (2017). What is the role of place identity in older farming couples’ retirement considerations? *Journal of Rural Studies*, 50, 1–11. <https://doi.org/10.1016/j.jrurstud.2016.12.006>
- Dye, C. J., Willoughby, D. F., & Battisto, D. G. (2011). Advice from rural elders: What it takes to age in place. *Educational Gerontology*, 37(1), 74–93. <https://doi.org/10.1080/03601277.2010.515889>
- Dykstra, P. A. (2009). Childless Old Age. In P. Uhlenberg (Ed.), *International Handbook of Population Aging* (pp. 671–690). Springer. https://doi.org/10.1007/978-1-4020-8356-3_30
- Erickson, L. D., Call, V. R. A., & Brown, R. B. (2012). SOS-Satisfied or Stuck, Why Older Rural Residents Stay Put: Aging in Place or Stuck in Place in Rural Utah*. *Rural Sociology*, 77(3), 408–434. <https://doi.org/10.1111/j.1549-0831.2012.00084.x>
- Erickson, M. A., Krout, J., Ewen, H., & Robison, J. (2006). Should I stay or should I go? *Journal of Housing For the Elderly*, 20(3), 5–22. https://doi.org/10.1300/J081v20n03_02
- Ewen, H. H., Hahn, S. J., Erickson, M. A., & Krout, J. A. (2014). Aging in place or relocation? Plans of community-dwelling older adults. *Journal of Housing for the Elderly*, 28(3), 288–309. <https://doi.org/10.1080/02763893.2014.930366>



- Fang, E. F., Scheibye-Knudsen, M., Jahn, H. J., Li, J., Ling, L., Guo, H., Zhu, X., Preedy, V., Lu, H., Bohr, V. A., Chan, W. Y., Liu, Y., & Ng, T. B. (2015). A research agenda for aging in China in the 21st century. *Ageing Research Reviews*, 24, 197–205. <https://doi.org/10.1016/j.arr.2015.08.003>
- Feng, Z. (2018). Childlessness and vulnerability of older people in China. *Age and Ageing*, 47(2), 275–281. <https://doi.org/10.1093/ageing/afx137>
- Feng, Z., Glinskaya, E., Chen, H., Gong, S., Qiu, Y., Xu, J., & Yip, W. (2020). Long-term care system for older adults in China: policy landscape, challenges, and future prospects. *The Lancet*, 396, 1362–1372. [https://doi.org/10.1016/S0140-6736\(20\)32136-X](https://doi.org/10.1016/S0140-6736(20)32136-X)
- Feng, Zhanlian. (2019). Global convergence: aging and long-term care policy challenges in the developing world. *Journal of Aging & Social Policy*, 31(4), 291–297. <https://doi.org/10.1080/08959420.2019.1626205>
- Finlay, J. M., Gaugler, J. E., & Kane, R. L. (2020). Ageing in the margins: expectations of and struggles for ‘a good place to grow old’ among low-income older Minnesotans. *Ageing and Society*, 40(4), 759–783. <https://doi.org/10.1017/S0144686X1800123X>
- Furstenberg, F. F., Harris, L. E., Pesando, L. M., & Reed, M. N. (2020). Kinship practices among alternative family forms in western industrialized societies. *Journal of Marriage and Family*. <https://doi.org/10.1111/jomf.12712>
- Gardner, P. J. (2011). Natural neighborhood networks - Important social networks in the lives of older adults aging in place. *Journal of Aging Studies*, 25(3), 263–271. <https://doi.org/10.1016/j.jaging.2011.03.007>
- Gaugler, J. E., Duval, S., Anderson, K. A., & Kane, R. L. (2007). Predicting nursing home admission in the U.S: A meta-analysis. *BMC Geriatrics*, 7. <https://doi.org/10.1186/1471-2318-7-13>



- Gioia, D. A., Corley, K. G., & Hamilton, A. L. (2013). Seeking qualitative rigor in inductive research: notes on the gioia methodology. *Organizational Research Methods, 16*(1), 15–31. <https://doi.org/10.1177/1094428112452151>
- Gitlin, L. N. (2003). Conducting research on home environments: lessons learned and new directions. *Gerontologist, 43*(5), 628–637. <https://doi.org/10.1093/geront/43.5.628>
- Glaser, B. G. (2002). Constructivist grounded theory? *Forum Qualitative Sozialforschung / Forum: Qualitative Social Research, 3*(3). <https://doi.org/https://doi.org/10.17169/fqs-3.3.825>
- Glaser, B. G., & Strauss, A. L. (2000). *Discovery of grounded theory: Strategies for qualitative research*. Routledge.
- Glauben, T., Herzfeld, T., Rozelle, S., & Wang, X. (2012). Persistent poverty in rural China: Where, why, and how to escape? *World Development, 40*(4), 784–795. <https://doi.org/10.1016/j.worlddev.2011.09.023>
- Golant, S. M. (2008). Commentary: Irrational exuberance for the aging in place of vulnerable low-income older homeowners. *Journal of Aging and Social Policy, 20*(4), 379–397. <https://doi.org/10.1080/08959420802131437>
- Golant, S. M. (2011). The quest for residential normalcy by older adults: Relocation but one pathway. *Journal of Aging Studies, 25*(3), 193–205. <https://doi.org/10.1016/j.jaging.2011.03.003>
- Golant, S. M. (2012). Out of their residential comfort and mastery zones: Toward a more relevant environmental gerontology. *Journal of Housing for the Elderly, 26*(1–3), 26–43. <https://doi.org/10.1080/02763893.2012.655654>
- Golant, S. M. (2015). Residential Normalcy and the Enriched Coping Repertoires of Successfully Aging Older Adults. *The Gerontologist, 55*(1), 70–82.



<https://doi.org/10.1093/geront/gnu036>

- Golant, S. M. (2018). Intellectual origins of residential normalcy theory. *Annual Review of Gerontology and Geriatrics*, 38(1), 57–72.
<https://doi.org/10.1891/0198-8794.38.57>
- Goodwin, J., & Horowitz, R. (2002). Introduction: The methodological strengths and dilemmas of qualitative sociology. *Qualitative Sociology*, 25(1), 33–47.
<https://doi.org/10.1023/A:1014300123105>
- Granbom, M., Himmelsbach, I., Haak, M., Löfqvist, C., Oswald, F., & Iwarsson, S. (2014). Residential normalcy and environmental experiences of very old people: Changes in residential reasoning over time. *Journal of Aging Studies*, 29(1), 9–19. <https://doi.org/10.1016/j.jaging.2013.12.005>
- Greenfield, E. A. (2012). Using ecological frameworks to advance a field of research, practice, and policy on aging-in-place initiatives. *Gerontologist*, 52(1), 1–12.
<https://doi.org/10.1093/geront/gnr108>
- Grenier, A., Phillipson, C., Laliberte Rudman, D., Hatzifilalithis, S., Kobayashi, K., & Marier, P. (2017). Precarity in late life: Understanding new forms of risk and insecurity. *Journal of Aging Studies*, 43, 9–14.
<https://doi.org/10.1016/j.jaging.2017.08.002>
- Grossoehme, D., & Lipstein, E. (2016). Analyzing longitudinal qualitative data: the application of trajectory and recurrent cross-sectional approaches. *BMC Research Notes*, 9(1), 136. <https://doi.org/10.1186/s13104-016-1954-1>
- Gu, D., Dupre, M. E., & Liu, G. (2007). Characteristics of the institutionalized and community-residing oldest-old in China. *Social Science and Medicine*, 64(4), 871–883. <https://doi.org/10.1016/j.socscimed.2006.10.026>
- Gu, D., Liu, G., Vlosky, D. A., & Yi, Z. (2007). Factors associated with place of



- death among the Chinese oldest old. *Journal of Applied Gerontology*, 26(1), 34–57. <https://doi.org/10.1177/0733464806296057>
- Guo, M. (2014). Parental status and late-life well-being in rural China: the benefits of having multiple children. *Aging & Mental Health*, 18(1), 19–29. <https://doi.org/10.1080/13607863.2013.799117>
- Hadley, R. A. (2018). The lived experience of older involuntary childless men. *The Annual Journal of the British Sociological Association Study Group on Auto/Biography 2017*, 1(1), 93–108.
- Hadley, R. A. (2019). “It’s most of my life - Going to the pub or the group”: The social networks of involuntarily childless older men. *Ageing and Society*, 1(1), 1–26. <https://doi.org/10.1017/S0144686X19000837>
- Han, W., Zhang, X., & Zhang, Z. (2019). The role of land tenure security in promoting rural women’s empowerment: Empirical evidence from rural China. *Land Use Policy*, 86(December 2018), 280–289. <https://doi.org/10.1016/j.landusepol.2019.05.001>
- Haushofer, J., & Fehr, E. (2014). On the psychology of poverty. *Science*, 344(6186), 862–867. <https://doi.org/10.1126/science.1232491>
- Heckhausen, J. (1997). Developmental regulation across adulthood: primary and secondary control of age-related challenges. *Developmental Psychology*, 33(1), 176–187. <https://doi.org/10.1037/0012-1649.33.1.176>
- Hershatter, G. (2011). *Gender of Memory*. University of California Press.
- Hillcoat-Nallétamby, S., & Ogg, J. (2014). Moving beyond “ageing in place”: Older people’s dislikes about their home and neighbourhood environments as a motive for wishing to move. *Ageing and Society*, 34(10), 1771–1796. <https://doi.org/10.1017/S0144686X13000482>



- Hollander, J. A. (2013). "I demand more of people": Accountability, Interaction, and Gender Change. *Gender & Society*, 27(1), 5–29.
<https://doi.org/10.1177/0891243212464301>
- Hollos, M. (2003). Profiles of Infertility in Southern Nigeria : Women ' s Voices from Amakiri Source. *African Journal of Reproductive Health*, 7(2), 46–56.
- Holton, J. A. (2007). The Coding Process and Its Challenges. In A. Bryant & K. Charmaz (Eds.), *The SAGE Handbook of Grounded Theory* (pp. 265–289). SAGE Publications Ltd. <https://doi.org/10.4135/9781848607941.n13>
- Hsieh, N., & Zhang, Z. (2021). Childlessness and Social Support in Old Age in China. *Journal of Cross-Cultural Gerontology*. <https://doi.org/10.1007/s10823-021-09427-x>
- Hsu, H., & Shyu, Y.-I. L. (2003). Implicit Exchanges in Family Caregiving for Frail Elders in Taiwan. *Qualitative Health Research*, 13(8), 1078–1093.
<https://doi.org/10.1177/1049732303256370>
- Huang, W., & Zhang, C. (2021). The Power of Social Pensions: Evidence from China's New Rural Pension Scheme. *American Economic Journal: Applied Economics*, 13(2), 179–205. <https://doi.org/10.1257/app.20170789>
- Huang, X., & Wu, B. (2020). Impact of urban-rural health insurance integration on health care: Evidence from rural China. *China Economic Review*, 64(September), 101543. <https://doi.org/10.1016/j.chieco.2020.101543>
- Ivanova, K., & Dykstra, P. (2015). Aging Without Children. *Public Policy & Aging Report*, 25(3), 98–101. <https://doi.org/10.1093/ppar/prv014>
- Jansuwan, P., & Zander, K. K. (2021). What to do with the farmland? Coping with ageing in rural Thailand. *Journal of Rural Studies*, 81, 37–46.
<https://doi.org/10.1016/j.jrurstud.2020.12.003>



- Jiang, N., Lou, V. W. Q., & Lu, N. (2018). Does social capital influence preferences for aging in place? Evidence from urban China. *Aging & Mental Health*, 22(3), 405–411. <https://doi.org/10.1080/13607863.2016.1249455>
- Jiang, Q., Feldman, M. W., & Jin, X. (2005). Estimation of the number of missing females in China: 1900-2000. *Chinese Journal of Population Science*, 4, 2–11.
- Jin, X., Liu, L., Li, Y., Feldman, M., & Li, S. (2013). “bare branches” and the marriage market in rural China. *Chinese Sociological Review*, 46(1), 83–104. <https://doi.org/10.2753/CSA2162-0555460104>
- Johansson, K., Josephsson, S., & Lilja, M. (2009). Creating possibilities for action in the presence of environmental barriers in the process of “ageing in place.” *Ageing and Society*, 29(1), 49–70. <https://doi.org/10.1017/S0144686X08007538>
- Kahana, E., Lovegreen, L., Kahana, B., & Kahana, M. (2003). Influences on Residential Satisfaction of Elders. *Environment and Behavior*, 35(3), 434–453. <https://doi.org/10.1177/0013916503251447>
- Kahneman, D., & Tversky, A. (1979). Prospect theory: An analysis of decision under risk. *Econometrica*, 47(2), 263–291.
- Kazyak, E., & Park, N. K. (2020). Doing family: The reproduction of heterosexuality in accounts of parenthood. *Journal of Sociology*, 56(4), 646–663. <https://doi.org/10.1177/1440783319888288>
- Kendig, H., Dykstra, P. A., Van Gaalen, R. I., & Melkas, T. (2007). Health of aging parents and childless individuals. *Journal of Family Issues*, 28(11), 1457–1486. <https://doi.org/10.1177/0192513X07303896>
- Kendig, H., Gong, C. H., Cannon, L., & Browning, C. (2017). Preferences and Predictors of Aging in Place: Longitudinal Evidence from Melbourne, Australia. *Journal of Housing for the Elderly*, 31(3), 259–271.



<https://doi.org/10.1080/02763893.2017.1280582>

- King, S., & Dabelko-Schoeny, H. (2009). Quite frankly, i have doubts about remaining: Aging-in-place and health care access for rural midlife and older lesbian, gay, and bisexual individuals. *Journal of LGBT Health Research*, 5(1–2), 10–21. <https://doi.org/10.1080/15574090903392830>
- Klaus, D., & Schnettler, S. (2016). Social networks and support for parents and childless adults in the second half of life: Convergence, divergence, or stability? *Advances in Life Course Research*, 29, 95–105. <https://doi.org/10.1016/j.alcr.2015.12.004>
- Koss, C., & Ekerdt, D. J. (2016). Residential Reasoning and the Tug of the Fourth Age. *The Gerontologist*, 57(5), 921–929. <https://doi.org/10.1093/geront/gnw010>
- Kuhn, T. S. (1962). *The structure of scientific revolutions*. University of Chicago Press.
- Kvale, S. (1995). The Social Construction of Validity. *Qualitative Inquiry*, 1(1), 19–40. <https://doi.org/10.1177/107780049500100103>
- Lai, L. (2016). *Hygiene, Society, and Culture in Contemporary Rural China: the Uncanny New Village*. Amsterdam University Press.
- Lawton, M. P. (1999). Quality of life in chronic illness. *Gerontology*, 45(4), 181–183. <https://doi.org/10.1159/000022083>
- Leach, B., & Joseph, G. (2011). Rural long-term care work, gender, and restructuring. *Canadian Journal on Aging*, 30(2), 211–221. <https://doi.org/10.1017/S0714980811000031>
- Lehning, A. J., Smith, R. J., & Dunkle, R. E. (2015). Do age-friendly characteristics influence the expectation to age in place? A comparison of low-income and



higher income detroit elders. *Journal of Applied Gerontology*, 34(2), 158–180.
<https://doi.org/10.1177/0733464813483210>

Lesthaeghe, R. (2010). The unfolding story of the second demographic transition. *Population and Development Review*, 36(2), 211–251.
<https://doi.org/10.1111/j.1728-4457.2010.00328.x>

Li, M., & Walker, R. (2018). On the origins of welfare stigma: Comparing two social assistance schemes in rural China. *Critical Social Policy*, 38(4), 667–687.
<https://doi.org/10.1177/0261018317748315>

Li, Yan. (2018). Who will care for the health of aging Chinese parents who lose their only child? A review of the constraints and implications. *International Social Work*, 61(1), 40–50. <https://doi.org/10.1177/0020872815603782>

Li, Yuheng, Su, B., & Liu, Y. (2016). Realizing targeted poverty alleviation in China: People's voices, implementation challenges and policy implications. *China Agricultural Economic Review*, 8(3), 443–454. <https://doi.org/10.1108/CAER-11-2015-0157>

Lincoln, Y. S. (2010). “What a long, strange trip it’s been...”: Twenty-five years of qualitative and new paradigm research. *Qualitative Inquiry*, 16(1), 3–9.
<https://doi.org/10.1177/1077800409349754>

Lindquist, L. A., Ramirez-Zohfeld, V., Sunkara, P., Forcucci, C., Campbell, D., Mitzen, P., & Cameron, K. A. (2016). Advanced life events (ALEs) that impede aging-in-place among seniors. *Archives of Gerontology and Geriatrics*, 64, 90–95. <https://doi.org/10.1016/j.archger.2016.01.004>

Litwak, E., & Longino, C. F. (1987). Migration Patterns Among the Elderly: A Developmental Perspective. *The Gerontologist*, 27(3), 266–272.
<https://doi.org/10.1093/geront/27.3.266>



- Liu, L.-J. ;, Sun, X. ;, Zhang, C.-L. ;, & Guo, Q. (2007). Health-Care Utilization among Empty-Nesters in the Rural Area of a Mountainous County in China. *Public Health Reports*, 122(3), 407–413.
- Liu, Y., Liu, J., & Zhou, Y. (2017). Spatio-temporal patterns of rural poverty in China and targeted poverty alleviation strategies. *Journal of Rural Studies*, 52, 66–75. <https://doi.org/10.1016/j.jrurstud.2017.04.002>
- Lloyd, A., Kendall, M., Starr, J. M., & Murray, S. A. (2016). Physical, social, psychological and existential trajectories of loss and adaptation towards the end of life for older people living with frailty: A serial interview study. *BMC Geriatrics*, 16(1), 1–15. <https://doi.org/10.1186/s12877-016-0350-y>
- Lofqvist, C., Granbom, M., Himmelsbach, I., Iwarsson, S., Oswald, F., & Haak, M. (2013). Voices on relocation and aging in place in very old age--a complex and ambivalent matter. *Gerontologist*, 53(6), 919–927. <https://doi.org/10.1093/geront/gnt034>
- Lou, V. W. Q., & Ci, Q. (2014). Ageing under the one-child policy: long-term care needs and policy choices in urban China. *International Journal of Public Policy*, 10(4/5), 231–242. <https://doi.org/10.1504/IJPP.2014.063095>
- Lou, V. W. Q., & Ng, J. W. (2012). Chinese older adults resilience to the loneliness of living alone: A qualitative study. *Aging and Mental Health*, 16(8), 1039–1046. <https://doi.org/10.1080/13607863.2012.692764>
- Lu, B., Liu, X., & Yang, M. (2017). A Budget Proposal for China's Public Long-Term Care Policy. *Journal of Aging and Social Policy*, 29(1), 84–103. <https://doi.org/10.1080/08959420.2016.1187058>
- Lum, T. Y. S., Lou, V. W. Q., Chen, Y., Wong, G. H. Y., Luo, H., & Tong, T. L. W. (2016). Neighborhood support and aging-in-place preference among low-income elderly Chinese city-dwellers. *Journals of Gerontology - Series B Psychological*



Sciences and Social Sciences, 71(1), 98–105.

<https://doi.org/10.1093/geronb/gbu154>

Luo, B., & Zhan, H. (2012). Filial Piety and Functional Support: Understanding Intergenerational Solidarity Among Families with Migrated Children in Rural China. *Ageing International*, 37(1), 69–92. <https://doi.org/10.1007/s12126-011-9132-1>

Luttrell, A., Petty, R. E., & Briñol, P. (2016). Ambivalence and certainty can interact to predict attitude stability over time. *Journal of Experimental Social Psychology*, 63, 56–68. <https://doi.org/10.1016/j.jesp.2015.11.008>

Mackenzie, L., Curryer, C., & Byles, J. E. (2015). Narratives of home and place: Findings from the Housing and Independent Living Study. *Ageing and Society*, 35(8), 1684–1712. <https://doi.org/10.1017/S0144686X14000476>

Matlabi, H., Behtash, H. H., & Shafiei, M. (2016). Admission to a Nursing Home: Viewpoints of Institutionalized Older People about Replacement. *Elderly Health Journal Shahid Sadoughi University of Medical Sciences*, 2(1), 1–5.

Matsumoto, H., Naruse, T., Sakai, M., & Nagata, S. (2016). Who prefers to age in place? Cross-sectional survey of middle-aged people in Japan. *Geriatrics and Gerontology International*, 16(5), 631–637. <https://doi.org/10.1111/ggi.12503>

Mays, N., & Pope, C. (1995). Qualitative Research: Rigour and qualitative research. *BMJ*, 311(6997), 109–112. <https://doi.org/10.1136/bmj.311.6997.109>

Medeiros, K., Rubinstein, R. L., Onyike, C. U., Johnston, D. M., Baker, A., McNabney, M., Lyketsos, C. G., Rosenblatt, A., & Samus, Q. M. (2013). Childless Elders in Assisted Living: Findings from the Maryland Assisted Living Study. *Journal of Housing For the Elderly*, 27(1–2), 206–220. <https://doi.org/10.1080/02763893.2012.754823>



- Milbourne, P., & Doheny, S. (2012). Older people and poverty in rural Britain: Material hardships, cultural denials and social inclusions. *Journal of Rural Studies*, 28(4), 389–397. <https://doi.org/10.1016/j.jrurstud.2012.06.007>
- Miles, M. B., & Huberman, A. M. (1994). *Qualitative data analysis: An expanded sourcebook*. Sage.
- Miller, E. T. (2004). Filial Daughters, Filial Sons: Comparisons from Rural North China. In C. Ikels (Ed.), *Filial Piety: Practice and Discourse in Contemporary East Asia* (pp. 34–52).
- Morse M, J., Michael, B., Maria, M., Karin, O., & Jude, S. (2002). Verification Strategies for Establishing Reliability and Validity in Qualitative Research. *International Journal of Qualitative Methods*, 1(2), 1–19.
- Nagar-Ron, S., & Motzafi-Haller, P. (2011). “My Life? There Is Not Much to Tell”: On Voice, Silence and Agency in Interviews With First-Generation Mizrahi Jewish Women Immigrants to Israel. *Qualitative Inquiry*, 17(7), 653–663. <https://doi.org/10.1177/1077800411414007>
- National Bureau of Statistics of China. (2019). *Yearly Sensus Data*. <http://data.stats.gov.cn/easyquery.htm?cn=C01&zbs=A0P06&sj=2019>
- National Bureau of Statistics of China. (2015). *National economic and social development communication*. http://www.stats.gov.cn/tjsj/zxfb/201602/t20160229_1323991.html
- Ness, T. M., Hellzen, O., & Enmarker, I. (2014). Struggling for independence: The meaning of being an oldest old man in a rural area. Interpretation of oldest old men’s narrations. *International Journal of Qualitative Studies on Health and Well-Being*, 9(1), 1–8. <https://doi.org/10.3402/qhw.v9.23088>
- Ng, Y. M., Tilse, C., & Wilson, J. (2020). Lifecourse and housing careers of childless



- and poor older Malaysians. *Ageing and Society*, 40(5), 1130–1150.
<https://doi.org/10.1017/S0144686X18001617>
- Nie, G. (2020). Marriage squeeze, marriage age and the household savings rate in China. *Journal of Development Economics*, 147, 1–26.
<https://doi.org/10.1016/j.jdeveco.2020.102558>
- O’Callaghan, Z., & Warburton, J. (2017). No one to fill my shoes: Narrative practices of three ageing Australian male farmers. *Ageing and Society*, 37(3), 441–461.
<https://doi.org/10.1017/S0144686X1500118X>
- O’Meara, P. (2019). The ageing farming workforce and the health and sustainability of agricultural communities: A narrative review. *Australian Journal of Rural Health*, 27(4), 281–289. <https://doi.org/10.1111/ajr.12543>
- O’Reilly, M., & Parker, N. (2013). “Unsatisfactory Saturation”: A critical exploration of the notion of saturated sample sizes in qualitative research. *Qualitative Research*, 13(2), 190–197. <https://doi.org/10.1177/1468794112446106>
- Okamura, T., Ura, C., Sugiyama, M., Ogawa, M., Inagaki, H., Miyamae, F., Edahiro, A., Kugimiya, Y., Okamura, M., Yamashita, M., & Awata, S. (2020). Everyday challenges facing high-risk older people living in the community: A community-based participatory study. *BMC Geriatrics*, 20(1), 1–12.
<https://doi.org/10.1186/s12877-020-1470-y>
- Organisation for Economic Co-operation and Development. (2020). *Health at a Glance: Asia/Pacific 2020*. https://www.oecd-ilibrary.org/social-issues-migration-health/health-at-a-glance-asia-pacific-2020_26b007cd-en
- Oswald, F., Wahl, H. W., Mollenkopf, H., & Schilling, O. (2003). Housing and life satisfaction of older adults in two rural regions in Germany. *Research on Aging*, 25(2), 122–143. <https://doi.org/10.1177/0164027502250016>



- Oswald, F., Wahl, H. W., Schilling, O., Nygren, C., Fänge, A., Sixsmith, A., Sixsmith, J., Széan, Z., Tomsone, S., & Iwarsson, S. (2007). Relationships between housing and healthy aging in very old age. *Gerontologist*, 47(1), 96–107. <https://doi.org/10.1093/geront/47.1.96>
- Pan, Y. (2017). *Rural Welfare in China*. Springer.
- Pang, L., Brauw, A. de, & Rozelle, S. (2004). Working until You Drop: The Elderly of Rural China. *The China Journal*, 52(52), 73–94. <https://doi.org/10.2307/4127885>
- Pani-Harreman, K. E., Bours, G. J. J. W., Zander, I., Kempen, G. I. J. M., & Van Duren, J. M. A. (2020). Definitions, key themes and aspects of “ageing in place”: A scoping review. *Ageing and Society*, 1–34. <https://doi.org/10.1017/S0144686X20000094>
- Peace, S., Holland, C., & Kellaher, L. (2011). “Option recognition” in later life: Variations in ageing in place. *Ageing and Society*, 31(5), 734–757. <https://doi.org/10.1017/S0144686X10001157>
- Perrine Susan. (2005). 基因的改变NIH Public Access. *Bone*, 23(1), 1–7. <https://doi.org/10.1038/jid.2014.371>
- Perry, T. E., Andersen, T. C., & Kaplan, D. B. (2014). Relocation remembered: Perspectives on senior transitions in the living environment. *Gerontologist*, 54(1), 75–81. <https://doi.org/10.1093/geront/gnt070>
- Phoenix, C., & Sparkes, A. C. (2009). Being Fred: Big stories, small stories and the accomplishment of a positive ageing identity. *Qualitative Research*, 9(2), 219–236. <https://doi.org/10.1177/1468794108099322>
- Pimouguet, C., Rizzuto, D., Schön, P., Shakersain, B., Angleman, S., Lagergren, M., Fratiglioni, L., & Xu, W. (2016). Impact of living alone on institutionalization



- and mortality: A population-based longitudinal study. *European Journal of Public Health*, 26(1), 182–187. <https://doi.org/10.1093/eurpub/ckv052>
- Portacolone, E. (2013). The notion of precariousness among older adults living alone in the U.S. *Journal of Aging Studies*, 27(2), 166–174. <https://doi.org/10.1016/j.jaging.2013.01.001>
- Puvimanasinghe, T., Denson, L. A., Augoustinos, M., & Somasundaram, D. (2015). Narrative and silence: How former refugees talk about loss and past trauma. *Journal of Refugee Studies*, 28(1), 69–92. <https://doi.org/10.1093/jrs/feu019>
- Qian, Y., Chu, J., Ge, D., Zhang, L., Sun, L., & Zhou, C. (2017). Gender difference in utilization willingness of institutional care among the single seniors: evidence from rural Shandong, China. *International Journal for Equity in Health*, 16(1), 1–9. <https://doi.org/10.1186/s12939-017-0577-z>
- Quashie, N. T., Arpino, B., Antczak, R., & Mair, C. A. (2021). Childlessness and Health Among Older Adults: Variation Across Five Outcomes and 20 Countries. *The Journals of Gerontology: Series B*, 76(2), 348–359. <https://doi.org/10.1093/geronb/gbz153>
- Robison, J., Shugrue, N., Fortinsky, R. H., & Gruman, C. (2014). Long-term supports and services planning for the future: Implications from a statewide survey of baby boomers and older adults. *Gerontologist*, 54(2), 297–313. <https://doi.org/10.1093/geront/gnt094>
- Roth, E. G., Keimig, L., Rubinstein, R. L., Morgan, L., Eckert, J. K., Goldman, S., & Peeples, A. D. (2012). Baby Boomers in an active adult retirement community: Comity interrupted. *Gerontologist*, 52(2), 189–198. <https://doi.org/10.1093/geront/gnr155>
- Rowles, Gramham D. (1978). *The Prisoners of Space?: Exploring the Geographical Experience of Older People*. Westview Press.



- Rutagumirwa, S. K., & Bailey, A. (2019). "I Have to Listen to This Old Body": Femininity and the Aging Body. *Gerontologist*, 59(2), 368–377. <https://doi.org/10.1093/geront/gnx161>
- Ryan, A., & McKenna, H. (2013). "Familiarity" as a key factor influencing rural family carers' experience of the nursing home placement of an older relative: A qualitative study. *BMC Health Services Research*, 13(1). <https://doi.org/10.1186/1472-6963-13-252>
- Scharlach, A. E. (2017). Aging in Context: Individual and Environmental Pathways to Aging-Friendly Communities-The 2015 Matthew A. Pollack Award Lecture. *Gerontologist*, 57(4), 606–618. <https://doi.org/10.1093/geront/gnx017>
- Scheidt, R., & Windley, P. (2003). *Physical environments and aging : Critical contributions of M. Powell Lawton to theory and practice*. Haworth Press.
- Schneider, M. (2015). What, then, is a Chinese peasant? Nongmin discourses and agroindustrialization in contemporary China. *Agriculture and Human Values*, 32(2), 331–346. <https://doi.org/10.1007/s10460-014-9559-6>
- Schnettler, S., & Wöhler, T. (2016). No children in later life, but more and better friends? Substitution mechanisms in the personal and support networks of parents and the childless in Germany. *Ageing and Society*, 36(7), 1339–1363. <https://doi.org/10.1017/S0144686X15000197>
- Scocco, P., Rapattonoi, M., & Fantoni, G. (2006). Nursing home institutionalization: A source of eustress or distress for the elderly? *International Journal of Geriatric Psychiatry*, 21(3), 281–287. <https://doi.org/10.1002/gps.1453>
- Sharon Spall. (1998). Peer debriefing in qualitative research: emerging operational models. *Qualitative Inquiry*, 4(2), 280–292.
- Söderberg, M., Ståhl, A., & Melin Emilsson, U. (2013). Independence as a



stigmatizing value for older people considering relocation to a residential home. *European Journal of Social Work*, 16(3), 391–406.
<https://doi.org/10.1080/13691457.2012.685054>

Spencer, R., Pryce, J. M., & Walsh, J. (2014). Philosophical Approaches to Qualitative Research. In P. Leavy (Ed.), *The Oxford Handbook of Qualitative Research* (pp. 80–98). Oxford University Press.
<https://doi.org/10.1093/oxfordhb/9780199811755.013.027>

Stafford, G. E. (2017). Maintaining residential normalcy through self-initiated assimilative and accommodative coping while aging-in-place. *Housing and Society*, 44(1–2), 22–40. <https://doi.org/10.1080/08882746.2017.1389560>

Stones, D., & Gullifer, J. (2016). “At home it’s just so much easier to be yourself”: Older adults’ perceptions of ageing in place. *Ageing and Society*, 36(3), 449–481. <https://doi.org/10.1017/S0144686X14001214>

Sun, Y. (2017). Among a Hundred Good Virtues, Filial Piety is the First: Contemporary Moral Discourses on Filial Piety in Urban China. *Anthropological Quarterly*, 90(3), 771–799. <https://doi.org/10.1353/anq.2017.0043>

Sussman, T., & Orav-Lakaski, B. (2020). “I Didn’t Even Make My Bed”: Hospital Relocations and Resident Adjustment in Long-Term Care Over Time. *The Gerontologist*, 60(1), 32–40. <https://doi.org/10.1093/geront/gny141>

Tang, F., & Lee, Y. (2011). Social support networks and expectations for aging in place and moving. *Research on Aging*, 33(4), 444–464.
<https://doi.org/10.1177/0164027511400631>

The Ministry of Civil Affairs. (2016). *Views on Promoting Home-based Elderly Care*. http://www.gov.cn/zhengce/content/2016-02/17/content_5042525.htm

The Ministry of Civil Affairs. (2019). *National Development Report*.



<http://images3.mca.gov.cn/www2017/file/202009/1601261242921.pdf>

The Organization for Economic Co-operation and Development. (2019). *Ageing and demographic change*. <https://www.oecd.org/economy/ageing-inclusive-growth/>

The Organization for Economic Co-operation and Development. (2020). *OECD Family Database*. <http://www.oecd.org/els/family/database.htm>

The State Council of China. (2006). *Regulation of Rural Five Guarantee Scheme*. http://www.gov.cn/zwgk/2006-01/26/content_172438.htm

The World Health Organization. (2015). *China country assessment report on ageing and health*.
https://apps.who.int/iris/bitstream/handle/10665/194271/9789241509312_eng.pdf?sequence=1

Thornberg, R. (2012). Informed Grounded Theory. *Scandinavian Journal of Educational Research*, 56(3), 243–259.
<https://doi.org/10.1080/00313831.2011.581686>

Timmermans, S., & Tavory, I. (2012). Theory construction in qualitative research: From grounded theory to abductive analysis. *Sociological Theory*, 30(3), 167–186. <https://doi.org/10.1177/0735275112457914>

Treiman, D. J. (2012). The “ difference between heaven and earth” : Urban-rural disparities in well-being in China. *Research in Social Stratification and Mobility*, 30(1), 33–47. <https://doi.org/10.1016/j.rssm.2011.10.001>

Tsalach, C. (2013). Between Silence and Speech: Autoethnography as an Otherness-Resisting Practice. *Qualitative Inquiry*, 19(2), 71–80.
<https://doi.org/10.1177/1077800412462986>

Tversky, A., & Wakker, P. (1995). Risk Attitudes and Decision Weights.



Econometrica, 63(6), 1255–1280. <https://doi.org/10.2307/2171769>

- Van Dijk, H. M., Cramm, J. M., Van Exel, J., & Nieboer, A. P. (2015). The ideal neighbourhood for ageing in place as perceived by frail and non-frail community-dwelling older people. *Ageing and Society*, 35(8), 1771–1795. <https://doi.org/10.1017/S0144686X14000622>
- van Hees, S. ., Horstman, K. ., Jansen, M. ., & Ruwaard, D. (2017). Photovoicing the neighbourhood: Understanding the situated meaning of intangible places for ageing-in-place. *Health Place*, 48, 11–19. <https://doi.org/10.1016/j.healthplace.2017.08.007>
- Vasara, P. (2015). Not ageing in place: Negotiating meanings of residency in age-related housing. *Journal of Aging Studies*, 35, 55–64. <https://doi.org/10.1016/j.jaging.2015.07.004>
- Vasunilashorn, S., Steinman, B. A., Liebig, P. S., & Pynoos, J. (2012). Aging in Place: Evolution of a Research Topic Whose Time Has Come. *Journal of Aging Research*, 2012, 1–6. <https://doi.org/10.1155/2012/120952>
- Verdery, A. M. (2015). Links Between Demographic and Kinship Transitions. *Population and Development Review*, 41(3), 465–484. <https://doi.org/10.1111/j.1728-4457.2015.00068.x>
- Verdery, A. M., Margolis, R., Zhou, Z., Chai, X., Rittirong, J., & Raymo, J. (2019). Kinlessness Around the World. *Journals of Gerontology - Series B Psychological Sciences and Social Sciences*, 74(8), 1394–1405. <https://doi.org/10.1093/geronb/gby138>
- Volckaert, E., Schillebeeckx, E., & De Decker, P. (2020). Beyond nostalgia: Older people's perspectives on informal care in rural Flanders. *Journal of Rural Studies*. <https://doi.org/10.1016/j.jrurstud.2020.07.006>



- Vrkljan, B. H., Leuty, V., & Law, M. (2011). Aging-in-Place: Exploring the Transactional Relationship between Habits and Participation in a Community Context. *OTJR: Occupation, Participation and Health*, 31(3), 151–159. <https://doi.org/10.3928/15394492-20110218-01>
- Wahl, H. W., & Gerstorf, D. (2020). Person-environment resources for aging well: Environmental docility and life space as conceptual pillars for future contextual gerontology. *Gerontologist*, 60(3), 368–375. <https://doi.org/10.1093/geront/gnaa006>
- Wahl Iwarsson, S., & Oswald, F., H. W. (2012). Aging well and the environment: toward an integrative model and research agenda for the future. *Gerontologist*, 52(3), 306–316. <https://doi.org/10.1093/geront/gnr154>
- Wang, J., Yang, Q., & Wu, B. (2020). Effects of Care Arrangement on the Age of Institutionalization among Community-dwelling Chinese Older Adults. *Journal of Aging and Social Policy*, 00(00), 1–16. <https://doi.org/10.1080/08959420.2020.1726720>
- Wang, S., Li, B., Ungvari, G. S., Ng, C. H., Chiu, H. F. K., Kou, C., Liu, Y., Tao, Y., Wu, Y., Fu, Y., Qi, Y., Yu, Y., & Xiang, Y. T. (2016). Poor mental health status and its associations with demographic characteristics and chronic diseases in Chinese elderly. *Social Psychiatry and Psychiatric Epidemiology*, 51(10), 1449–1455. <https://doi.org/10.1007/s00127-016-1271-y>
- Wanka, A. (2018). Disengagement as Withdrawal from Public Space: Rethinking the Relation between Place Attachment, Place Appropriation, and Identity-Building among Older Adults. *Gerontologist*, 58(1), 130–139. <https://doi.org/10.1093/geront/gnx081>
- Weaver, R. H., Roberto, K. A., & Brossoie, N. (2020). A Scoping Review: Characteristics and Outcomes of Residents Who Experience Involuntary Relocation. *The Gerontologist*, 60(1), 20–37.



<https://doi.org/10.1093/geront/gnz035>

Weeks, L. E., Keefe, J., & Macdonald, D. J. (2012). Factors Predicting Relocation Among Older Adults. *Journal of Housing for the Elderly*, 26(4), 355–371.

<https://doi.org/10.1080/02763893.2011.653099>

Wenger, G. C. (2001). Interviewing Older People. In Jaber F. Gubrium & J. A. Holstein (Eds.), *Handbook of Interview Research* (pp. 259–278). Sage.

<https://doi.org/10.4135/9781412973588.n17>

Wenger, G. C. (2009). Childlessness at the end of life: Evidence from rural Wales. *Ageing and Society*, 29(8), 1243–1259.

<https://doi.org/10.1017/S0144686X09008381>

Wiles, J. (2003). Daily geographies of caregivers: mobility, routine, scale. *Social Science and Medicine*, 57(7), 1307–1325. [https://doi.org/10.1016/S0277-9536\(02\)00508-7](https://doi.org/10.1016/S0277-9536(02)00508-7)

Wiles, J. L. (2005). Conceptualising the importance of place in the care of older people: the role of geographical gerontology. *International Journal of Nursing for Older People*, 14, 100–108.

Wiles, J. L., Leibing, A., Guberman, N., Reeve, J., & Allen, R. E. S. (2012). The meaning of “aging in place” to older people. *Gerontologist*, 52(3), 357–366.

<https://doi.org/10.1093/geront/gnr098>

Winterton, R., & Warburton, J. (2011). Does place matter? Reviewing the experience of disadvantage for older people in rural Australia. *Rural Society*, 20, 187–197.

Winterton, R., & Warburton, J. (2012). Ageing in the bush: The role of rural places in maintaining identity for long term rural residents and retirement migrants in north-east Victoria, Australia. *Journal of Rural Studies*, 28(4), 329–337.

<https://doi.org/10.1016/j.jrurstud.2012.01.005>



- Wiseman, R. F. (1980). Why Older People Move: Theoretical Issues. *Research on Aging*, 2(2), 141–154. <https://doi.org/10.1177/016402758022003>
- Wu, B., Mao, Z.-F., & Zhong, R. (2009). Long-Term Care Arrangements in Rural China: Review of Recent Developments. *Journal of the American Medical Directors Association*, 10(7), 472–477. <https://doi.org/10.1016/j.jamda.2009.07.008>
- Wu, B., Mao, Z., & Xu, Q. (2008). Institutional Care for Elders in Rural China. *Journal of Aging & Social Policy*, 20(2), 218–239. <https://doi.org/10.1080/08959420801977632>
- Wu, Y. T., Prina, A. M., Barnes, L. E., Matthews, F. E., & Brayne, C. (2015). Relocation at older age: Results from the Cognitive Function and Ageing Study. *Journal of Public Health*, 37(3), 480–487. <https://doi.org/10.1093/pubmed/fdv050>
- Yang, W., Wu, B., Tan, S. Y., Li, B., Lou, V. W. Q., Chen, Z., Chen, X., Fletcher, J. R., Carrino, L., Hu, B., Zhang, A., Hu, M., & Wang, Y. (2021). Understanding Health and Social Challenges for Aging and Long-Term Care in China. *Research on Aging*, 43(3–4), 127–135. <https://doi.org/10.1177/0164027520938764>
- Yuan, Y., Wang, M., Zhu, Y., Huang, X., & Xiong, X. (2020). Urbanization's effects on the urban-rural income gap in China: A meta-regression analysis. *Land Use Policy*, 99(October 2019), 104995. <https://doi.org/10.1016/j.landusepol.2020.104995>
- Zaidi, B., & Morgan, S. P. (2017). The Second Demographic Transition Theory: A Review and Appraisal. *Annual Review of Sociology*, 43(1), 473–492. <https://doi.org/10.1146/annurev-soc-060116-053442>
- Zeng, Y., Lin, C., Wang, L., Zhang, L., & Fang, Y. (2020). Rural-urban disparities in unmet long-term care needs and the demand for community care services among



elderly people in China. 1–23. <https://doi.org/10.21203/rs.3.rs-16467/v1>

Zhang, H. (2004). “Living alone” and the rural elderly: strategy and agency in post-Mao rural China. In C. Ikels (Ed.), *Filial Piety: Practice and Discourse in Contemporary East Asia* (pp. 63–87). Stanford University Press.

Zhang, W. (2007). Marginalization of Childless Elderly Men and Welfare Provision: a study in a North China village. *Journal of Contemporary China*, 16(51), 275–293. <https://doi.org/10.1080/10670560701194541>

Zhang, W., & Liu, G. (2007). Childlessness, psychological well-being, and life satisfaction among the elderly in China. *Journal of Cross-Cultural Gerontology*, 22(2), 185–203. <https://doi.org/10.1007/s10823-007-9037-3>

Zhang, Weiguo. (2006). Who Adopts Girls and Why? Domestic Adoption of Female Children in Contemporary Rural China. *The China Journal*, 56, 63–82. <https://doi.org/10.2307/20066186>

Zhou, J., & Walker, A. (2016). The need for community care among older people in China. *Ageing and Society*, 36(6), 1312–1332. <https://doi.org/10.1017/S0144686X15000343>

Zhou, S. (2015). Suku and the self-valorization of Chinese women workers: Before, during, and after enterprise privatization. *Frontiers of History in China*, 10(1), 145–167. <https://doi.org/10.3868/s020-004-015-0006-8>

Zhou, Z., Verdery, A. M., Margolis, R., & Chen, F. (2019). No Spouse, No Son, No Daughter, No Kin in Contemporary China: Prevalence, Correlates, and Differences in Economic Support. *Journals of Gerontology - Series B Psychological Sciences and Social Sciences*, 74(8), 1453–1462. <https://doi.org/10.1093/geronb/gby051>

Zhu, H. (2015). Unmet needs in long-term care and their associated factors among the



oldest old in China. *BMC Geriatrics*, 15(1), 1–11.
<https://doi.org/10.1186/s12877-015-0045-9>

Zhu, Y., & Österle, A. (2017). Rural-urban disparities in unmet long-term care needs in China: The role of the hukou status. *Social Science and Medicine*, 191, 30–37.
<https://doi.org/10.1016/j.socscimed.2017.08.025>

Zimmer, Z., & Kwong, J. (2003). Family size and support of older adults in urban and rural China: Current effects and future implications. *Demography*, 40(1), 23–44.
<http://dx.doi.org/10.1016/j.jaci.2012.05.050>



Appendix

Appendix I

Ethical approval

THE UNIVERSITY OF HONG KONG

香港



大學

February 14, 2019

Ms. Hongzhou Chen
PhD Student
Department of Social Work & Social Administration

Dear Ms. Chen,

Application for Ethics Approval
HREC's Reference Number: EA1901040

I refer to your application for ethics approval of your project entitled "Aging-in-Place Decision-making Process, Quality of Life and Residential Normalcy among Three-no Older People in Rural China".

2. I am pleased to inform you that the application has been approved by the Human Research Ethics Committee (HREC) regarding the ethical aspect of the above-mentioned research project, and the expiration date of the ethical approval is February 13, 2023.
3. Starting from April 1, 2015, the HREC's reference number of your project (i.e. EA1901040) has to be shown in all materials sent to potential and actual participants to enable participants to link the materials to an approved project.
4. You are reminded to report to the Committee any amendments and new information on the project. Any deviation from the study protocol or compliance incident that has occurred during a study and may adversely affect the rights, safety or well-being of any participant or breaches of confidentiality should be reported to the HREC within 15 calendar days from the first awareness of the deviation/incident by the PI. Application for amendment(s) of an approved project including project extension should be submitted using a prescribed form which can be downloaded from the Research Services homepage (<http://www.rss.hku.hk/integrity/ethics-compliance/hrec>). Application for extension should be submitted well before the initially approved expiration date.

Yours sincerely,

T. S. A. Veitch

Professor T.S. Veitch
Chairman
Human Research Ethics Committee

c.c. Dr. V.W.Q. Lou, Department of Social Work & Social Administration

TSV/vy/bw

THE REGISTRY 教務處

POKFULAM ROAD, HONG KONG. TEL: (852) 2859 2111 FAX: (852) 2858 2549

Printed on environmentally friendly paper



Appendix II

Interview guideline of residential decision-making among childless older adults in rural China (Chinese version)

Institutionalization group	AIP group
Interview guideline on general understanding stage (stage 1)	
1 总体情况—— 您今年多大？家里多少人？身体怎么样？ 2 请分享一下您目前在这里的生活情况—— 1) 请描述您的一天； 2) 您觉得自己生活中最大的困难是？怎么解决该困难？ 3) 总的来说，您如何评价自己现在的居住环境？ 3 请分享一下您决定搬来这里住的原因和过程—— 1) 你来这里多久了？什么时候来的，当时发生了什么？ 2) 如果让你再行选择，您会选择居家养老还是搬到这里？ 4 请分享一下您将来的打算？您怎样理解和安排死亡？为什么？	1 总体情况—— 您今年多大？家里多少人？身体怎么样？ 2 请分享一下您目前在这里的生活情况—— 1) 请描述您的一天。 2) 您觉得自己生活中最大的困难是？怎么解决该困难？ 3) 总的来说，您如何评价自己的居住环境？ 3 在此居住期间有没有人提议您搬到养老院？您是什么想法？如果再让你选择一次，你是否还会做这个选择？ 4 请分享一下您将来的打算？您怎样理解和安排死亡？为什么？
Interview guideline on the in-depth understanding stage (stage 2)	
1 总体情况—— 您今年多大？家里多少人？身体怎么样？平时住在这里都在做些什么？ 2 请分享一下您目前在这里的生活情况—— 1) 住在这里的时候是否遇到什么困难？如何解决？	1 总体情况—— 您今年多大？家里多少人？身体怎么样？平时住在这里都在做些什么？ 2 请分享一下您目前在这里的生活情况—— 1) 住在这里的时候是否遇到什么困难？如何解决？

2) 您如何评价当下的生活环境? 3 您觉得居住在此苦不苦? 怎么苦? 为什么? 相比起来住在村里苦还是这里苦? 4 住在这里有没有人和您的关系比较好 (不好)? 住在村里时有没有人和您关系比较好 (不好)? 为什么? 5 当初您为何选择来养老院—— 1) 谁来劝, 怎么劝? 2) 您是什么看法? 3) 您怎么评价自己的这个决定? 6 您将来怎么打算?	2) 您如何评价当下的生活环境? 3 您觉得居住在此苦不苦? 怎么苦? 为什么? 4 住在这里有没有人和您的关系比较好 (不好)? 怎么好 (不好)? 您将来可以依靠他养老吗? 5 在此期间是否有人来劝您去养老院—— 1) 谁来劝, 怎么劝? 2) 您是什么看法? 3) 您怎么评价自己的这个决定? 6 您将来怎么打算?
Interview guideline on the theoretical understanding stage (stage 3)	
1 总体情况—— 您今年多大? 家里多少人? 身体怎么样? 平时住在这里都在做些什么? 2 请分享一下您目前在这里的生活情况—— 1) 住在这里的时候是否遇到什么困难? 如何解决? 2) 您如何评价当下的生活环境? 舒服吗? 还是习惯? 或者是别的什么? 您如何评价搬来以前的生活环境? 3 说说您的童年? 是否读过书? 后来呢? 4 说说您的家庭状况? 多少兄弟姐妹? 他们现在在做什么? 你们平时来往吗? 怎么来往? 关系好不好? 5 说说您的婚姻状况? 6 这期间有没有什么特别难忘的记忆? (大集体/文革/包产责任制/改革开放?)	1 总体情况—— 您今年多大? 家里多少人? 身体怎么样? 平时住在这里都在做些什么? 2 请分享一下您目前在这里的生活情况—— 1) 住在这里的时候是否遇到什么困难? 如何解决? 2) 您如何评价当下的生活环境? 舒服吗? 还是习惯? 或者是别的什么? 3 说说您的童年? 是否读过书? 后来呢? 4 说说您的家庭状况? 多少兄弟姐妹? 他们现在在做什么? 你们平时来往吗? 怎么来往? 关系好不好? 5 说说您的婚姻状况? 6 您这房子什么时候盖的? 为什么? 说说您以前和现在的房子? 7 这期间有没有什么特别难忘的记忆? (大集体/文革/包产责任制/改革开放?)

7 您觉得您自己是个什么样的人？ 8 您觉得居住在此苦不苦？您觉得住在村里苦不苦？怎么苦？为什么？ 9 住在这里有没有人和您的关系比较好（不好）？住在村里有没有关系比较好的人？怎么好（不好）？ 10 您当时怎么来的养老院—— 1) 谁来劝，怎么劝？ 2) 您是什么看法？ 3) 您怎么评价自己的这个决定？ 12 您将来怎么打算？	开放？） 8 您觉得您自己是个什么样的人？ 9 您觉得居住在此苦不苦？怎么苦？为什么？ 10 住在这里有没有人和您的关系比较好（不好）？怎么好（不好）？您将来可以依靠他养老吗？ 11 在此期间是否有人来劝您去养老院—— 1) 谁来劝，怎么劝？ 2) 您是什么看法？ 3) 您怎么评价自己的这个决定？ 12 您将来怎么打算？
Interview guideline on verification (stage 4)	
我们已经采访到了 27 位朋友，这是我们的初步结果。请问你对这些结果有什么看法？ 1 居住在农村总的来说还挺苦，搬来这里（养老院）就好多了，您觉得呢？ 2 有些人来养老院因为苦不住，您觉得呢？ 3 有些人搬来养老院因为家里没人，您觉得呢？ 4 您还有什么补充？	我们已经采访到了 27 位朋友，这是我们的初步结果。请问你对这些结果有什么看法？ 1 居住在农村总的来说还挺苦，您觉得呢？ 2 有些人不去养老院因为苦得住，您觉得呢？ 3 有些人不去养老院因为家里有人。您怎么看？ 4 您还有什么补充？